

Baylor Scott & White Anticoagulation Clinic Protocol and Procedures

Updated: January 2026

Developed by: Andrew Tran, Clinical Pharmacist III

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I. STATEMENT

The Anticoagulation Clinic at the Temple Clinic (ACC) is a pharmacist/nurse-driven clinic that focuses on the outpatient management of warfarin therapy for patients using Baylor Scott & White primary care physicians or specialists in Hematology/Oncology, Pulmonary, or Cardiology for their medical care. Currently the ACC manages approximately 1300 patients on warfarin for a variety of indications including but not limited to atrial fibrillation, prosthetic heart valves, venous thrombotic events, hypercoagulable disorders, thrombosis associated with malignancy, and cerebrovascular disease. Anticoagulation therapy monitoring and dosing adjustments are based on a physician-approved protocol.

The ACC utilizes face-to-face patient visits for patient interviews and warfarin dosage adjustments. (There are exceptions: home health patients, home INR testing patients, those patients who live outside of a 30-mile radius from ACC and go to local labs for anticoagulation monitoring, etc, in which telephone interviews are conducted.) This procedure allows for the timeliest actions to be taken on blood testing results as well as provides for most effective communication of patient education and dosage adjustment information in our patient population. Patient communication is provided verbally and in writing whenever possible.

II. PURPOSE

To document the written protocol for drug therapy management for anticoagulation authorized by the physicians and mid-level providers Baylor Scott & White Anticoagulation Therapy delegated to clinical pharmacists. This protocol was developed taking into consideration the qualifications and training of the clinical pharmacist to manage the clinical situations expected in the delegation of drug therapy management outlined. This drug therapy management protocol is done to comply with the Texas State Board of Pharmacy and Texas Medical Board regulations regarding drug therapy management by a pharmacist under written protocol of a physician.

III. SCOPE

This protocol for drug therapy management will be limited to patients of the Baylor Scott & White Health Anticoagulation Clinic. The attending physicians authorized to prescribe drugs and responsible for the delegations of drug therapy management are Kathleen Halka, MD and Tiffany Brown, FNP. The clinical pharmacists authorized to engage in drug therapy management as delegated by providers are Lida Bineshian PharmD, Anarys Caballes PharmD, Melanie Maine-Timbs PharmD, Linda Mgbeoduru PharmD, Aimee Nguyen PharmD, Andrew Tran PharmD.

IV. STAFF & PERSONNEL

Providers: Dr. Kathleen Halka, MD; Tiffany Brown, FNP

Practice Administrator: Courtney Cuba

Clinic Operations Manager: Donna Valerio

Certified Medical Assistant (CMA): Amanda Puckett, CMA

Patient Service Specialist (PSS): Tyler Long, PSS

Nurse Supervisor: Brianne Reeders, RN

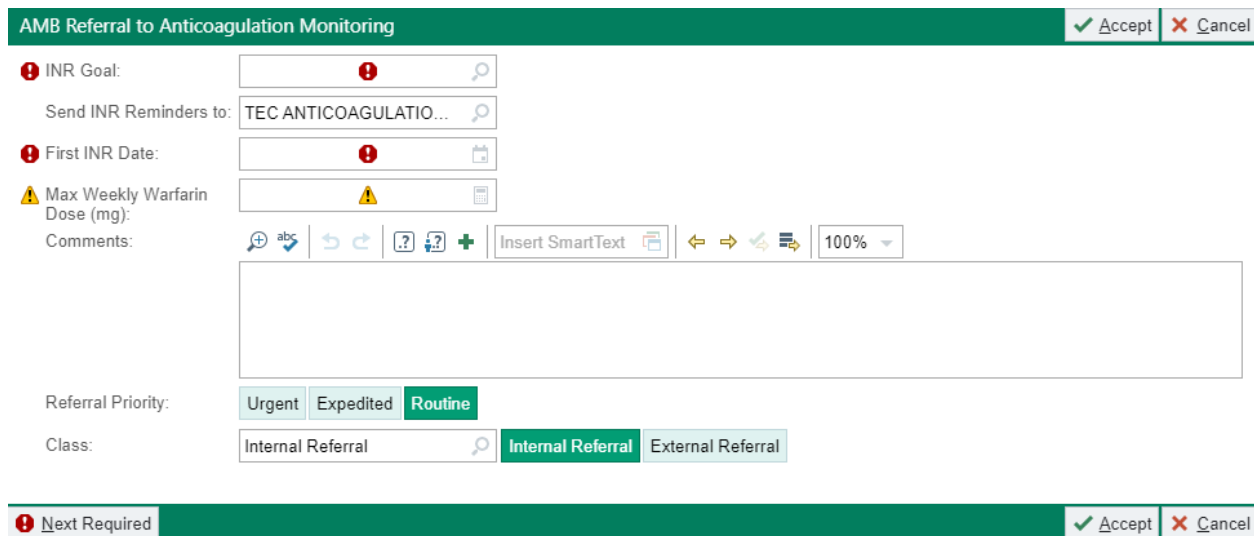
Registered Nurses: Melissa James, RN; Rashonda Venable, RN; Joyce Frei, RN; Linda Halamik, RN; Tia Beyers, RN; Donna Burns, RN; Melissa Brown, RN

Director of Pharmacy: Dwight Paulson, PharmD

Pharmacists: Lida Bineshian PharmD, Anarys Caballes PharmD, Melanie Maine-Timbs PharmD, Linda Mgbeoduru PharmD, Aimee Nguyen PharmD, Andrew Tran PharmD

V. PATIENT REFERRAL

- Enrollment in a Central Texas Outpatient Anticoagulation Clinic (ACC) is by provider referral only.
- The referring physician completes a written enrollment form or an electronic order in EPIC with the indications for anticoagulation, desired INR range, and the duration of the treatment.
- When anticoagulation therapy is initiated as an inpatient, a written referral or an electronic order is required prior to discharge.
- The referring provider is only responsible for the initial referral but will be able to view visit notes in Epic
- The medical director and administrator over an individual Anticoagulation Clinic may determine that only referrals by Baylor Scott & White providers are accepted.

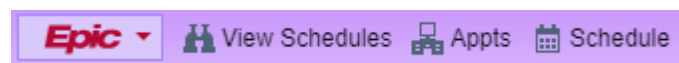


VI. SCHEDULING

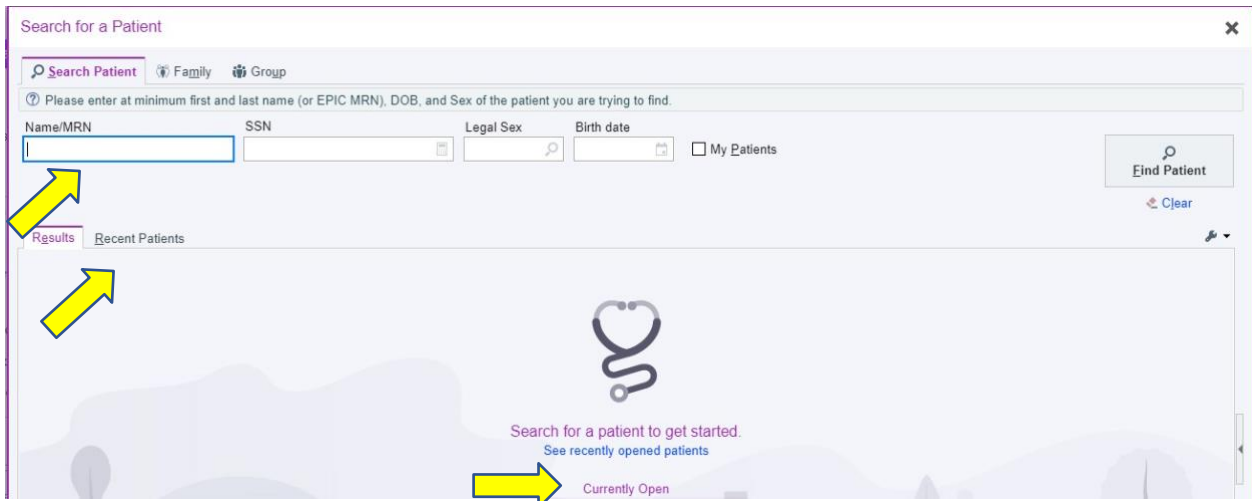
SCHEDULING PATIENT APPOINTMENT

Reference: [Anticoag Scheduling Rules Document](#)

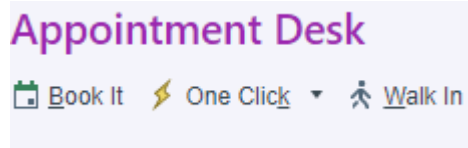
- Select **Appts** on the top left of the screen.



- Select patient, if you have their chart open it should show them below as currently opened, or you can search by name/MRN, or select recent patients

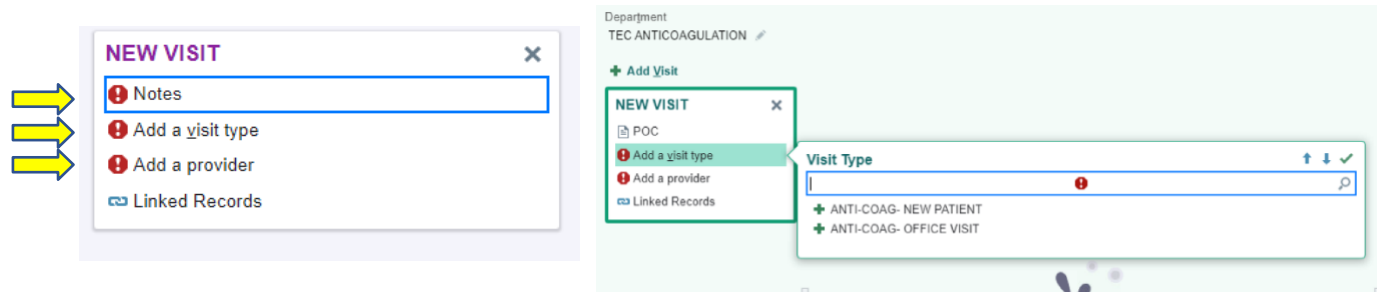


C. Once you have the patient chart open, select **Book It** in the top left corner.



D. Fill in all required fields:

- Note: POC , Eliquis 6m f/u, 6MF2F lab/self test
- Visit Type:
 - For Follow-up appointments : **ANTI-COAG-OFFICE VISIT**
 - For new patient visits: **ANTI-COAG-NEW PATIENT**
- Provider:
 - Point-of-Care visits – **TEC ANTICOAGULATION 1-4**
 - New patient education in AM /DOAC f/u PM - under **Brown, Tiffany Lauren, APRN, FNP in TEC ANTICOAGULATION**



Provider Selection

tec ANTI Any Provider Subgroup Department Specialty

Providers and Resources Selected: 4

Provider	Prov ID	Provider Type
TEC ANTICOAGULATION 1	69924	Resource
TEC ANTICOAGULATION 2	69923	Resource
TEC ANTICOAGULATION 3	69922	Resource
TEC ANTICOAGULATION 4	69899	Resource
TEC ANTICOAGULATION 5	101943	Resource
TEC ANTICOAGULATION 6	137475	Resource
SDA TEC ANTICOAGULATION	111505	Resource

Departments

- E. Click on Schedules to see list of the appointments available in each coag room.
- F. Make sure to have the appropriate day selected on the calendar on the right.

Book It

Department
TEC ANTICOAGULATION

+ Add Visit

ANTI-COAG- OFFICE VISIT

POC

Multiple providers (4)

Select a time

Additional resources

Linked Records

Solutions **Schedules**

TEC ANTICOAGULATION
TEC ANTICOAGULATION
20%

1	1130	Admin
1	1140	Admin
1	1150	Admin

7 10 20 7/18/2023

Su	Mo	Tu	We	Th	Fr	Sa
25	26	27	28	29	30	1
2	3	4	5	6	7	8
9	10	11	12	13	14	15
16	17	18	19	20	21	22
23	24	25	26	27	28	29
30	1	2	3	4	5	

Filters

Days/Times

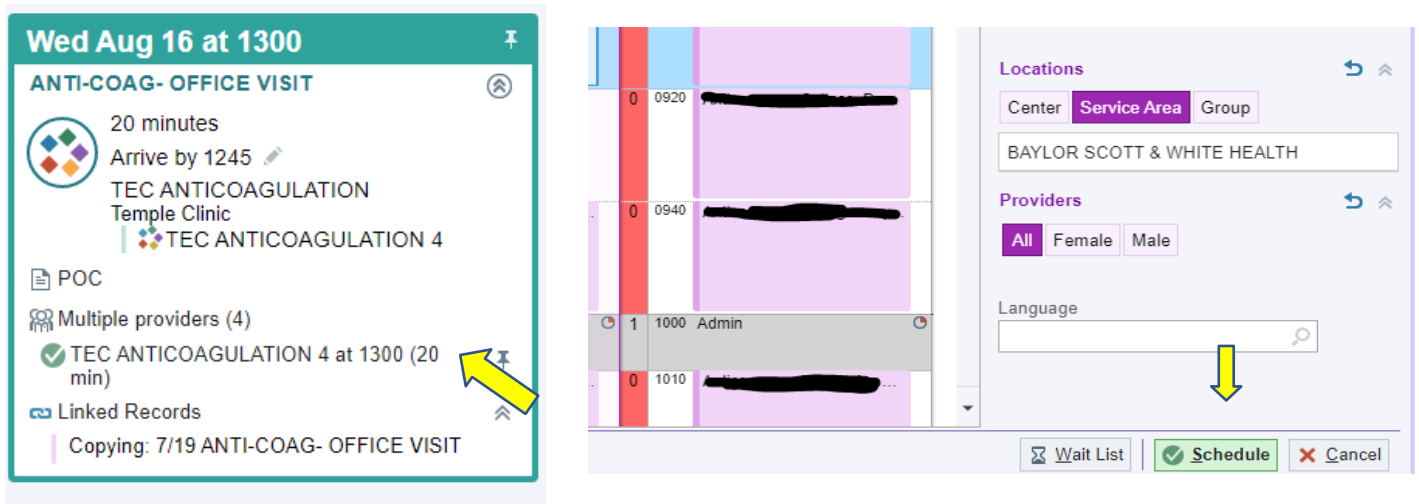
Sun Mon Tue Wed Thu Fri Sat

Morning Midday Afternoon Custom

☐ Hide full days

☒ Hide days with no template

- G. If you are needing to edit duration of visits from 20 min to 40 min (6MF2F, DOAC follow up, Spanish speaking): change it in the green box on the left. When ready, click **Schedule**.



Wed Aug 16 at 1300

ANTI-COAG- OFFICE VISIT

20 minutes
Arrive by 1245
TEC ANTICOAGULATION
Temple Clinic
TEC ANTICOAGULATION 4

POC

Multiple providers (4)

TEC ANTICOAGULATION 4 at 1300 (20 min)

Linked Records

Copying: 7/19 ANTI-COAG- OFFICE VISIT

Calendar view showing slots: 0920, 0940, 1000 Admin, 1010.

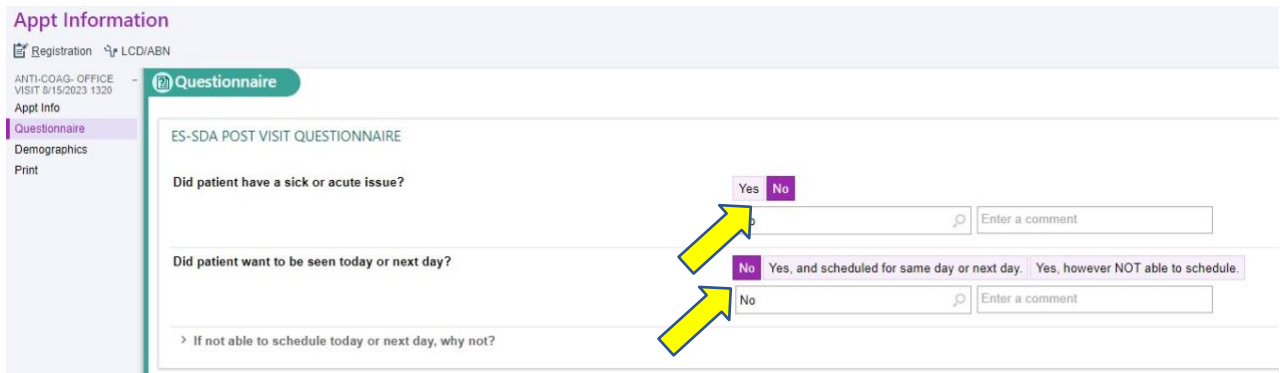
Locations: Center, **Service Area**, Group
BAYLOR SCOTT & WHITE HEALTH

Providers: All, Female, Male

Language: [Search]

Wait List, **Schedule**, Cancel

H. Travel Screening/Post Visit questions, Answer then click **Accept** in the bottom right hand corner



Appt Information

Registration LCD/ABN

ANTI-COAG- OFFICE VISIT 8/15/2023 1320

Appt Info

Questionnaire

Demographics

Print

ES-SDA POST VISIT QUESTIONNAIRE

Did patient have a sick or acute issue?

Yes No

Enter a comment

Did patient want to be seen today or next day?

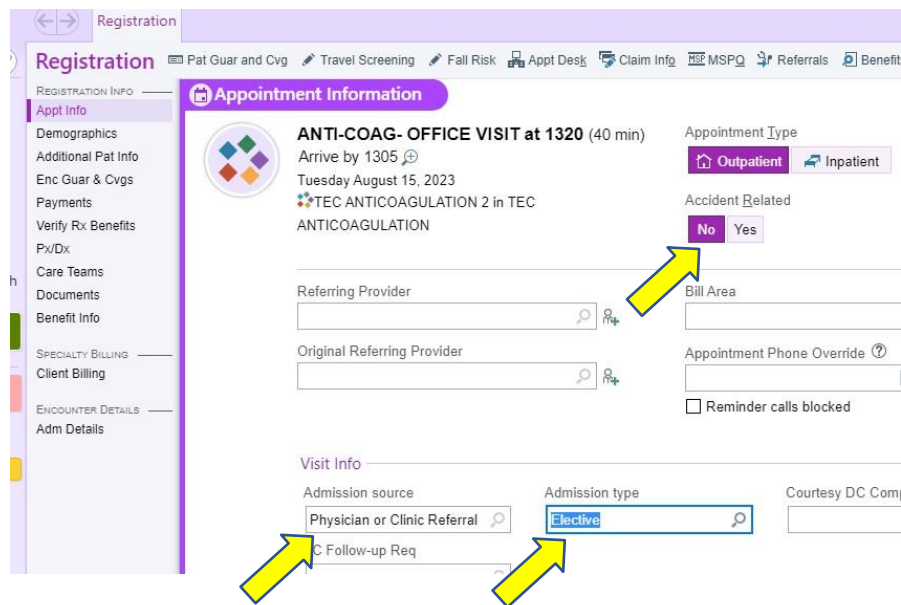
No Yes, and scheduled for same day or next day. Yes, however NOT able to schedule.

Enter a comment

> If not able to schedule today or next day, why not?

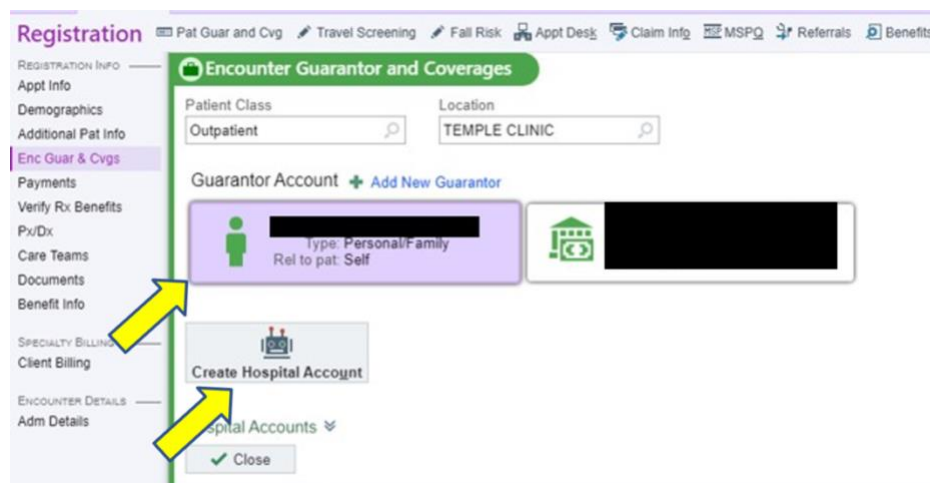
I. Appointment Information:

- Accident Related: **No**
- Admission source: **Physician or clinic Referral**
- Admission type: **Elective**



J. Encounter Guarantor and Coverages:

- Select **Personal/Family** Insurance
- Click **Create Hospital Account**
- Click **finish** in the bottom right hand and **continue anyways**.

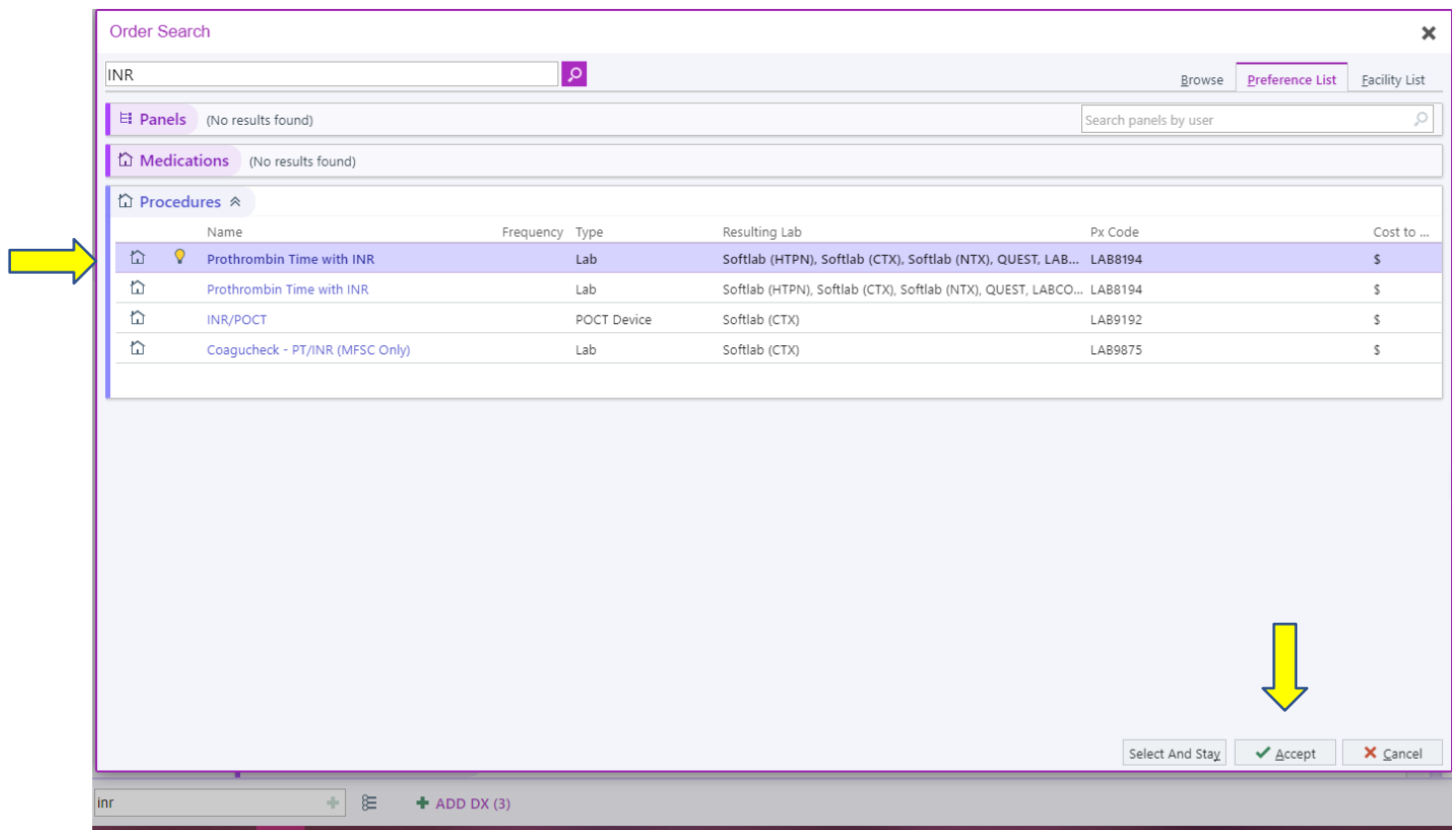


SCHEDULING LAB APPOINTMENT

Reference: Lab Abbreviations PDF

- A. Click **ADD ORDER** and then select appropriate lab order: Prothrombin Time with INR, CBC, CMP, and click **Accept in** the bottom right corner.





Order Search

INR

Browse Preference List Facility List

Panels (No results found) Search panels by user

Medications (No results found)

Procedures

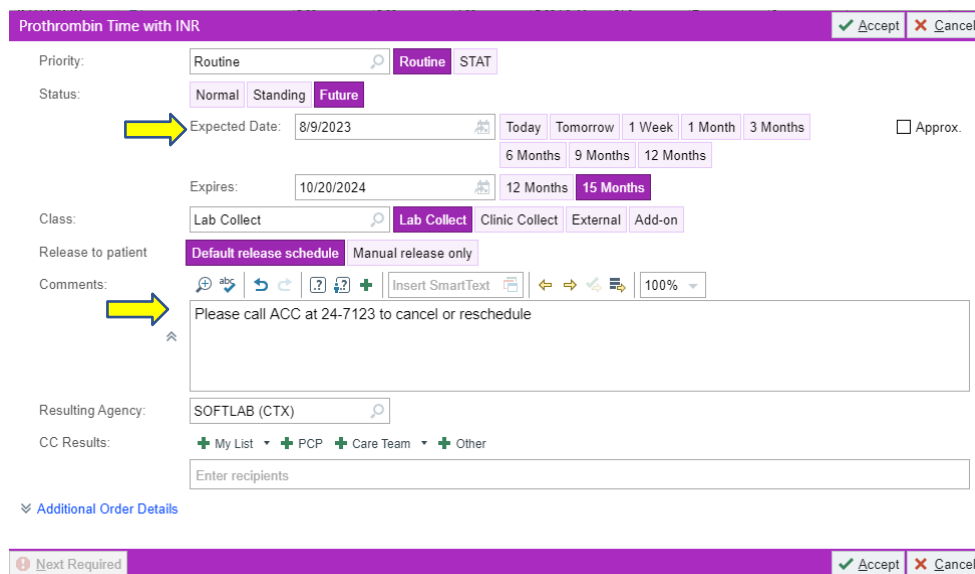
Name	Frequency	Type	Resulting Lab	Px Code	Cost to ...
Prothrombin Time with INR		Lab	Softlab (HTPN), Softlab (CTX), Softlab (NTX), QUEST, LAB...	LAB8194	\$
Prothrombin Time with INR		Lab	Softlab (HTPN), Softlab (CTX), Softlab (NTX), QUEST, LABCO...	LAB8194	\$
INR/POCT		POCT Device	Softlab (CTX)	LAB9192	\$
Coaguchek - PT/INR (MFSC Only)		Lab	Softlab (CTX)	LAB9875	\$

Select And Stay Accept Cancel

inr + ADD DX (3)

K. Fill in required fields: Note and comments and sign the order

- Comments: **Please call ACC at 24-7123 to cancel or reschedule**
- Choose date for lab draw.



Prothrombin Time with INR Accept Cancel

Priority: Routine Routine STAT

Status: Normal Standing Future

Expected Date: 8/9/2023 Today Tomorrow 1 Week 1 Month 3 Months 6 Months 9 Months 12 Months 15 Months Approx.

Expires: 10/20/2024 12 Months 15 Months

Class: Lab Collect Lab Collect Clinic Collect External Add-on

Release to patient: Default release schedule Manual release only

Comments: Please call ACC at 24-7123 to cancel or reschedule

Resulting Agency: SOFTLAB (CTX)

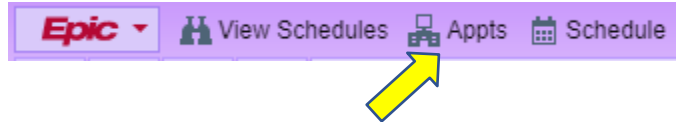
CC Results: My List PCP Care Team Other

Enter recipients

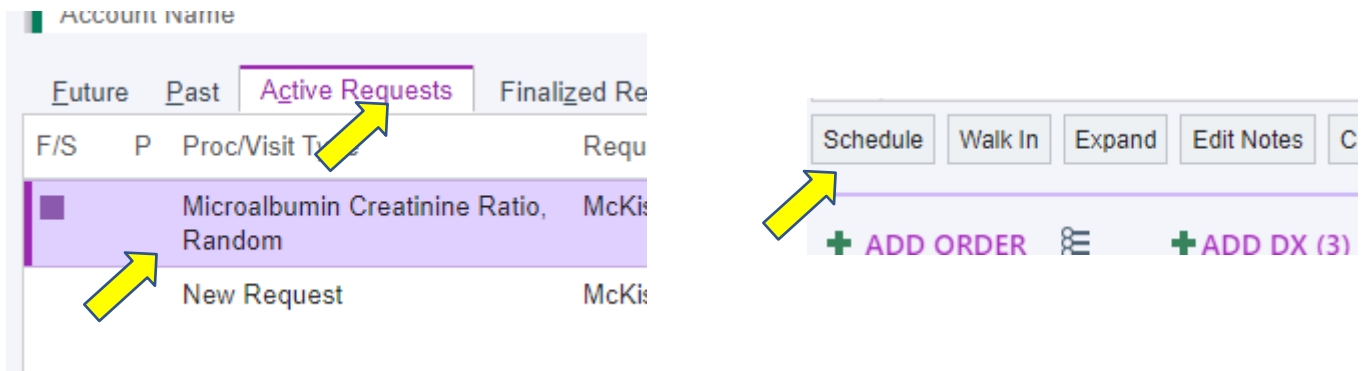
Additional Order Details

Next Required Accept Cancel

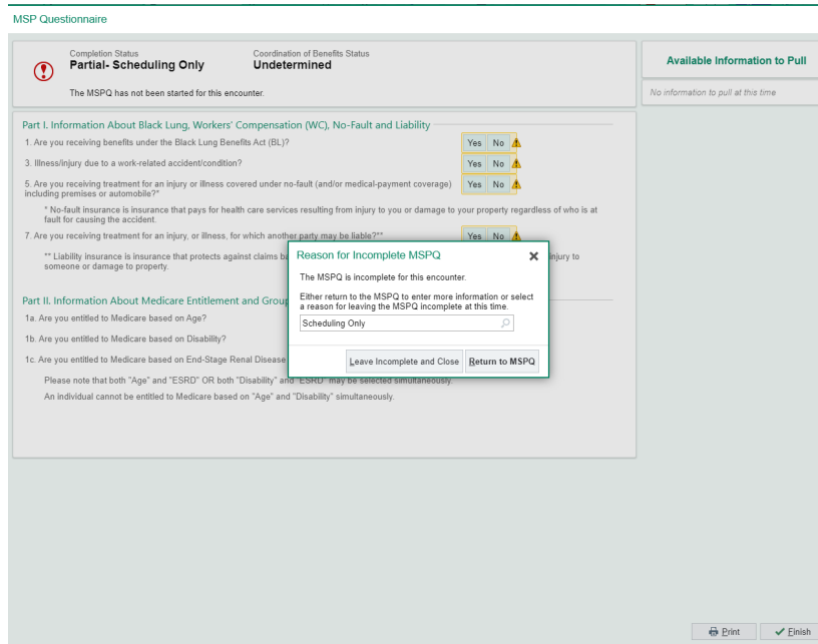
L. Selects Appts in the top left screen.



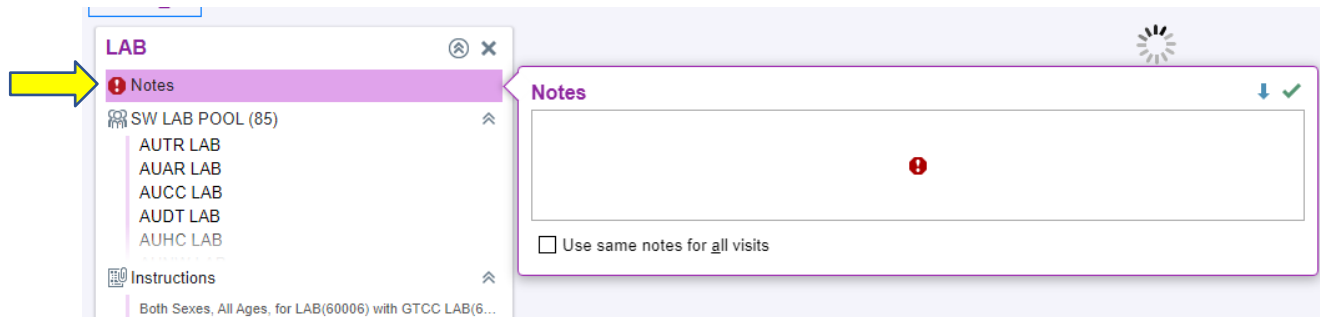
- M. Select **Active Requests** and find the appropriate lab order that needs to be scheduled. If your needing more than one lab hold down control and select multiple labs, when they are highlighted that means they are selected (see highlighted lab below). Select schedule at the bottom of the screen



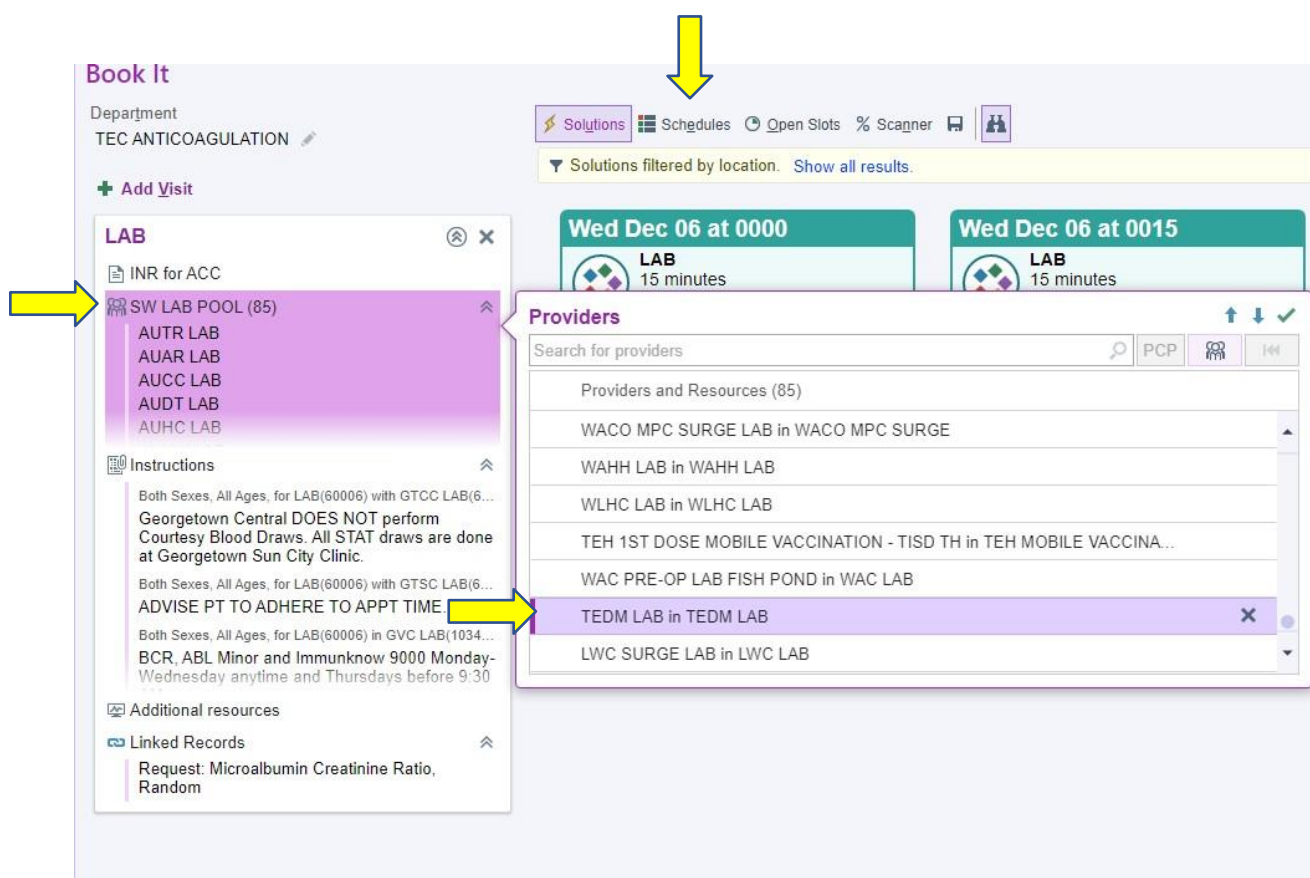
- N. You may get the **MSP Questionnaire**. Just Click **Finish** at the bottom then select **Scheduling Only** and **Leave Incomplete and Close**.



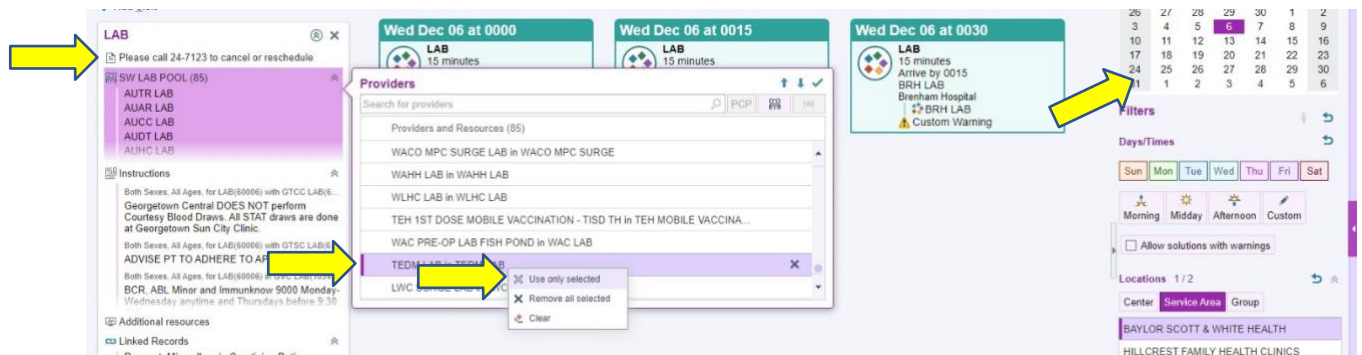
- O. Fill in required fields. Add to notes: **Please call 24-7123 to cancel or reschedule**



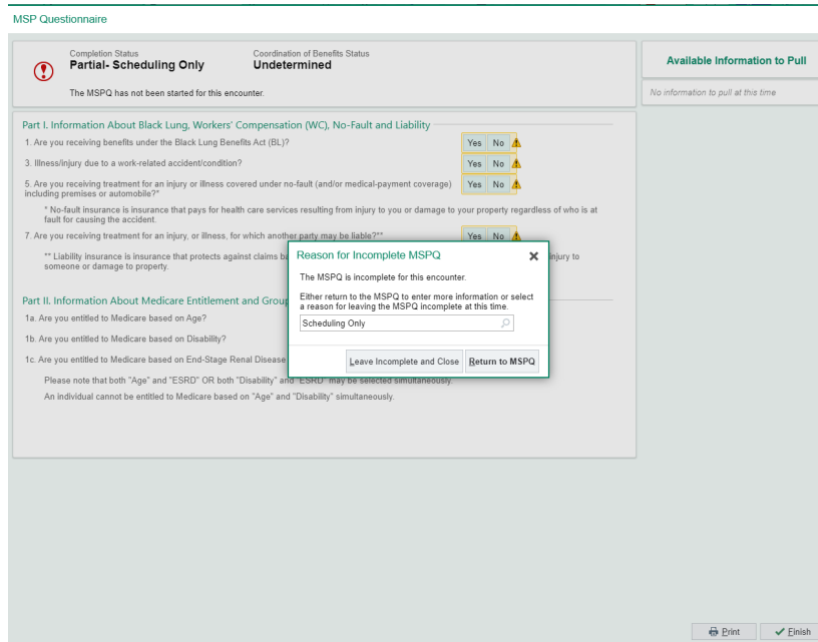
SW LAB POOL: select the lab your are wanting to schedule at (see [Lab Abbreviations PDF](#)) TEDM for CDM lab in Temple selected here, right click and select “use only selected” to see only the selected location (TEDM), click schedules to at the top to see times available.



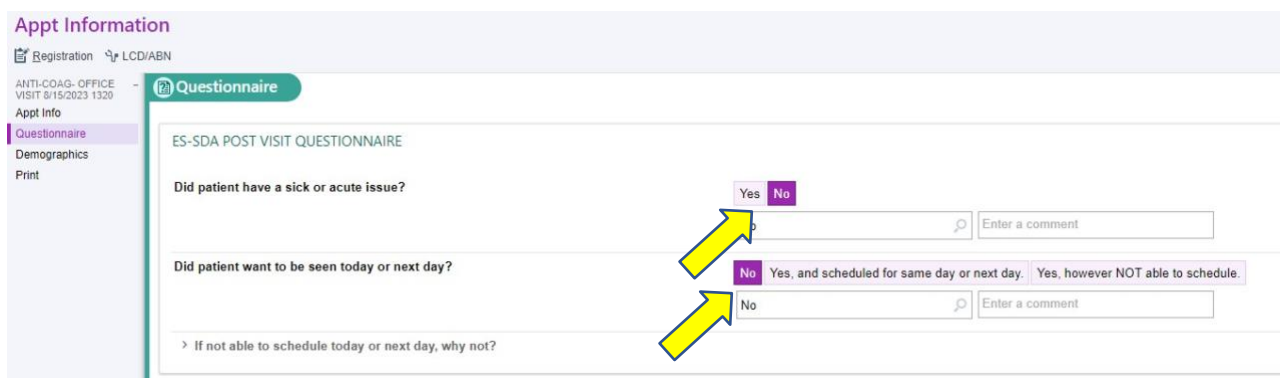
- P. See below- Please call 24-7123 to cancel or reschedule, TEDM lab on 12/6/23. Make sure you have the note, correct lab location, and correct date, and choose the time needed.



Q. You may get the **MSP Questionnaire**. Just Click **Finish** at the bottom then select **Scheduling Only** and **Leave Incomplete and Close**.



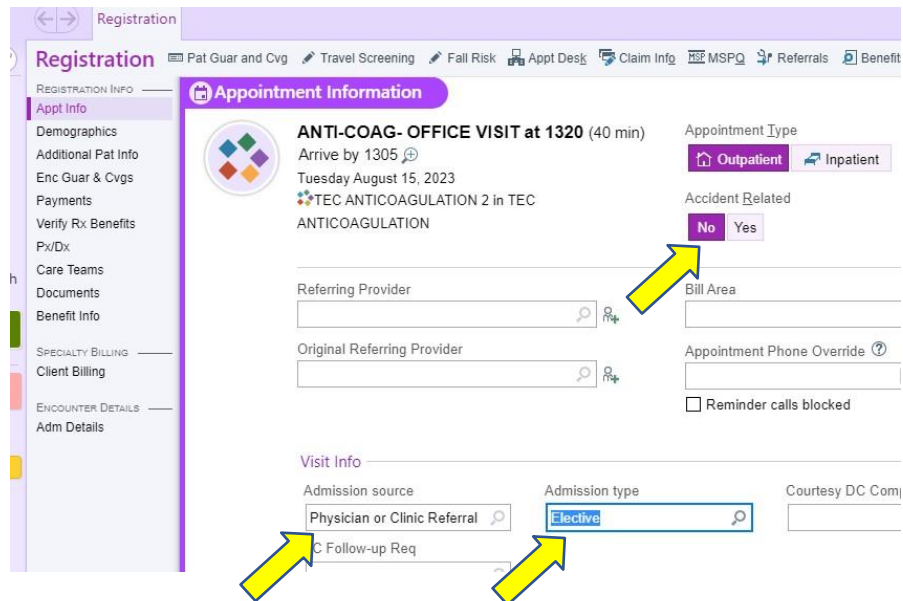
R. Travel Screening/Post Visit questions, Answer then click **Accept** in the bottom right hand corner



S. Appointment Information:

- Accident Related: **No**
- Admission source: **Physician or clinic Referral**

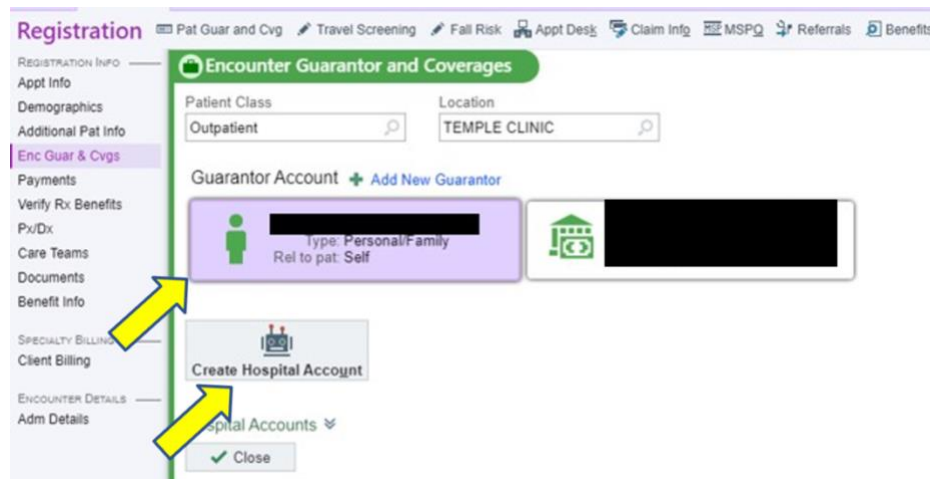
Admission type: **Elective**



T. Encounter Guarantor and Coverages:

- Select **Personal/Family** Insurance
- Click **Create Hospital Account**

U. Click **finish** in the bottom right hand and **continue anyways**.



VII. NEW PATIENT VISITS

- New patients will be scheduled with Tiffany Brown, FNP or appropriate mid-level provider in Temple.
- Initial enrollment/education visit must be in on-site in Temple.
- Patient Visit Process:

- a. Patient checks in at VASICEK CANCER for ACC appointment and receives a New Patient Intake form to begin filling out as well as an Anticoagulation Clinic Patient Agreement Form. After the patient checks in, he/she will have an “arrived” status next to their appointment in EPIC.
 - b. Call patient name and escort patient to room
 - c. Go through Intake form and conduct visit. Review with patient the introductory anticoagulation clinic letter and review clinic procedures with the patient. Provide patient education. Provide the patient with their pro-time results before they leave the clinic (can use routing slip or After Visit Summary (AVS) printed from EPIC. Reschedule patient for their next PT/INR as a point of care (POC) visit (some exceptions can be made: live outside 30 mile radius from ACC, have home health, work schedule, other appointments, etc – please ask staffing pharmacist if there are any questions about this). Emphasize at initial visit – face to face visits/POC visits are preferred, pointing out patient safety as the goal; however, we want to be flexible with the patient and their schedule. Please take into account patient needs transportation to the clinic, prior PT/INR monitoring stability, and patient special needs such as deafness and mental status when discussing this with the patient.
 - d. Patient education to include Baylor Scott & White New Patient warfarin or NOAC education packet, (general information about warfarin/NOAC, bleeding/clotting symptoms, dietary considerations if appropriate, drug-drug interactions, stroke education, nosebleed management, GI bleeding risks associated with aspirin use, medical alert/warfarin card). This may need to be provided in more than one visit.
 - e. Set up follow-up visit
 1. Provide patient with warfarin or DOAC dosing instructions.
 2. Schedule next visit or lab appointment. If sending patient to lab or if next INR is to be done with home health assistance and patient will need a phone call follow up rather than a face to face visit, please add patient’s name to ACC “Star List.”
- B. Complete documentation of visit
1. Complete medical record documentation and billing using anticoag navigator in EPIC.
 2. Route your note to your staffing pharmacist for review.
 3. Order appropriate labs for the next day’s new visits.
 4. Route to Donna Valerio to add patient to active list.

VIII. INR - POINT-OF-CARE VISITS

- A. Patient checks in at VASICEK CANCER for ACC for ACC appointment. After the patient checks in, he/she will have an “arrived” status next to their appointment in EPIC.
- B. Call patient name and escort to patient room

C. Conduct visit and obtain INR via point-of-care. (NOTE: If INR ≥ 4.5 , must order venipuncture PT/INR and send patient to lab). Will need to telephone patient later with results and further instructions. Add patient to ACC “Star List” for telephone call with redraw result and dosing.

1. If visit is for DOAC follow up, DO NOT perform point of care INR. Review their labs from the week prior.

D. Set up follow-up visit

1. Provide patient with warfarin or DOAC dosing instructions, filled out with patient’s information, date/time of next visit, and any other special instructions – use routing slip for this.

2. Send patient to PSS check out to schedule next visit.

E. Complete documentation of the visit

1. Complete medical record documentation and billing using anticoag navigator in EPIC.

F. Route your note to your staffing pharmacist for review.

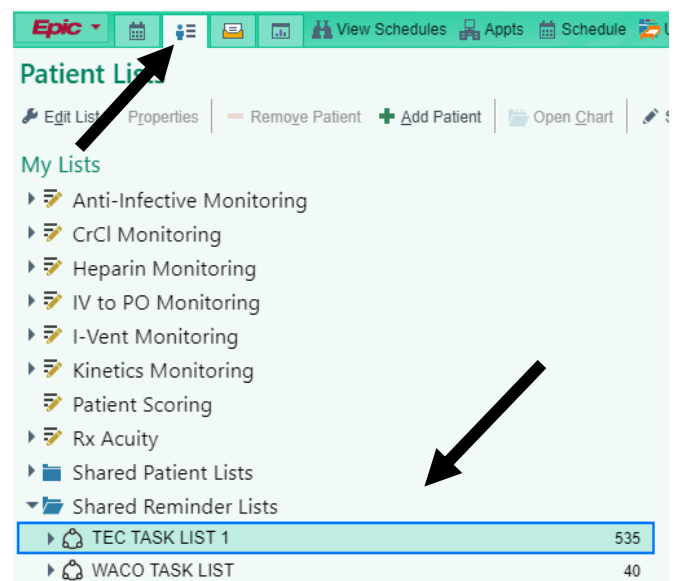
G. Send any Staff Messages through EPIC to triage for follow up on issues: upcoming procedures, other special concerns about patient

H. Complete any refill requests – if you are unable to do so yourself, please route to triage as a staff message with “Refill” as your title.

IX. INR - TELEPHONE VISITS

THE STARLIST

- The list of INR calls that need to be done each day is called the “STAR” list.
- The “STAR” list contains patient with results from lab, self-testing, and home health.
- Click on the icon at the top left to get to **Patient Lists**. The list can be found under **Patient Lists** → **Shared Reminder Lists** → **TEC TASK LIST 1** for Temple and **WAC TASK LIST** for Waco.



- Each task that needs to be complete on current day will be in red.

Patient Lists

[Edit List](#)
[Properties](#)
[Remove Patient](#)
[Add Patient](#)
[Open Chart](#)
[Sign In](#)
[Sign Out](#)

My Lists

- IV to PO Monitoring
- I-Vent Monitoring
- Kinetics Monitoring
- Patient Scoring
- Rx Acuity
- Shared Patient Lists
- Shared Reminder Lists
- TEC TASK LIST 1** 535

TEC TASK LIST 1 535 Patients

Refreshed just now

Reminders	Most Recent INR	Patient name and MRN	Age/Gender	Indications	INR Goal	Next INR Date	Specialty Comments
07/02/2024 - 1D Lab Due 07/01/2024	2.5 at 07/01/24 0000	Test, Testone 15889198	124 y.o. / F	Long term (current) use of anticoagulants [Z79.01] Atrial fibrillation	2.0-...	07/01/2024	

- E. To claim your task, double click on the highlighted task to open it. Click on the **subject** to edit it.

TEC TASK LIST 1 - Edit Reminders

Test, Testone
124 y.o. / F

[+ Add Reminder](#)

Due	Subject	Comments	Done
07/02/2024	1D Lab Due 07/01/2024 2.0		☐

- F. Place your initials at the end of the subject so other people know you are working on it. Then click **Accept** on the subject change AND click **Accept** on the task (bottom right). In the comments section you can add important notes (procedures/specific requests/time to call, etc).

TEC TASK LIST 1 - Edit Reminders

Test, Testone
124 y.o. / F

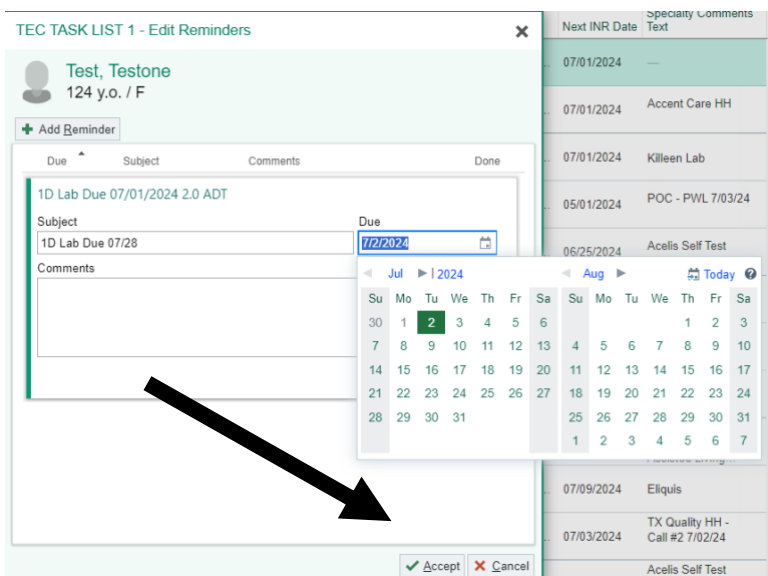
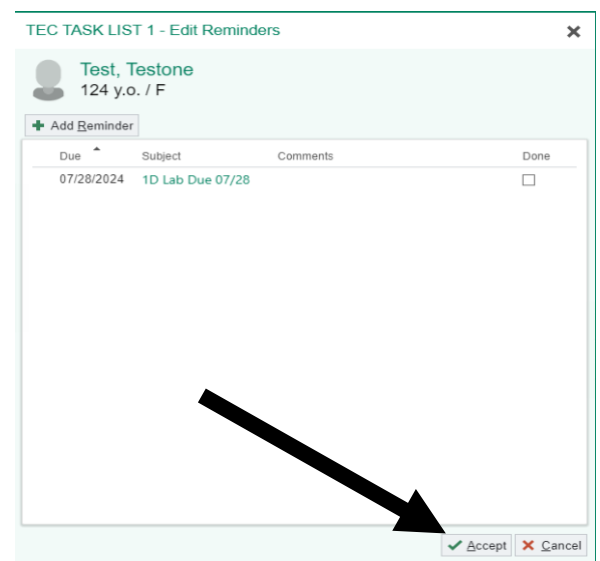
[+ Add Reminder](#)

Due	Subject	Comments	Done
1D Lab Due 07/01/2024 2.0	1D Lab Due 07/01/2024 2.0 ADT		☐

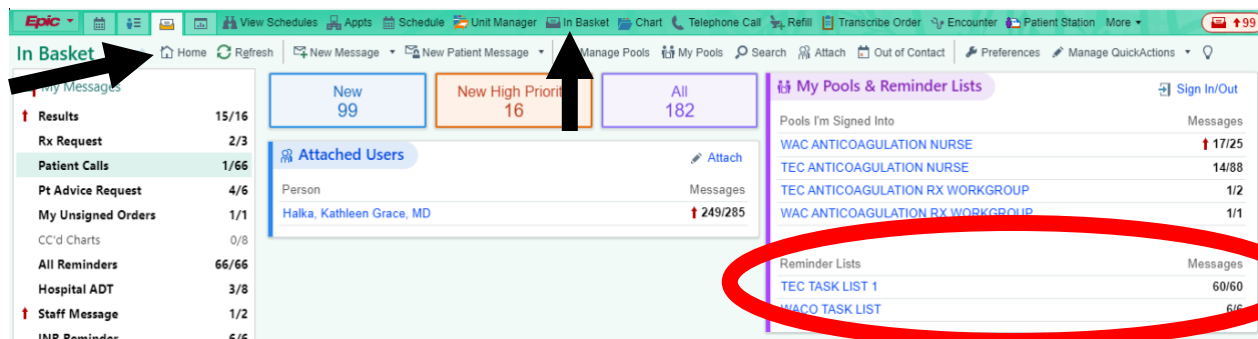
- G. The task will then reflect who is working on it. Make sure to complete or follow-up with the tasks you claim.

TEC TASK LIST 1 535 Patients			
Reminders	Most Recent INR	Patient name and MRN	Age
07/02/2024 - 1D Lab Due 07/01/2024 2.0 ADT	2.5 at 07/01/24 0000	Test, Testone 15889198	124

- H. When you are done, update the task subject to the next due date and move it to applicable date by clicking on the calendar to the right. Click **Accept** when complete and the task will move to the appropriate date. Click **Accept** again to finish.
- I. You only check **DONE** on the right if you are removing that task permanently and there is no longer a need to follow-up using the STAR list (i.e. critical lab re-draw or patient switching to point-of-care or discharge, etc). Otherwise just move the task.

- J. To see the amount of remaining tasks on the list, click on **In Basket** at top → Home → Then view under **My Pools & Reminder Lists** → **Reminder Lists**.



TELEPHONE ENCOUNTERS

- A. Once you find a patient with a result and claim them, you can create a telephone encounter.
- B. To begin you can click **Open Chart** on the top left or simply double click on the patient's name.

Patient Lists

Edit List • Properties • Remove Patient • Add Patient • **Open Chart** • Sign In • Sign Out

My Lists

- IV to PO Monitoring
- I-Vent Monitoring
- Kinetics Monitoring
- Patient Scoring
- Rx Acuity
- Shared Patient Lists
- Shared Reminder Lists

TEC TASK LIST 1 535 Patients

Refreshed just now Search All My Lists

Reminders	Most Recent INR	Patient name and MRN	Age/Gender	Indications	INR Goal	Next INR Date	Specialty Comments
07/02/2024 - 1D Lab Due 07/01/2024	2.5 at 07/01/24 0000	Test, Testone 15889198	124 y.o. / F	Long term (current) use of anticoagulants [Z79.01], Atrial fibrillatio...	2.0-...	07/01/2024	---

C. When in the patient's chart, click **Telephone Call** on top.

Epic Test, Testone View Schedules Appts Schedule Unit Manager In Basket **Chart** **Telephone Call** Refill Transcribe Order More

SnapShot **Chart Review** Episodes of Care MAR Communications

Chart Review

Encounters Notes Labs Imaging Procedures Surgeries Card Proc LDAs Other Orders Meds Episodes Letters Referrals Misc Reports

Preview Refresh (4:41 PM) Select All Deselect All Review Selected Side-by-Side Flowsheet Route Route Face Sheet Load Remaining Encounter Add to

Filters Default filter Me Anticoagulation Baylor Scott & White... Admissions

When	Type	With	Description
07/01/2024	Travel		

D. In the new encounter under the **Call Intake** tab, click on **Reason for Call**.

Chart Review SnapShot Episodes of Care **Call Intake** Communications MAR

7/28/2024 visit with Tran, Andrew, PharmD for Telephone

Switch Family Member Questionnaires References Open Orders Dosage Table Appts Care Teams OB Triage Anticoagulation

TELEPHONE/REFILL ENCOUNTER CCM Patient Enrollment **Reason for Call** Allergies Remote Vitals OB/Gyn Status Medication Review

Verify Rx Benefits Outside Meds Meds & Orders Routing COVID-19 Screen

Chronic Care Management Patient Enrollment

No advisories to address.

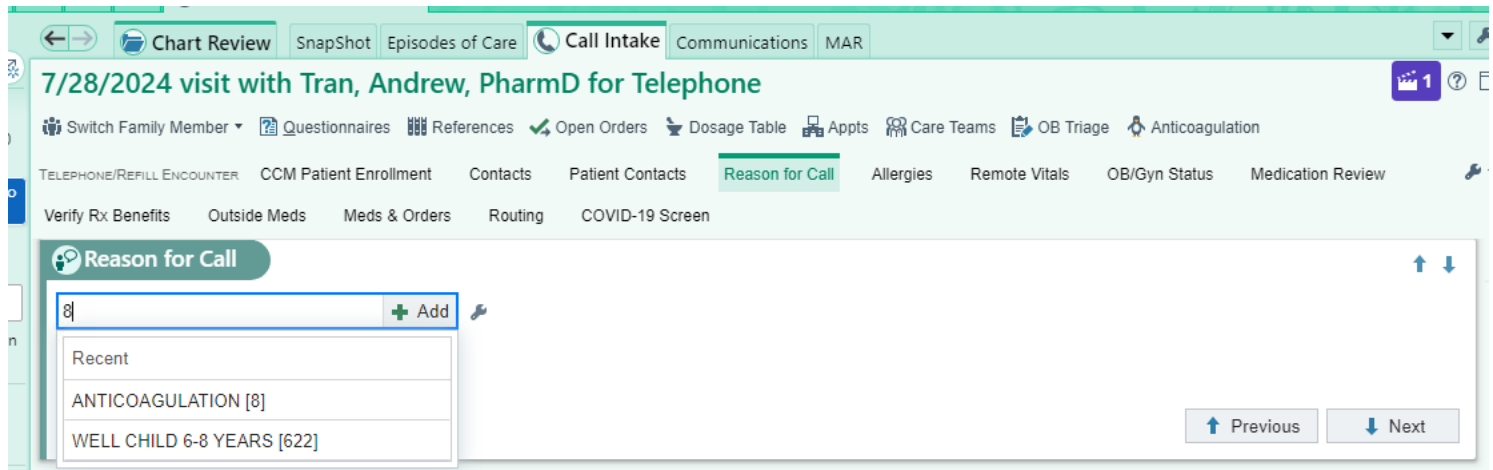
Contacts

+ Incoming Call + Outgoing Call + Other

No contacts

You can use the button to the upper left to add an item to the list.

E. For **Reason for Call**, Type in 8. That will pull up the **ANTICOAGULATION** category to label the encounter.



- F. In comments, you can label the telephone encounter with informatin such as INR LAB RESULT or INR RESULT (LMCB) if you had to leave a message (LMCB = left message to call back). This will show in **Encounter** under Chart Review.

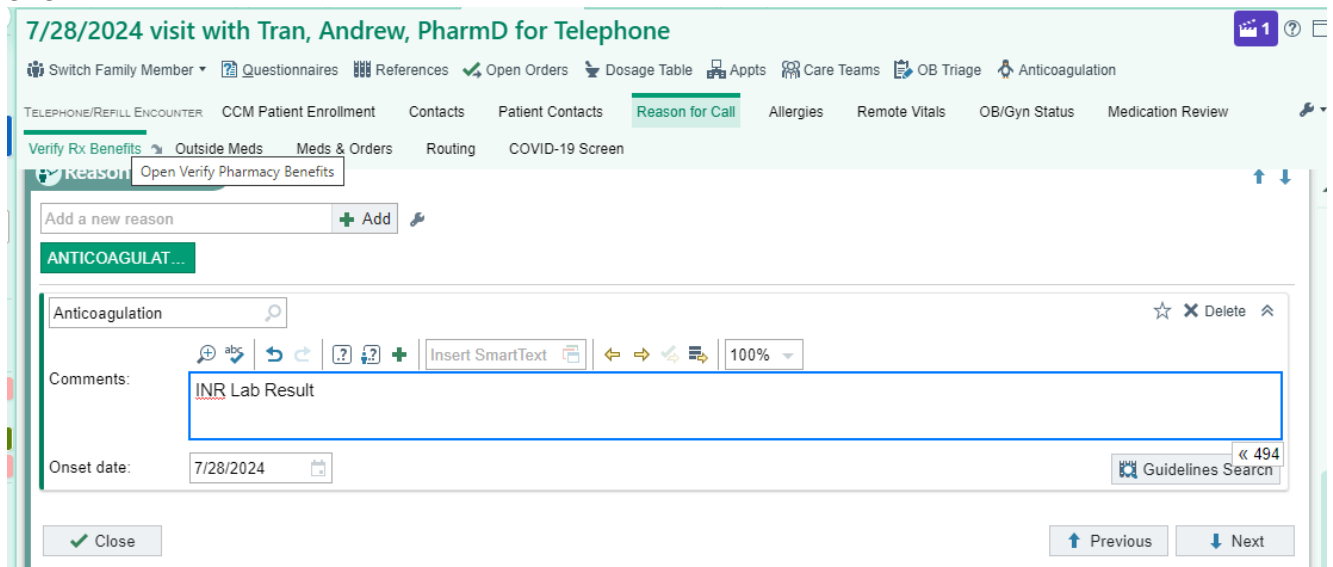


Chart Review

Encounters | Notes | Labs | Imaging | Procedures | Surgeries | Card Proc | LDAs | Other Orders | Meds | Episod

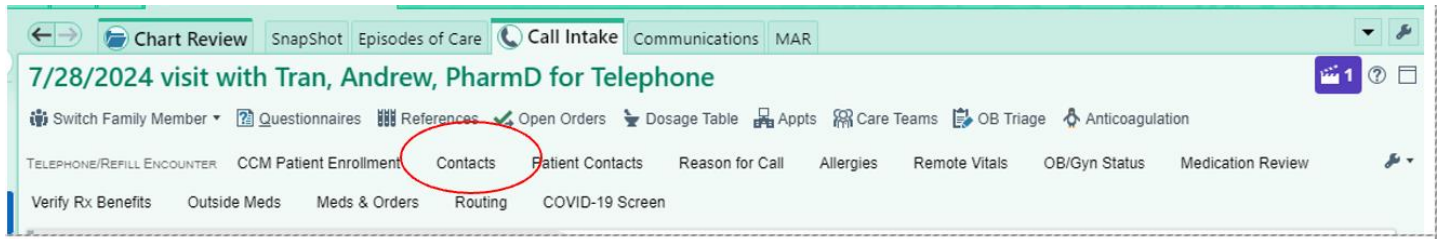
☐ Preview ☒ Refresh (4:53 PM) ☐ Select All ☐ Deselect All ☐ Review Selected ☐ Side-by-Side ☐ Flowsheet ☐ Route ☐ Route Face

Filters ☒ Default filter ☐ Me ☐ Anticoagulation ☐ Baylor Scott & White... ☐ Admissions

When	Type	With	Description
Today	Telephone	Me	Anticoagulation (INR Lab Result)

Recent Visits

- G. To find the patient's phone numbers, click on **Contacts**.



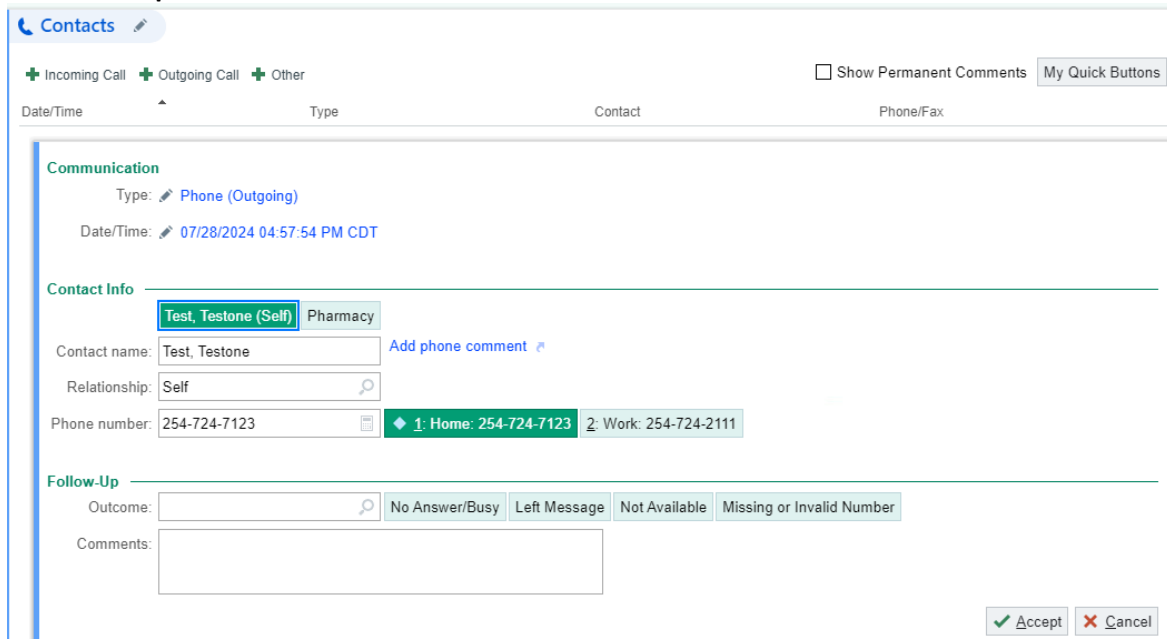
7/28/2024 visit with Tran, Andrew, PharmD for Telephone

Switch Family Member Questionnaires References Open Orders Dosage Table Appts Care Teams OB Triage Anticoagulation

TELEPHONE/REFILL ENCOUNTER CCM Patient Enrollment **Contacts** Patient Contacts Reason for Call Allergies Remote Vitals OB/Gyn Status Medication Review

Verify Rx Benefits Outside Meds Meds & Orders Routing COVID-19 Screen

- H. Once in **Contacts**, click on **Outgoing Call** if you are dialing out or **Incoming Call** if you're being a transferred a call from clerical staff.
- I. Click on who you are calling. The preferred number is marked with the diamond. Select the number you are dialing so it can be documented. If you have to leave a message or patient is unable to answer, click the appropriate reason under **Follow-Up**.



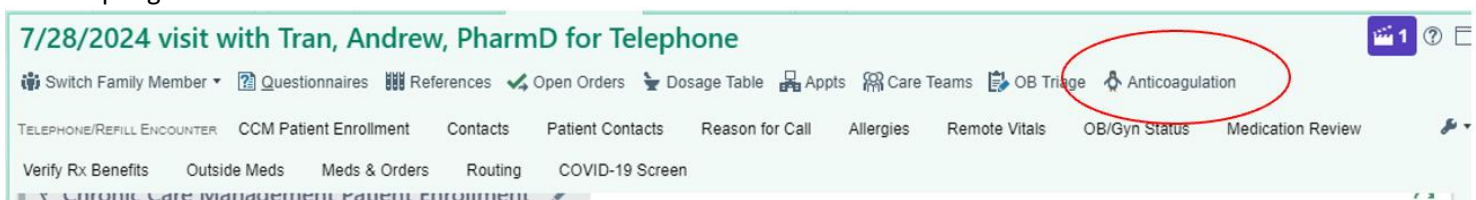
Contacts

+ Incoming Call + Outgoing Call + Other

☐ Show Permanent Comments My Quick Buttons

Date/Time	Type	Contact	Phone/Fax
Communication Type: Phone (Outgoing) Date/Time: 07/28/2024 04:57:54 PM CDT Contact Info Test, Testone (Self) Pharmacy Contact name: Test, Testone Add phone comment Relationship: Self Phone number: 254-724-7123 1: Home: 254-724-7123 2: Work: 254-724-2111 Follow-Up Outcome: No Answer/Busy Left Message Not Available Missing or Invalid Number Comments: Accept Cancel			

- J. Once the patient answer, you can open an Anticoagulation Encounter. Click up top on **Anticoagulation** with the penguin.



7/28/2024 visit with Tran, Andrew, PharmD for Telephone

Switch Family Member Questionnaires References Open Orders Dosage Table Appts Care Teams OB Triage **Anticoagulation**

TELEPHONE/REFILL ENCOUNTER CCM Patient Enrollment Contacts Patient Contacts Reason for Call Allergies Remote Vitals OB/Gyn Status Medication Review

Verify Rx Benefits Outside Meds Meds & Orders Routing COVID-19 Screen

- K. Once in **the Anticoagulation** tab, you will document your notes in the **Anticoag Track**. If prompted, go ahead and the link the anticoagulation episode and add the applicable related problems.

7/28/2024 visit with Tran, Andrew, PharmD for Telephone

Chart Review | SnapShot | Episodes of Care | Call Intake | **Anticoagulation** | Communications | MAR

Contacts

+ Incoming Call + Outgoing Call + Other ☐ Show Permanent Comments My Quick Buttons

Date/Time	Type	Contact	Phone/Fax
07/28/2024 04:54 PM CDT by Tran, Andrew, PharmD	Incoming	—	—
07/28/2024 04:57 PM CDT by Tran, Andrew, PharmD	Outgoing	Test, Testone (Self)	254-724-7123 (Home)

Episodes

+ New Episode Show: ☐ Resolved ☐ Deleted

Linked	Name	Type	Noted	Resolved
	ANTICOAGULATION	ANTICOAGULATION MONITORING	7/1/2024	☒ Resolve
	Warfarin Start 07/	ANTICOAGULATION	0-3.0	

Vitals

+ New Set of Vitals

7/28/2024 visit with Tran, Andrew, PharmD for Telephone

ROOMING | Contacts | Episodes | Vitals | Allergies | Medication Review | Verify Rx Benefits | **Warfarin Anticoagulation Tracking**

The patient's anticoagulation episode is not linked to the encounter.
A linked anticoagulation episode is required to document an anticoagulation encounter.
[Link the anticoagulation episode](#)

Episode

Do you want to add the episode's related problems to visit diagnoses?

Long term (current) use of anticoagulants
Atrial fibrillation (MULTI-HCC)

☒ Yes ☐ No

- L. Once in Anticoag Track, there will be the most recent INR available if received from BSWH or you can enter the INR (if from outside source).

Warfarin Anticoagulation Tracking

INR Goal: 2.0-3.0

For dosing: No INR

Warfarin Dosing Instructions ☐ None given ☐ No change

Tablets on hand: 5 mg &

Maintenance plan: 5 mg every day

Planned weekly total: 35 mg

Last 7 day total: 35 mg

Next 7 day total: 35 mg

Warfarin Anticoagulation Tracking

INR Goal: 2.0-3.0

For dosing: 3.6 (7/26/24)

Warfarin Dosing Instructions ☐ None given ☐ No change

Tablets on hand: 2.5 mg & 5 mg

Maintenance plan: 5 mg every Sat; 7.5 mg all other days

Planned weekly total: 50 mg

Last 7 day total: 50 mg

Next 7 day total: 42.5 mg

- M. To enter an INR, click **New External**. Type in your result and collection information then click **Accept**.

External INR Entry

INR

INR value: 2.5

Reference Range: 0.9 - 1.1

Flag: Abnormal

INR comment:

Collection

Collected: 7/28/2024

Resulted: 7/28/2024

☐ No collection information available

Lab:

Lab description:

Providers & Results Messaging

Authorizing provider: PROVIDER, HISTORICAL

Result message: ☐ Send results message

Accept

Cancel

- N. Once available, click on the result you are addressing. You can document your phone call with the patient on the description below and make sure to fill out the date of the next inr.
- O. The dosing calendar on the right is where you can document any discrepancies in dosing such as patient taking extra or missing doses. Click on the day you want to adjust and you will be prompted for what the dose was.
- P. \

Warfarin Anticoagulation Tracking

INR

Goal: 2.0-3.0

For dosing: 2.5 (7/28/24)

No INR

New POCT

New External

Warfarin Dosing Instructions

☐ None given ☐ No change

Tablets on hand: 5 mg &

Maintenance plan: 5 mg every day

Planned weekly total: 35 mg

Last 7 day total: 30 mg

Next 7 day total: 35 mg

Follow-Up

Priority: Routine (every 7 days) Maintenance (every 28 days)

Date of next INR: 8/4/2024

The INR reminder message will be sent on 8/4/2024.

Description

INR 2.5. Spoke to the patient on the phone. Reports adherence to warfarin 5 mg daily. Denies any missed or change doses. Advised to continue warfarin 5 mg daily. Next INR in 1 week via POC on 08/04/2024 @ Temple Acc. Patient agrees with plan and verbalized understanding.

July 14 - August 17, 2024

Sun	Mon	Tue	Wed	Thu	Fri	Sat	Week Total
14 5 mg 1 x 5 mg	15 5 mg 1 x 5 mg	16 5 mg 1 x 5 mg	17 5 mg 1 x 5 mg	18 5 mg 1 x 5 mg	19 5 mg 1 x 5 mg	20 5 mg 1 x 5 mg	35 mg
21 5 mg 1 x 5 mg	22 Missed	23 5 mg 1 x 5 mg	24 5 mg 1 x 5 mg	25 5 mg 1 x 5 mg	26 5 mg 1 x 5 mg	27 5 mg 1 x 5 mg	30 mg
28 5 mg 1 x 5 mg	29 5 mg 1 x 5 mg	30 5 mg 1 x 5 mg	31 5 mg 1 x 5 mg	Aug 1 5 mg 1 x 5 mg	2 5 mg 1 x 5 mg	3 5 mg 1 x 5 mg	35 mg
					9 5 mg 1 x 5 mg	10 5 mg 1 x 5 mg	35 mg
					16 5 mg 1 x 5 mg	17 5 mg 1 x 5 mg	35 mg

Dose taken: 0 mg

Missed

Deviated from instructed dose

Comments:

Undo

Accept

Cancel

Close

Previous

Next

- Q. In **Pt findings**, you will find the anticoagulation related questions to ask the patient and you will fill that out as you ask the patient. You can click the + or – signs on each question. Double click on a pertinent finding to add a note.
- R. Once you click close, it'll show which findings were positive with your notes.

7/28/2024 visit with Tran, Andrew, PharmD for Telephone

Patient Findings

+ Missed Doses	–	+ Falls	–	+ Aspirin Use	–
+ Extra Doses	–	+ Injury	–	+ Medication Changes	–
+ Antibiotic Use	–	+ Alcohol Use	–	+ Diet Changes	–
+ Bleeding	–	+ Smoking	–	+ Recent Illness or Health Changes	–
+ Any Bruising	–	+ ER Visits	–	+ Dental/Other Procedures	–
+ Signs of Clotting	–	+ Hospitalization	–	+ Herbal Medication Use	–
+ Other Complaints	–	+ Pregnant	–	Otherwise Negative	

Comments

Close Cancel Previous Next

Problem List Care Coordination Note

7/28/2024 visit with Tran, Andrew, PharmD for Telephone

Patient Findings

Missed Doses	–	Falls	–	+ Aspirin Use
Extra Doses	–	Injury	–	Comment for Aspirin Use
Antibiotic Use	–	Alcohol Use	–	81 mg daily
Bleeding	–	Smoking	–	Dental/Other Procedures
Any Bruising	–	ER Visits	–	Herbal Medication Use
Signs of Clotting	–	Hospitalization	–	Otherwise Negative
Other Complaints	–	Pregnant	–	

Comments

Close Cancel Previous Next

Problem List Care Coordination Note

Chart Review Snapshot Episodes of Care Call Intake **Anticoagulation** Communications MAR

7/28/2024 visit with Tran, Andrew, PharmD for Telephone

ROOMING
Contacts
Episodes
Vitals
Allergies
Medication Review
Verify Rx Benefits
Rev Flowsheet

Patient Findings

Positives: **Aspirin Use (81 mg daily)**
Negatives: ER Visits, Bleeding, Signs of clotting, Herbal Medication Use, Hospitalizations, Alcohol Use, Recent Illness or Health Change, Smoking, Pregnant, Injury, Falls, Missed doses, Extra doses, Change in medications, Antibiotic use, Change in diet/appetite, Dental/Other procedures, Bruising, Other complaints

S. Before you finish the encounter, make sure you add a note documentation on the side indicating you called the patient.

Chart Review Snapshot Episodes of Care Call Intake **Anticoagulation** Communications MAR

7/28/2024 visit with Tran, Andrew, PharmD for Telephone

ROOMING
Contacts
Episodes
Vitals
Allergies
Medication Review
Verify Rx Benefits
Rev Flowsheet
History
CHA2DS2 VASc
PT Findings
ANTICOAG ORDERS
Problem List
Documentation
Anticoag Track
Meds & Orders
Error
DISCHARGE
PT Instructions
Follow-up
Sign Encounter

Warfarin Anticoagulation Tracking

INR Goal: 2.0-3.0
For dosing: 2.5 (7/28/24)
No INR
+ New POCT + New External

Warfarin Dosing Instructions
None given No change Restore Dosing
Tablets on hand: 5 mg & 1 x 5 mg
Maintenance plan: 5 mg every day
Edit Plan Do Not Use Plan

Planned weekly total: 35 mg
Last 7 day total: 30 mg
Next 7 day total: 35 mg

Follow-Up
Priority: Routine (every 7 days) Maintenance (every 28 days)
Date of next INR: 8/4/2024
The INR reminder message will be sent on 8/4/2024.

Description
100%
INR 2.5. Spoke to the patient on the phone. Reports adherence to warfarin 5 mg daily. Denies any missed or change doses. Advised to continue warfarin 5 mg daily. Next INR in 1 week via POC on 08/04/2024 @ Temple Acc. Patient agrees with plan and verbalized understanding.

Dose taken: 5 mg 1 x 5 mg Missed
Deviated from instructed dose
Comments:
Undo Accept Cancel

Sun	Mon	Tue	Wed	Thu	Fri	Sat	Week Total
14 5 mg 1 x 5 mg	15 5 mg 1 x 5 mg	16 5 mg 1 x 5 mg	17 5 mg 1 x 5 mg	18 5 mg 1 x 5 mg	19 5 mg 1 x 5 mg	20 5 mg 1 x 5 mg	35 mg
21 5 mg 1 x 5 mg	22 Missed	23 5 mg 1 x 5 mg	24 5 mg 1 x 5 mg	25 5 mg 1 x 5 mg	26 5 mg 1 x 5 mg	27 5 mg 1 x 5 mg	30 mg
28 5 mg 1 x 5 mg	29 5 mg 1 x 5 mg	30 5 mg 1 x 5 mg	31 5 mg 1 x 5 mg	Aug 1 5 mg 1 x 5 mg	2 5 mg 1 x 5 mg	3 5 mg 1 x 5 mg	35 mg
4 5 mg 1 x 5 mg	5 5 mg 1 x 5 mg	6 5 mg 1 x 5 mg	7 5 mg 1 x 5 mg	8 5 mg 1 x 5 mg	9 5 mg 1 x 5 mg	10 5 mg 1 x 5 mg	35 mg
11 5 mg 1 x 5 mg	12 5 mg 1 x 5 mg	13 5 mg 1 x 5 mg	14 5 mg 1 x 5 mg	15 5 mg 1 x 5 mg	16 5 mg 1 x 5 mg	17 5 mg 1 x 5 mg	35 mg

Documentation
Create Note
My Note Signed
4:56 PM
Add Delete Copy
Patient contacted today for phone visit. See Anticoagulation Encounter from today for dosing and information from today's visit.

T. To wrap up the call, on the right click on **Follow-Up** under **DISCHARGE**. Then make sure you click on the **return in** interval and **return on** date.

7/28/2024 visit with Tran, Andrew, PharmD for Telephone

Follow-up

Return in: 4 Weeks 3 Months 6 Months 1 Years
1 Days Weeks Months Years +

Return on: 8/4/2024 ☒ Approximately

☐ PRN

For: ☐ Therapy Plan Ordered

Additional Details

Check-out note: [Rich Text Editor]

Follow-up: [Text Field]

Send Chart Upon Closing Section

+ My List + PCP + Other Remove All

Enter recipients

Comments: [Rich Text Editor]

High Priority Low Priority

Send Now

Previous Next

Restore Close

U. When complete, you can **sign encounter** on the left or bottom right.

7/28/2024 visit with Tran, Andrew, Pha

Sign Encounter

Route
+ My List + PCP Remove

☒ Sign and Route

☒ Close

ROOMING
Contacts
Episodes
Vitals
Allergies
Medication Review
Verify Rx Benefits
Rev Flowsheet
History
CHA2DS2 VASc
Pt Findings

ANTICOAG ORDERS
Problem List
Documentation
Anticoag Track
Meds & Orders
Error

DISCHARGE
Pt. Instructions
Follow-up
Sign Encounter

Sign at Exit WS Accept Cancel

LEVEL OF SERVICE SIGN ENCOUNTER

V. Make sure patient has appropriate follow-up (schedule next lab or write a home health order or tell patient when to self-test next).

ACELIS RESULTS

- Results from self-tester using Acelis can be found on: <https://www.coagclinic.com/>
- Subscriber ID: **SWAC** for the clinics and log in with your user ID and password.

 Your session has expired. Please sign back in below.

SIGN IN

SUBSCRIBER ID *

SWAC

USER ID *

A.Tran

PASSWORD *

..... 

☒ REMEMBER ME

LOGIN

[I forgot my password...](#)

- C. Patients can be searched at the top or you click on **PST Results (s) Pending Review** for the results that have been reported.

CoagClinic™

PATIENT

Advanced

CURRENT PATIENT:

SHORTCUTS ▾

HOME PATIENT ▾ REPORTS ▾ PREFERENCES HELP ▾

Welcome Andrew Tran

The date is: 7/2/2024

2 Patient(s) Scheduled for Today	97 Overdue Patient(s)	4 Follow-up Call Patient(s)	0 eRx Pending Signature	59 eRx Action Required	6 PST Result(s) Pending Review
--	------------------------------------	--	--------------------------------------	-------------------------------------	---

- D. To go into a patient's result to address it, click on the hyperlink by **Submission Date** column.

Submission Date ▾	Patient ▾	INR Test ▾	Office ▾
Sorted by: Date (asc)			
6/27/2024 9:31 AM CDT			Scott and White Temple Anti-Coag Clinic 
6/28/2024 5:54 PM CDT			Scott and White Temple Anti-Coag Clinic ☐ Accept
6/29/2024 10:48 AM CDT			Scott and White Temple Anti-Coag Clinic 
7/1/2024 11:45 AM CDT			Scott and White Temple Anti-Coag Clinic 
7/2/2024 7:30 AM CDT			Scott and White Killeen Anticoagulation Clinic 
7/2/2024 9:23 AM CDT			Scott and White Killeen Anticoagulation Clinic ☐ Accept

- E. Scroll down and you will find the patient's test date and the clinical questions they submitted.

PST Patient Test Date
6/27/2024

PST Patient Submission Date: 6/27/2024 9:31 AM CDT

PST INR
2.7

Origination
Five9

Linked Subscriber
Acellis Connected Health
-CoagClinic™(Active)

Self Test Frequency
 days

PST Questions

1: Have you started, stopped, or made changes to any medications (including antibiotics, prescription, or over the counter products or supplements)?
☐ Yes ☒ No ☐ Not Applicable

2: Have you had any signs or symptoms of bruising or bleeding?
☐ Yes ☒ No ☐ Not Applicable

3: Have you missed any doses or taken extra doses of Warfarin in the last week?
☐ Yes ☒ No ☐ Not Applicable

4: Have you been sick or hospitalized?
☐ Yes ☒ No ☐ Not Applicable

F. You can then perform the telephone encounter as normal.

G. When you are done with the call, you can put the next self-test date and click **Save** to clear it out of the que.

Scheduling

Next Office Appointment

Currently Scheduled Visit for

Change scheduled return visit

Self Test Date

New PST Test Date

☐ Flag patient for followup phone call

Supervising Clinician *
Halka, Kathleen

I have reviewed this patient encounter and all affiliated data and am completing this encounter.

SELF-TESTERS - MDINR & RCS & OTHERS

A. For other self-testers such as MDINR & RCS, we do not have a portal. We receive a fax and clerical staff will then scan in the faxes into **Media**. You can confirm patient's result under **Chart Review** and **Media**.

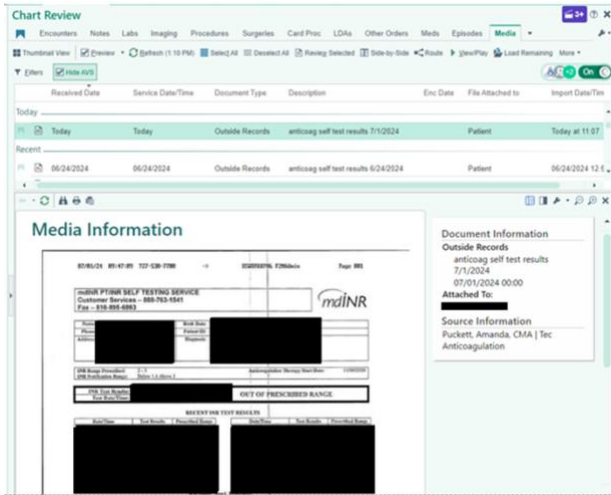
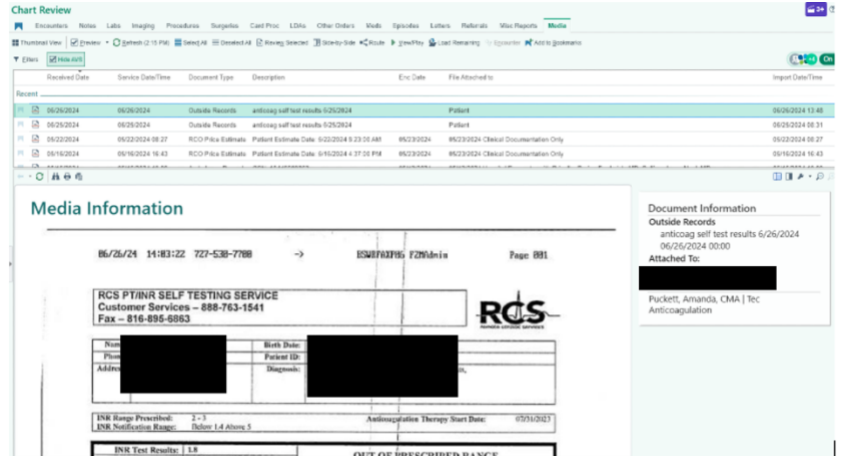
Navigation: SnapShot | **Chart Review** | Episodes of Care | MAR | Communications

Chart Review

Encounters | Notes | Labs | Imaging | Procedures | Surgeries | Card Proc | LDAs | Other Orders | Meds | Episodes | Letters | Referrals | Misc Reports | **Media**

Preview | Refresh (5:38 PM) | Select All | Deselect All | Review Selected | Side-by-Side | Flowsheet | Route | Route Face Sheet | Load Remaining | Encounter | Add to Bookmark

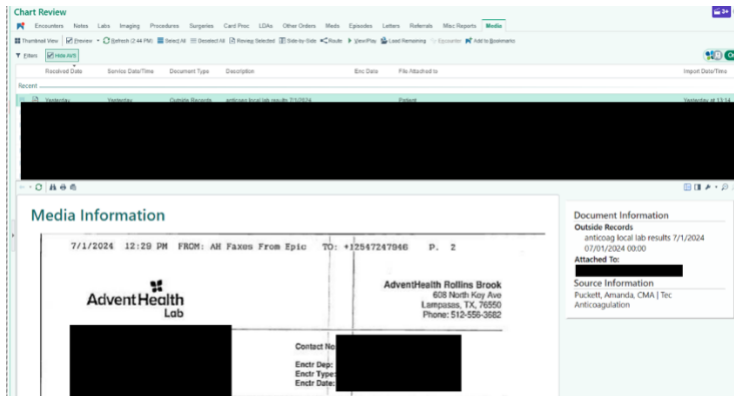
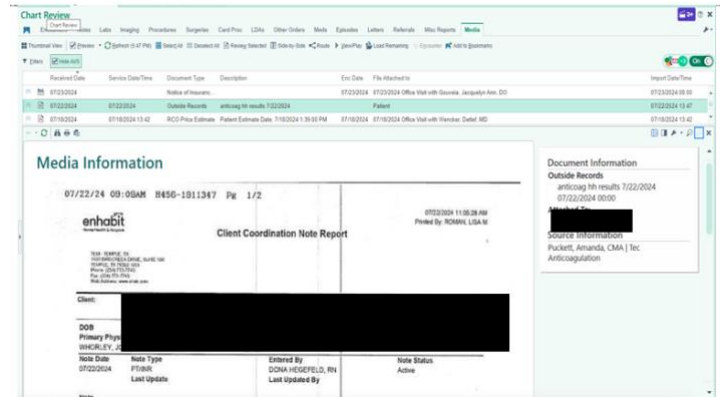
Filters: ☒ Default filter | ☐ Me | ☐ Anticoagulation | ☐ Baylor Scott & White... | ☐ Admissions

B. You enter the result manually in the anticoag track and the rest of the telephone encounter will be the same.

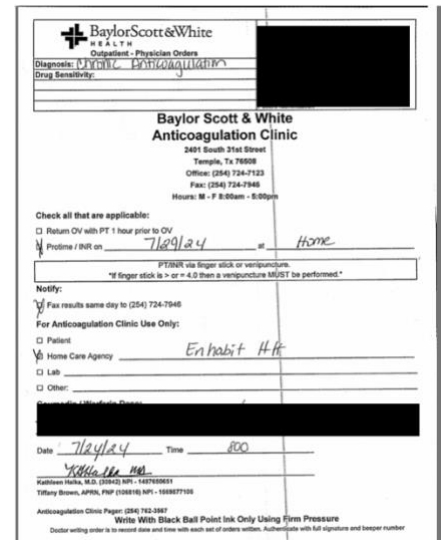
HOME HEALTH AND LOCAL LAB PATIENTS

A. For patients receiving home health or they go to an outside lab, we will usually receive faxes for these results. We receive a fax and clerical staff will then scan in the faxes into **Media**. You can confirm patient's result under **Chart Review** and **Media**.

B. You enter the result manually in the anticoag track and the most of the telephone encounter will be the same.

- C. However on the follow-up, you will need to write out a home health order manually by hand in order to be faxed. For local lab or BSW ref lab patients, you will have to write the lab. For home health patients, you will fill out the correct Home Care Agency.
- D. BSW ref lab patients will need a home health order and a PT/INR order in Epic but there result will populate in Epic.



Baylor Scott & White
HEALTH
Outpatient - Physician Orders
Diagnosis: PT/INR - Anticoagulation
Drug Sensitivity: 3

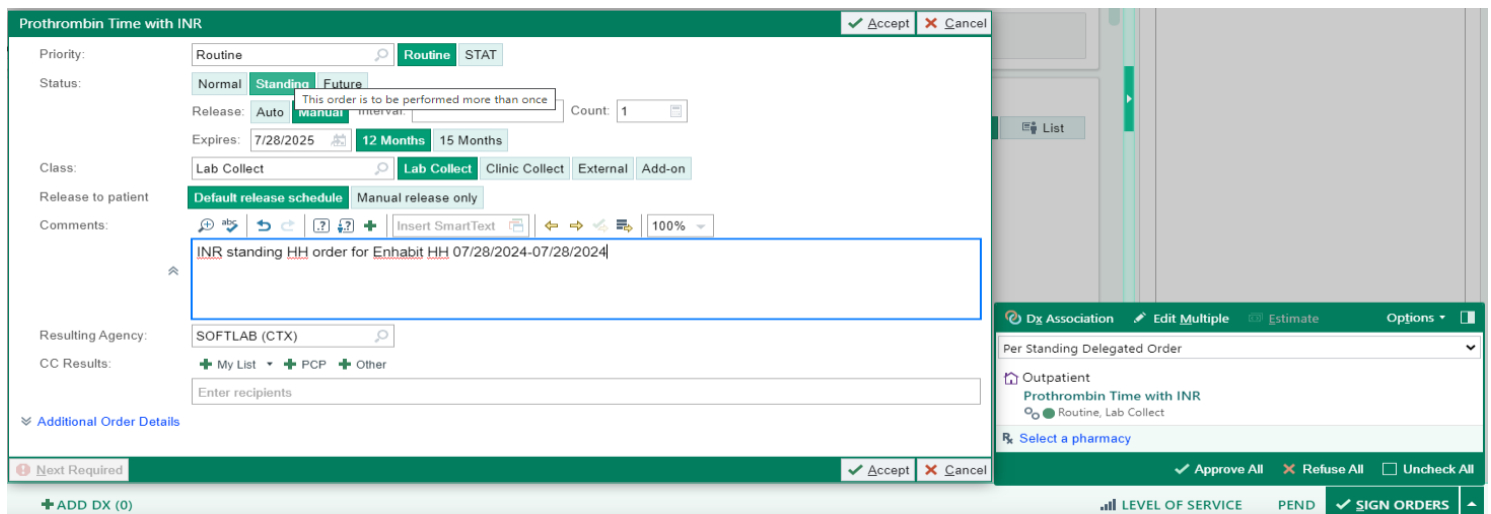
Baylor Scott & White
Anticoagulation Clinic
2401 South 21st Street
Tempe, TX 76069
Office: (254) 724-7123
Fax: (254) 724-7946
Hours: M - F 8:00am - 5:00pm

Check all that are applicable:
☐ Return OIV with PT 1 hour prior to OIV
☒ PT/INR via finger stick or venipuncture
 If finger stick is > or = 4.0 then a venipuncture MUST be performed

Notify:
☒ Fax results same day to (254) 724-7946
 For Anticoagulation Clinic Use Only:
☐ Patient
☒ Home Care Agency Enhabit HH
☐ Lab
☐ Other:

Date: 7/28/24 Time: 800
Karen J. M.
 Address: 1001 N. 10th St., Suite 100, Tempe, AZ 85281
 Tiffany Brown, APRN, NP (200818) NP - 100007100
 Anticoagulation Clinic Paper (254) 724-7946
 Write With Black Ball Point Ink Only Using Firm Pressure
 Doctor writing order is to record date and time with each set of orders written. Authenticate with full signature and keeper number

- E. You will also need a “standing” INR order home health patients, incase the home health needs to draw a venous INR at a BSWH lab.
- F. To do this you add a PT/INR order but instead of **Future** or **Normal**, you set it to **Standing**, put the **Count** as 1, and add a comment as needed.



Prothrombin Time with INR [Accept] [Cancel]

Priority: Routine [Routine] [STAT]

Status: Normal [Standing] [Future]
 Release: Auto [manual] This order is to be performed more than once Count: 1

Expires: 7/28/2025 [12 Months] [15 Months]

Class: Lab Collect [Lab Collect] [Clinic Collect] [External] [Add-on]

Release to patient: Default release schedule [Manual release only]

Comments: [Insert SmartText] [100%]
 INR standing HH order for Enhabit HH 07/28/2024-07/28/2024

Resulting Agency: SOFTLAB (CTX)

CC Results: [My List] [PCP] [Other]

Enter recipients

Additional Order Details

Next Required [Accept] [Cancel]

+ ADD DX (0)

Dx Association [Edit Multiple] [Estimate] [Options]

Per Standing Delegated Order

Outpatient
 Prothrombin Time with INR
 Routine, Lab Collect

Select a pharmacy

Approve All [Refuse All] [Uncheck All]

LEVEL OF SERVICE PEND SIGN ORDERS

- G. Document the date of the standing order in the **Anticoag Track** and the starlist notes for others to refer to.

Follow-Up

Priority: Routine (every 7 days) Maintenance (every 28 days)

Date of next INR: 8/4/2024

The INR reminder message will be sent on 8/4/2024.

Description

100%

INR test. Denies any missed or change doses. Advised to continue warfarin 5 mg daily. Next INR in 1 week via Enhabit HH. Patient agrees with plan and verbalized understanding.

INR standing HH order 07/28/2014

Close

Postcard(PC)/Mychart Message

- A. If patient has INR that is within range and their task has **PC** listed on it, this means they have consented to communications for INR results via mychart msg without a call if there are NO CHANGES.
- B. Open your telephone encounter as normal. CONFIRM results are correct and make sure it is entered correctly.
- C. Document normally as you would for an INR Call except document that patient was sent a mychart msg.

7/2/2024 visit with Tran, Andrew, PharmD for Telephone

ROOMING

Contacts

Episodes

Vitals

Allergies

Medication Review

Verify Rx Benefits

Rev Flowsheet

History

CHA2DS2 VASc

Pt Findings

ANTICOAG ORDERS

Problem List

Documentation

Anticoag Track

Meds & Orders

Error

DISCHARGE

Pt. Instructions

Follow-up

Sign Encounter

Warfarin Anticoagulation Tracking

INR

Goal:

2.0-3.0

For dosing:

2.6 (6/28/24)

No INR

+ New POCT

+ New External

Warfarin Dosing Instructions

☐ None given
 ☐ No change
 [Restore Dosing](#)

Tablets on hand:

7.5 mg

&

Maintenance plan:

11.25 mg every Tue, Thu; 7.5 mg all other days

Edit Plan

Do Not Use Plan

Planned weekly total:

60 mg

Last 7 day total:

60 mg

Next 7 day total:

60 mg

Follow-Up

Priority:

Routine (every 7 days)

Maintenance (every 28 days)

Date of next INR:

7/5/2024

The INR reminder message will be sent on 7/5/2024.

Description

abc

↶

↷

?

+

+

Insert SmartText

↶

↷

↶

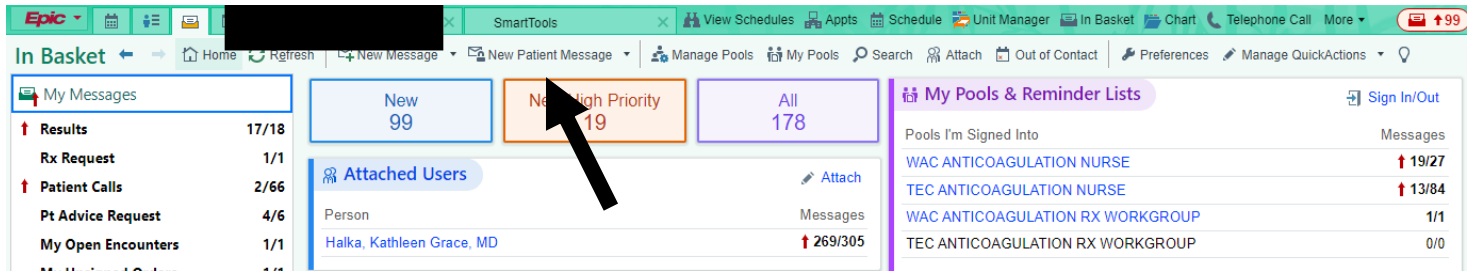
↷

100%

Acelis self test INR= 2.6 LVAD pt, Goal 2.0 - 3.0) Patient was sent a mychart msg today stating INR result, next self test date, and to continue current warfarin dosing. Advised to continue warfarin 11.25 mg on Tues/thurs and 7.5 mg on all other days. Next Self-test in 1 week on 07/05/24

The Anticoagulation Clinic

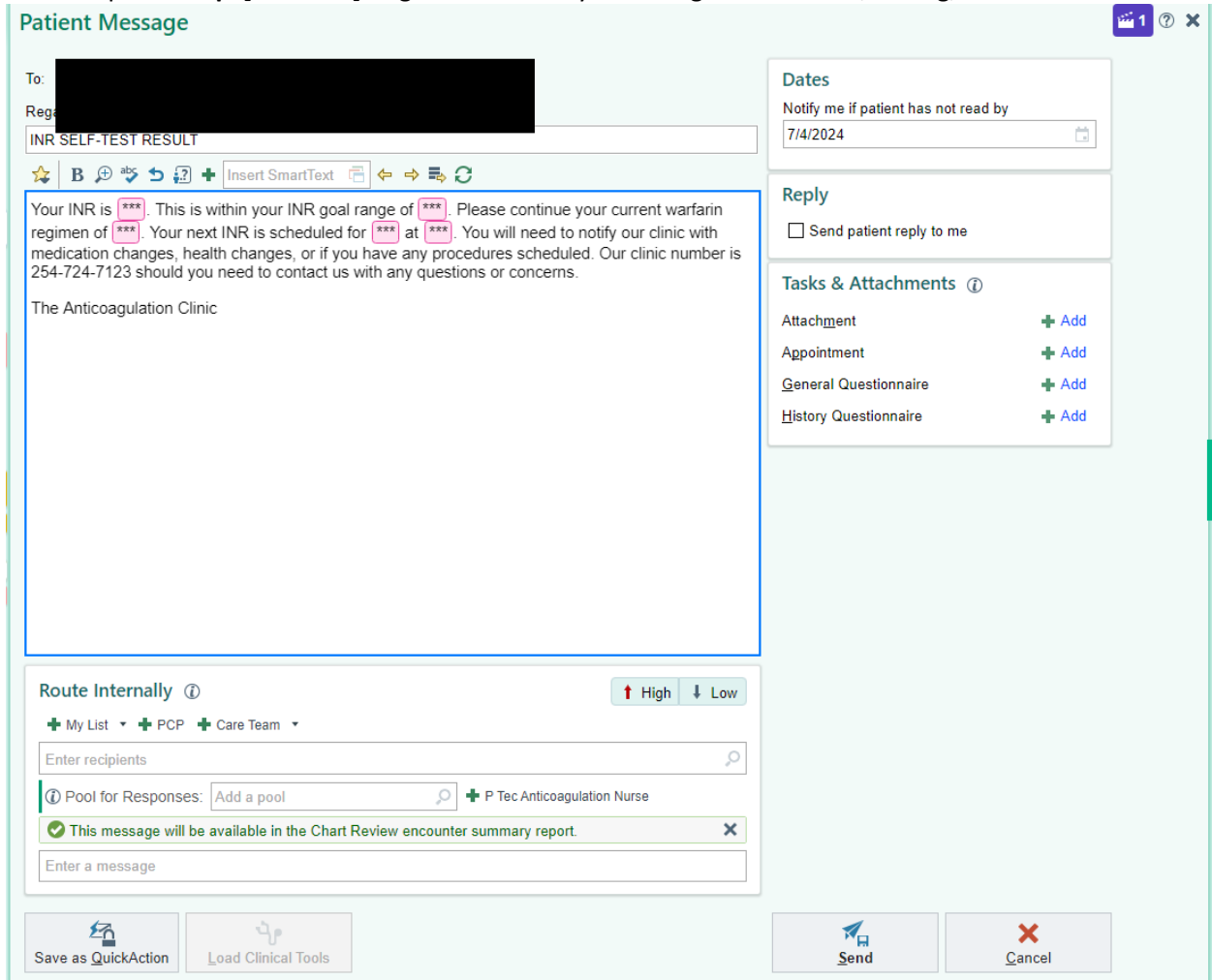
- D. To send patient the mychart msg. Go to **In-Basket** and click on **New Patient Message**.



The screenshot shows the Epic In Basket interface. At the top, there are tabs for SmartTools, View Schedules, Appts, Schedule, Unit Manager, In Basket, Chart, Telephone Call, and More. Below the tabs, there are buttons for New (99), New High Priority (19), and All (178). A black arrow points to the 'New High Priority' button. On the left, there is a list of message types: Results (17/18), Rx Request (1/1), Patient Calls (2/66), Pt Advice Request (4/6), My Open Encounters (1/1), and My Unread Messages (1/1). On the right, there is a section for 'My Pools & Reminder Lists' with a table showing pools and their message counts.

Pools I'm Signed Into	Messages
WAC ANTICOAGULATION NURSE	↑ 19/27
TEC ANTICOAGULATION NURSE	↑ 13/84
WAC ANTICOAGULATION RX WORKGROUP	1/1
TEC ANTICOAGULATION RX WORKGROUP	0/0

E. Use smartphrase .apc[1535480] to generate the mychart msg. Fill in results, dosing, and next INR check. Click **Send**.



The screenshot shows the Epic Patient Message form. The 'To' field is filled with a patient name. The 'Subject' field is 'INR SELF-TEST RESULT'. The message body contains a smartphrase template for an INR self-test result. The template text is: 'Your INR is [***]. This is within your INR goal range of [***]. Please continue your current warfarin regimen of [***]. Your next INR is scheduled for [***] at [***]. You will need to notify our clinic with medication changes, health changes, or if you have any procedures scheduled. Our clinic number is 254-724-7123 should you need to contact us with any questions or concerns. The Anticoagulation Clinic'. The 'Route Internally' section is expanded, showing a list of recipients: 'My List', 'PCP', and 'Care Team'. The 'Pool for Responses' section is also expanded, showing a list of pools: 'Add a pool' and 'P Tec Anticoagulation Nurse'. The 'Send' button is highlighted.

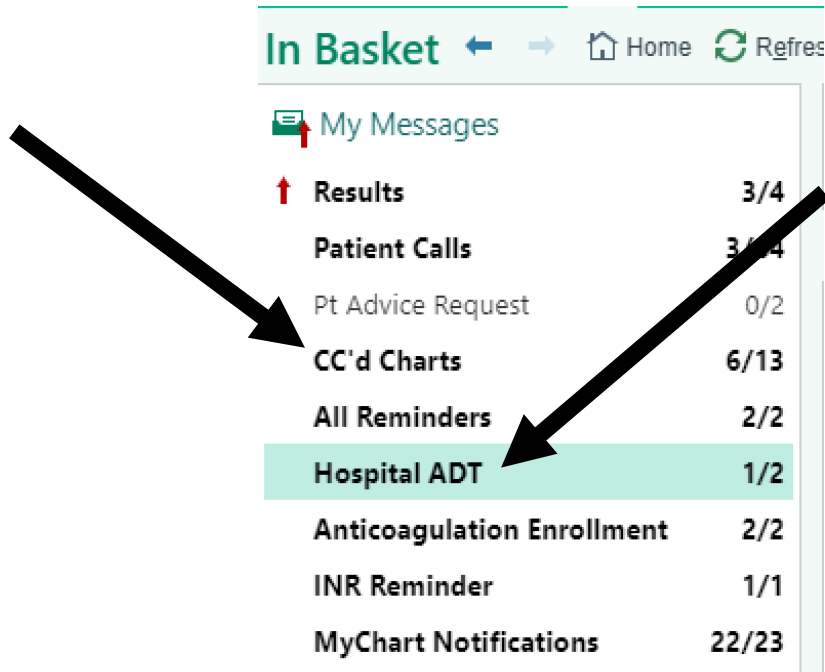
F. Complete the rest of the telephone encounter and update starlist as normal.

X. HOSPITAL DISCHARGE FOLLOW-UP

- The Hospital Discharge responsibility involves addressing ACC patients who have been admitted or discharged from the hospital, emergency department, or day surgery. Pharmacist covering hospital discharge will be responsible for researching hospital admission, ED visit, or surgery respond while addressing any medication changes, lab results, or any other pertinent information that may affect patient's anticoagulation.
- The goal is to ensure ACC patients that are hospitalized have appropriate follow-up and transitions of care. Pharmacist is responsible for following-up with patients the day after discharge and subsequent patients that

have yet to be reached after discharge. Patients will be contacted to confirm changes and scheduled for subsequent follow-up for anticoagulation therapy.

- C. These patients are found in two ques in the in-basket for TEC Anticoagulation Nurse: **Hospital ADT** and **CC'd Charts**.



- D. **Hospital ADT** que is where either admitted or discharged patients will be routed to **P Tec Anticoagulation Nurse**.
- E. **CC'd Charts** is where patients that need to be followed-up by the pharmacist covering hospitals. This may include patients that we were unable to reach upon discharge (additional call attempts), patients that are currently admitted that are awaiting discharge, patients discharged to a skilled nursing facility (SNF) to rehabilitation facility and hospitalizations that may be discovered after the fact.

Hospital ADT 0 new, 2 total					
↑	Status	Type	Sent Date	Patient	
	Pend	Admission	06/27/2024	[Redacted]	
	Admit Date: 6/27/2024		Unit: Teh Micu		
	Pend	Discharge	06/28/2024	[Redacted]	
	Admit Date: 6/28/2024		Unit: Tepv Or		

CC'd Charts 5 new, 13 total						
↑	MRN	Status	Doc Description	Visit Date	Patient	Chief Complaint
	[Redacted]	New		06/20/2024	[Redacted]	Anticoagulation
		Sender Comment:		Comment:		
		Pend		06/27/2024		Anticoagulation
		Sender Comm...		Comment:		
		Pend		06/27/2024		Anticoagulation
		Sender Comm...		Comment:		

- F. Documentation: Clinician will contact patient and document in the patient's chart using the following Epic smart phrase, **AHOSPT [172888]** under a telephone encounter.

Documentation

Create Note

My Note

Telephone Encounter • 6/29/2024 01:38 PM

Summary:

.ahosp

Name	Description
AHOSPT	Spoke with patient/Unable to reach patient

My Note

Telephone Encounter • 6/29/2024 01:38 PM

Summary:

**HOSPITAL FOLLOW UP
ANTICOAGULATION**

Testone Test is a 124 y.o. year old female recently admitted to the hospital. HE/SHE was discharged on HE/SHE was seen for the following problems:

HOSPITAL COURSE:

PROCEDURE/IMAGING:

MEDICATION CHANGES:

WARFARIN ADMINISTRATION:

INR:

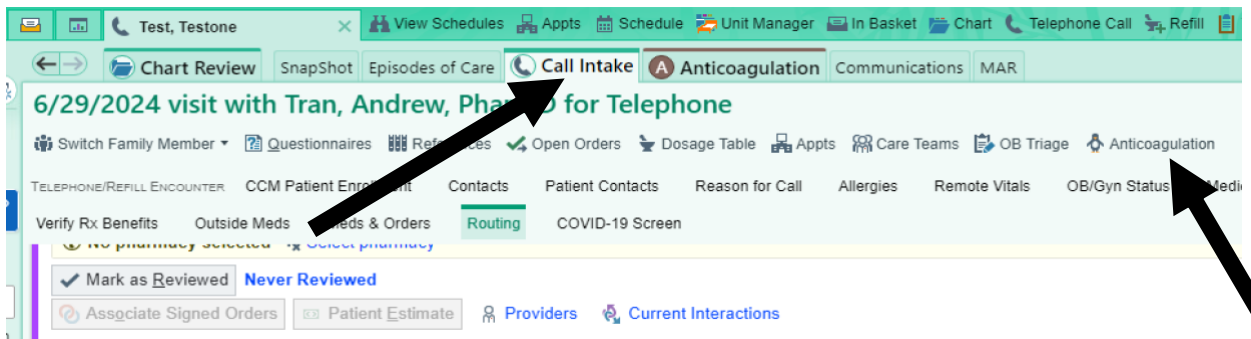
HGB:

PLAN/FOLLOW UP:

Spoke with patient. HE/SHE reports Patient was instructed to warfarin HE/SHE is scheduled to recheck INR on at HE/SHE verbalized understanding.

Attempted to contact patient, however, no answer. Message was left for patient to call the Coumadin Clinic regarding hospital follow up.

- G. If **Hospital ADT** patients are unable to be reached, they will be routed to **CC'd Charts**.
- H. To route patient's telephone encounter, an anticoagulation encounter must be opened from the **Call Intake** tab. Click on Anticoagulation "penguin" to open Anticoagulation encounter.



Test, Testone

View Schedules Appts Schedule Unit Manager In Basket Chart Telephone Call Refill

Chart Review Snapshot Episodes of Care **Call Intake** Anticoagulation Communications MAR

6/29/2024 visit with Tran, Andrew, PharmD for Telephone

Switch Family Member Questionnaires References Open Orders Dosage Table Appts Care Teams OB Triage Anticoagulation

TELEPHONE/REFILL ENCOUNTER CCM Patient Encounter Contacts Patient Contacts Reason for Call Allergies Remote Vitals OB/Gyn Status Medications

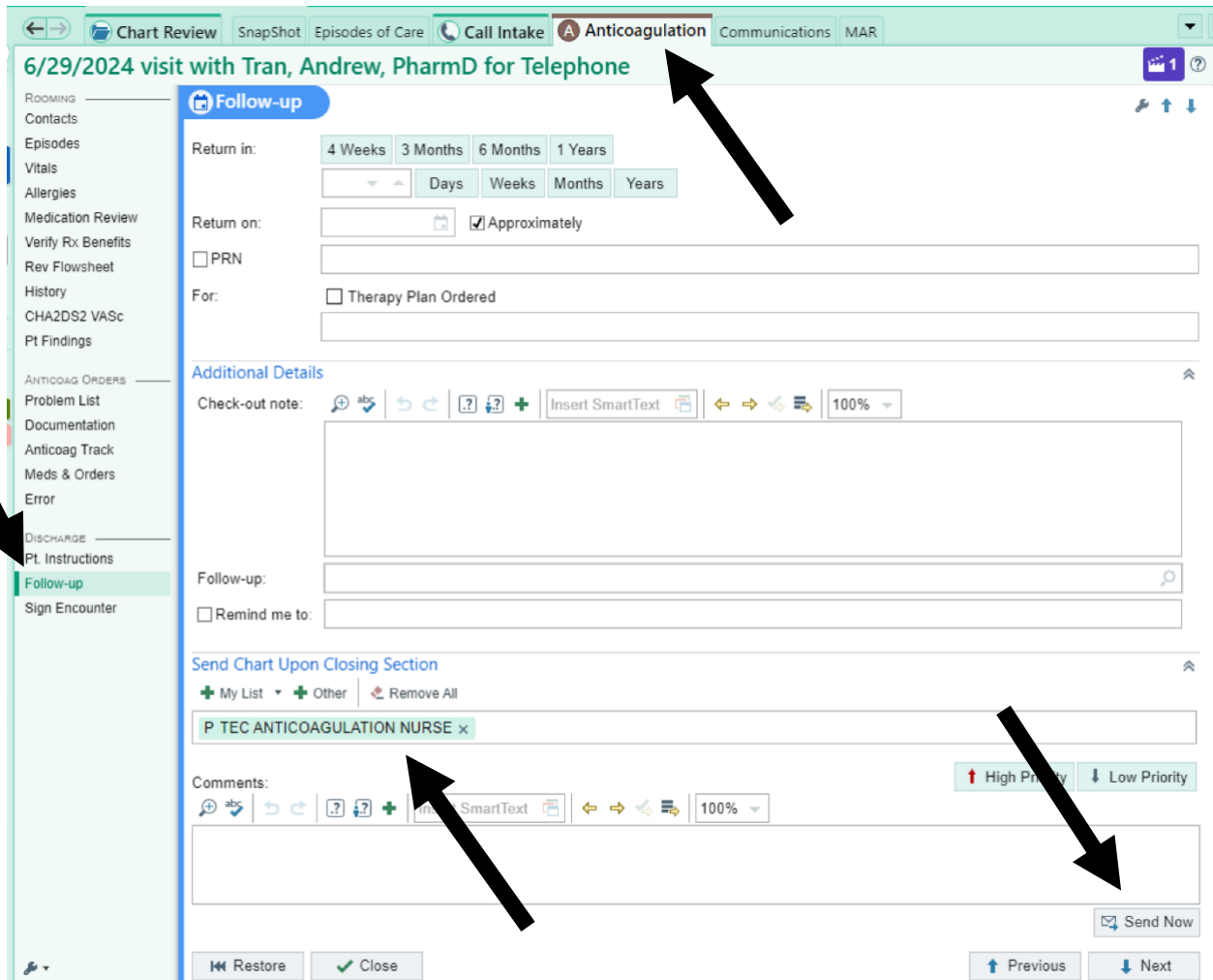
Verify Rx Benefits Outside Meds Orders & Orders Routing COVID-19 Screen

no pharmacy selected Select pharmacy

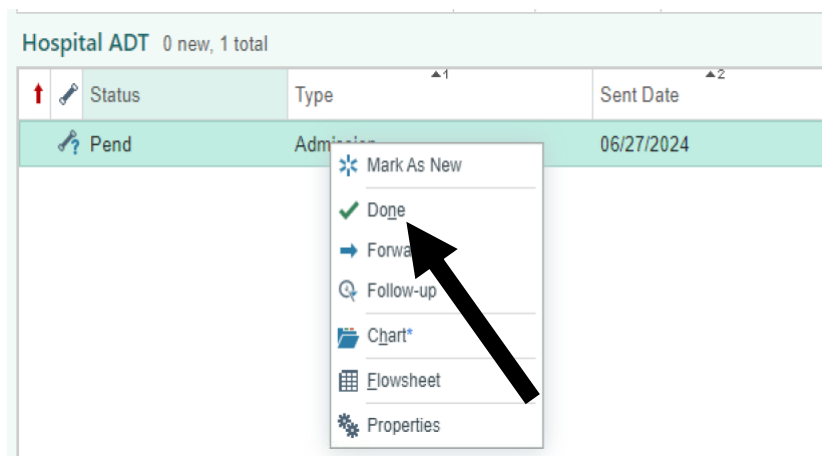
✓ Mark as Reviewed Never Reviewed

Associate Signed Orders Patient Estimate Providers Current Interactions

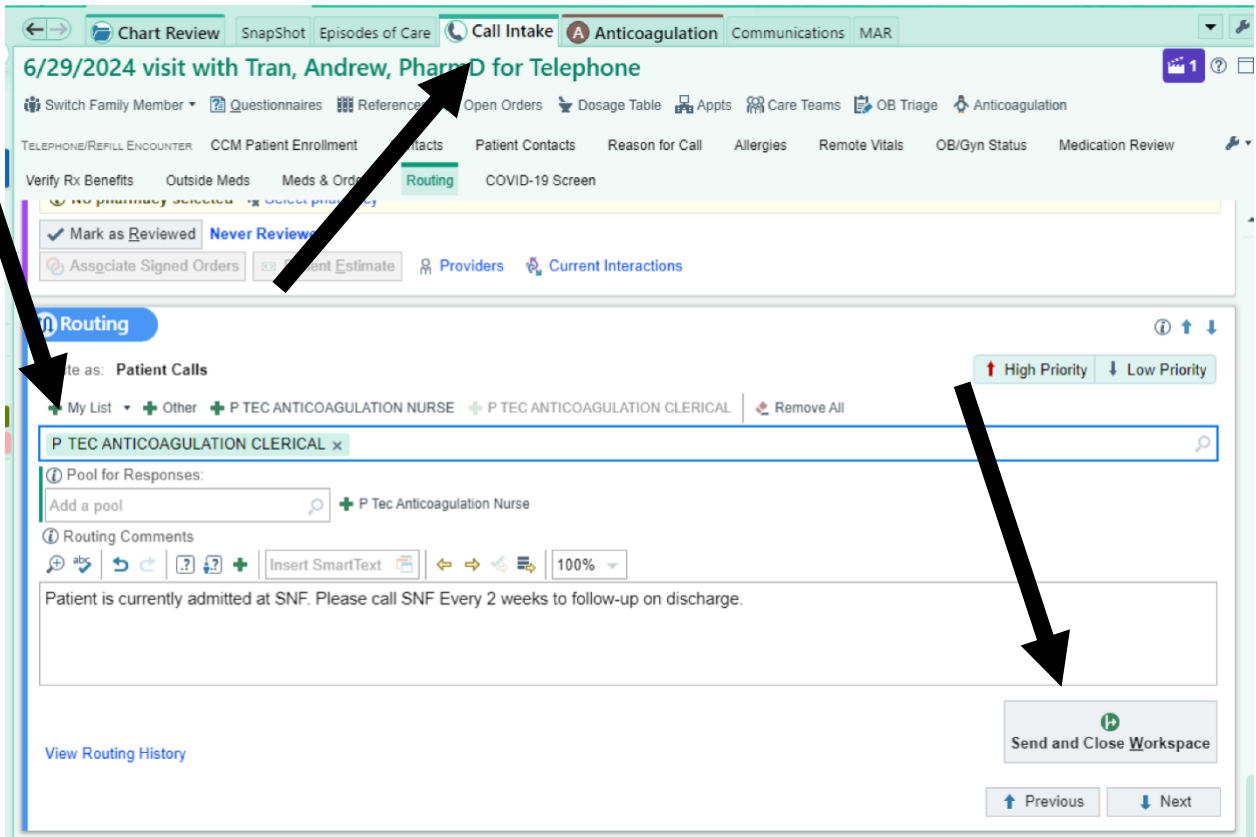
- I. Under Anticoagulation tab on the left side of the screen go to **Follow-up** under **DISCHARGE**. Under **Send Chart Upon Closing Section**, type in P TEC ANTICOAGULATION NURSE. Click send now or simply sign encounter to route to **CC'd Charts**.



- J. Hospital pharmacist will ensure patient has appropriate follow-up appointment (telephone or point-of-care visit) with our clinic, sign encounter, and remove from ques by clicking **Done**.



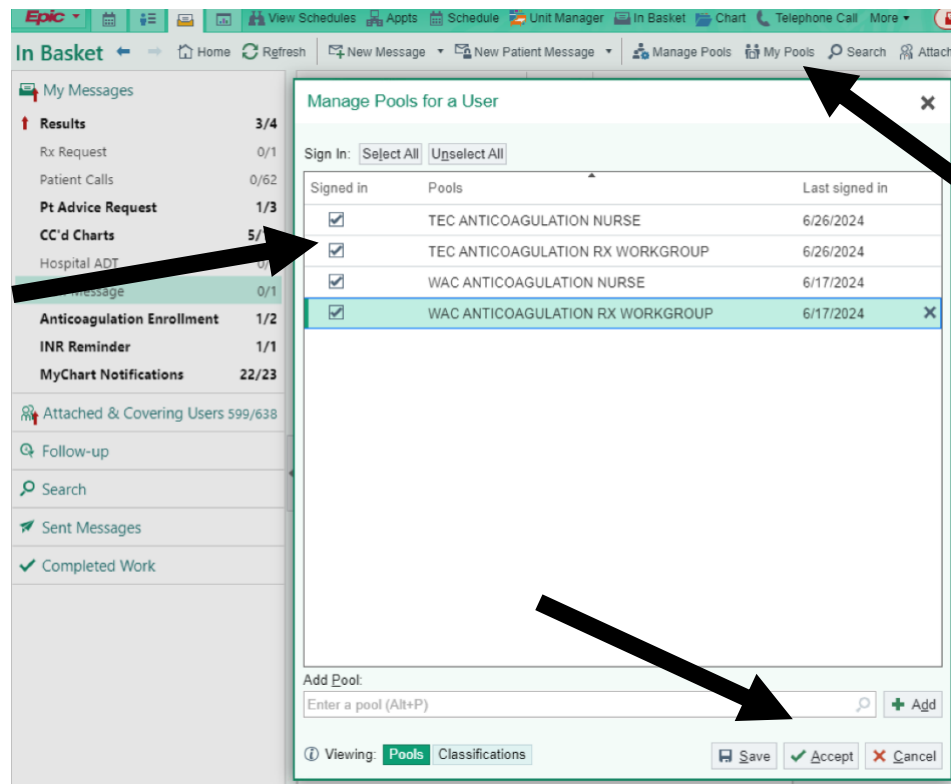
- K. If patient is discharged to a SNF or rehab facility, pharmacist will call facility to confirm that warfarin and INR are being managed by appropriate facility or assist in facilitating the management. If patient is confirmed to be managed by facility, clinician will route telephone encounter under **Call Intake** to **P TEC ANTICOAGULATION CLERICAL** with name of facility and contact for clerical staff to follow-up every 2 weeks until discharge.



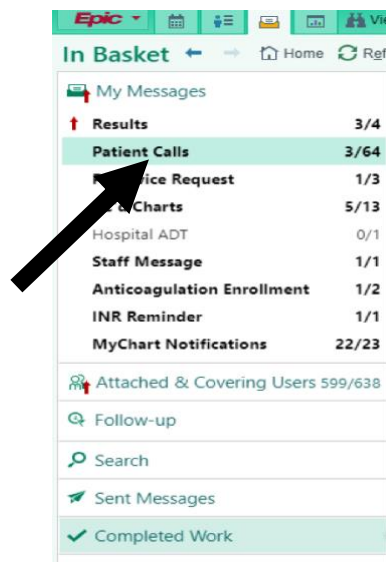
XI. TRIAGE RESPONSIBILITIES

TRIAGE IN-BASKET

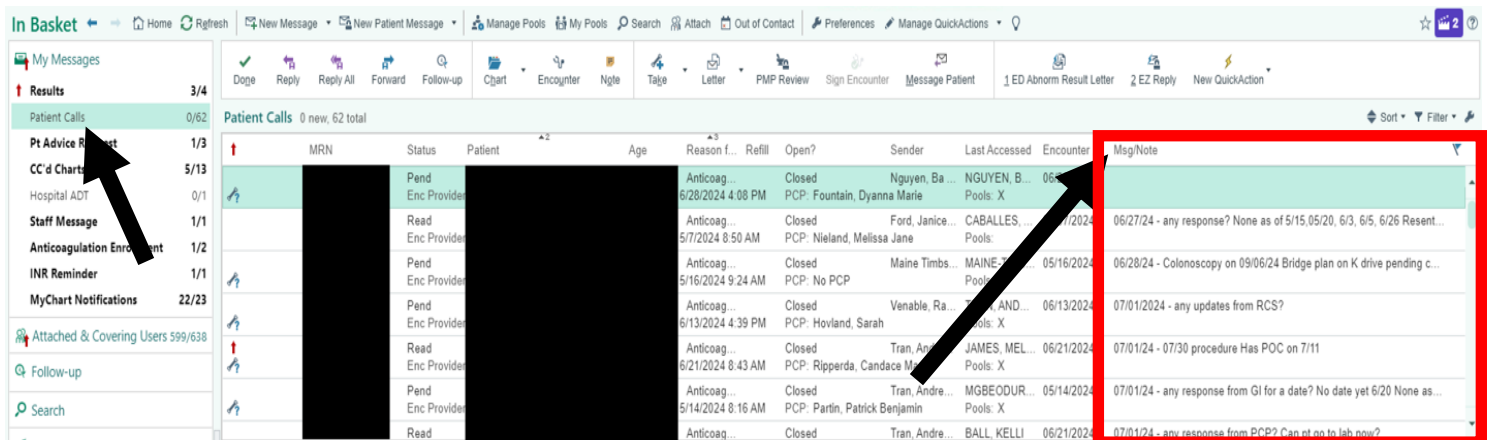
- The Triage pharmacist will be responsible for all miscellaneous tasks involved in the care of ACC patients. This includes but not limited to: peri-operative therapy plans, provider or patient concerns/questions, refills, medication changes, self-test initiation, coordination of care, enrollment questions etc.
- Triage pharmacist will be first-line if a patient or other healthcare personnel calls with questions or concerns outside of a simple INR result.
- To make ques visible, click on **My Pools** and select the appropriate que for Temple (**TEC ANTICOAGULATION NURSE[2102100697]**, **TEC ANTICOAGULATION RX WORKGROUP [2102100699]**) or Waco (**WAC ANTICOAGULATION NURSE [2102101473]**, **WAC ANTICOAGULATION RX WORKGROUP[2102101475]**). Click accept once appropriate ques are selected.



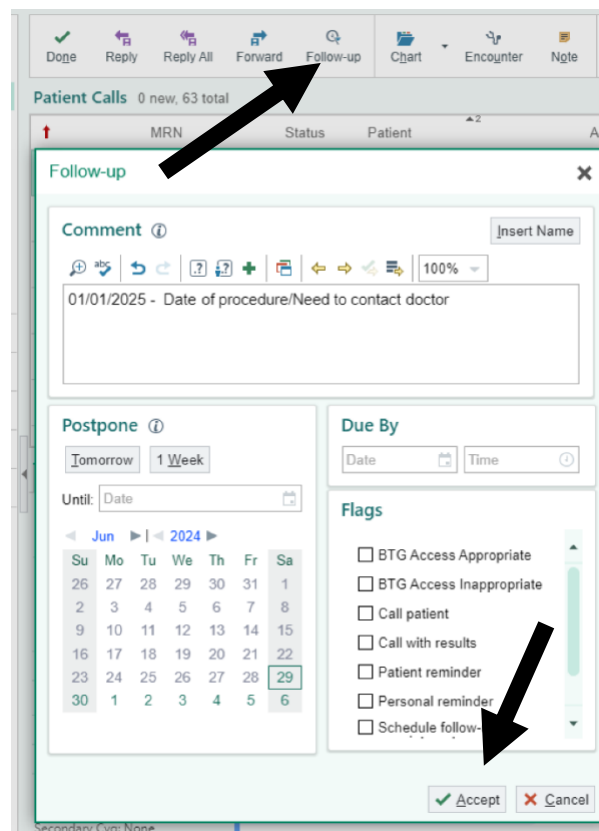
- L. The main que that the triage pharmacist will cover will be **Patient Calls** in the in-basket. However, the pharmacist will additionally need to cover other ques such as **Refills**, **Pt Advice Requests**, **Staff Message**, **Anticoagulation Enrollment**, and other miscellaneous ques outside of Hospital ADT, CC'd Charts, or Results



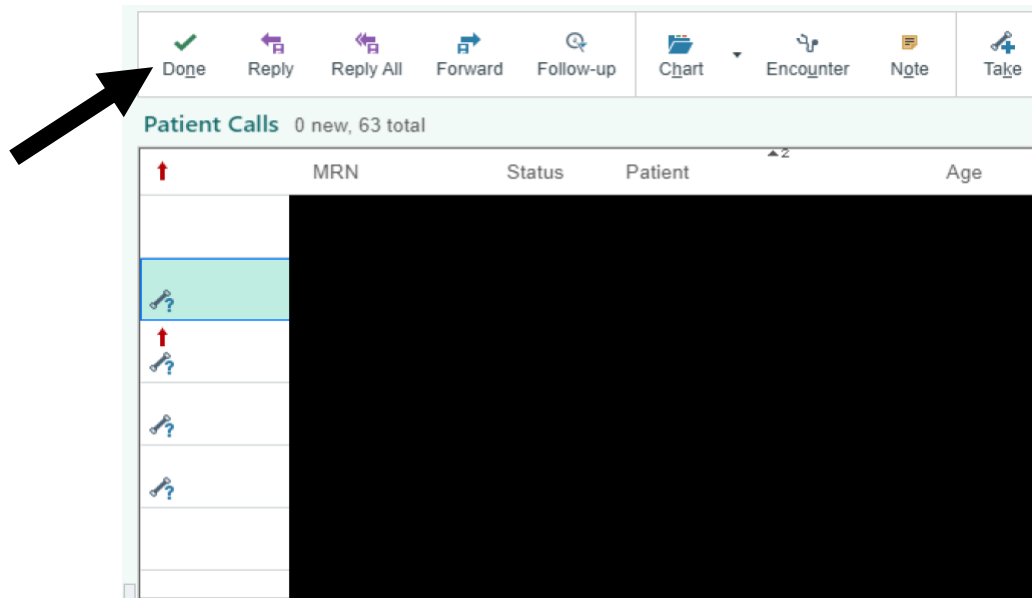
- D. Under **Patient Calls**, tasks are organized based on date under the **Msg/Note** column. Additional relevant notes will be included after the date.



M. To prioritize date or add notes, click on **Follow-Up** and type in appropriate date – information and click accept.

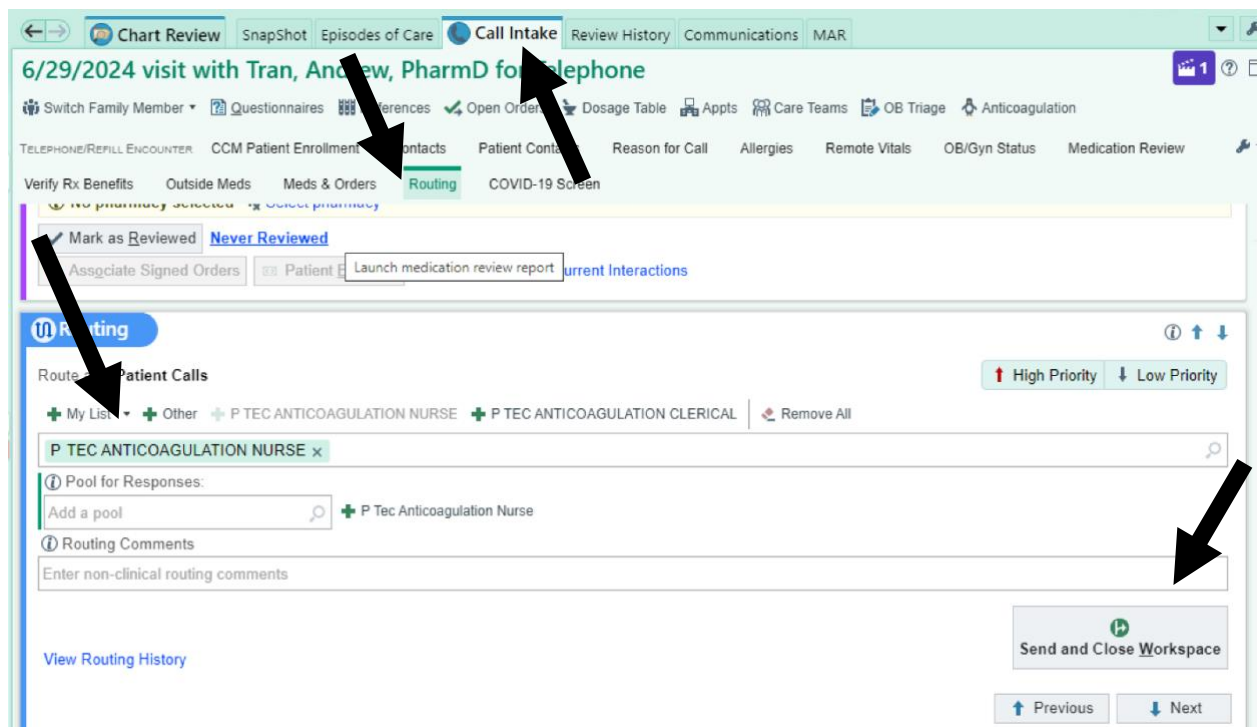


- N. If a task is not complete, needs to be moved to a further date, or followed-up again, triage pharmacist will add the future date with notes.
- O. To complete a task, click on the task and click **Done**.

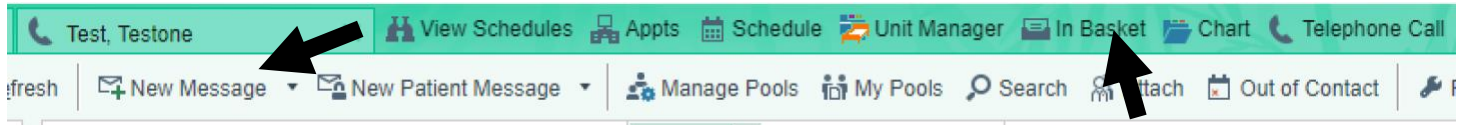
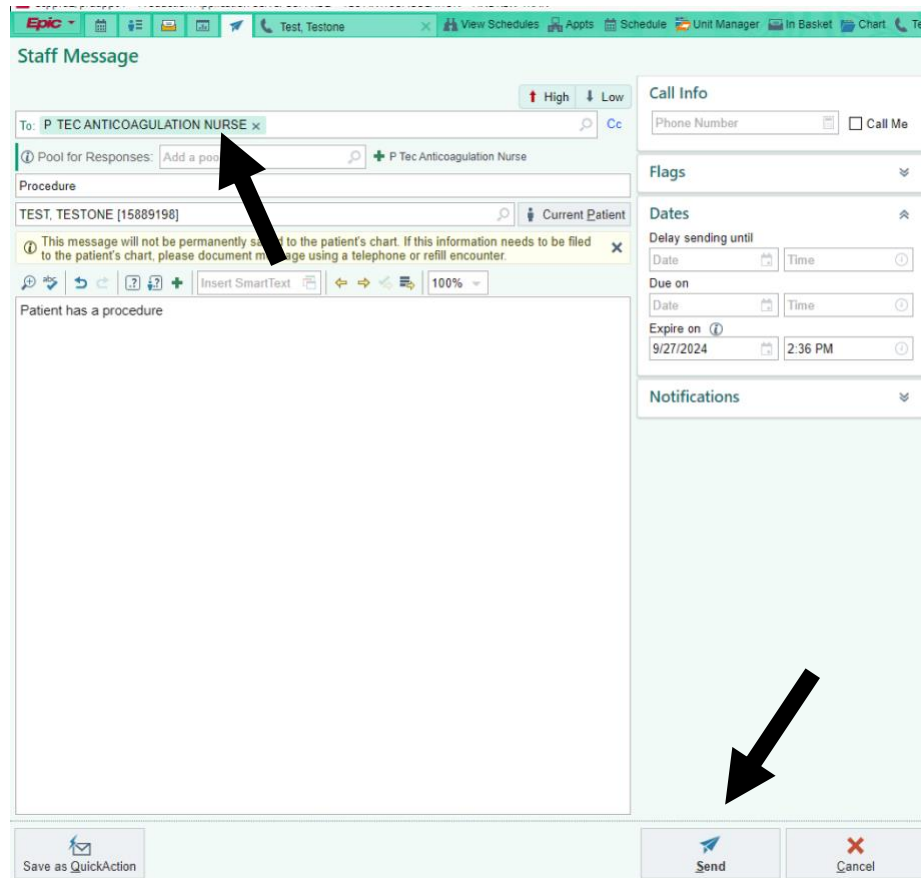


ROUTING TO TRIAGE

- P. Tasks are routed to triage by telephone encounter or staff message.
- Q. To route a telephone encounter, go under **Call Intake** tab → **Routing** → add **P TEC ANTICOAGULATION NURSE** → Click **Send and Close Workspace**.



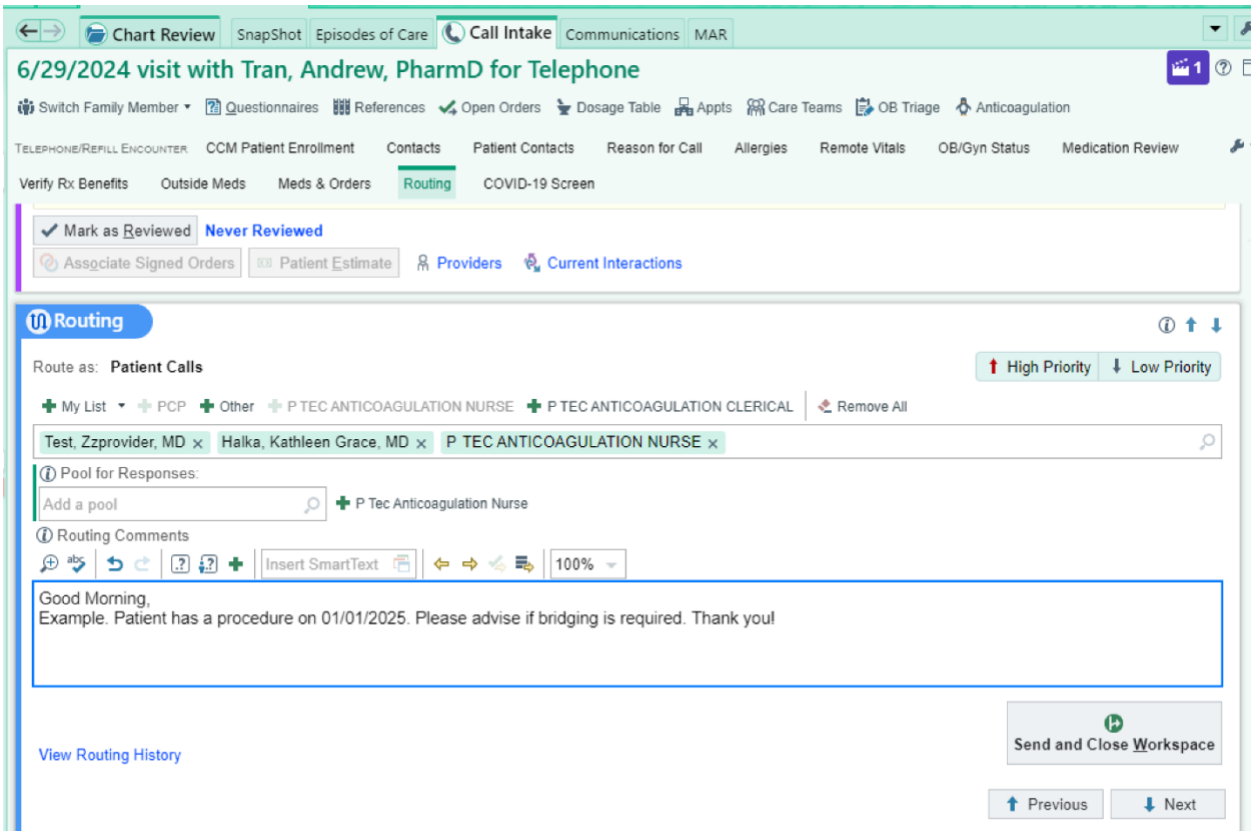
- R. To send a **staff message** to triage, click on **In Basket** at the top of Epic → click on **New Message** → Type in **P TEC ANTICOAGULATION NURSE** → Type appropriate subject, attach patient, and click **send**.

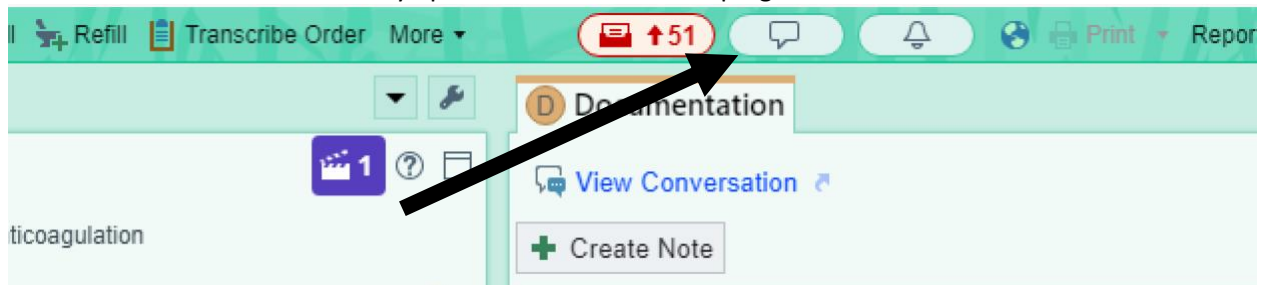
The image shows the 'Staff Message' form in Epic. The 'To' field is populated with 'P TEC ANTICOAGULATION NURSE'. The 'Pool for Responses' dropdown is set to 'Add a pool'. The 'Procedure' field is populated with 'TEST, TESTONE [15889198]'. The 'Patient' field is set to 'Current Patient'. A yellow warning message states: 'This message will not be permanently saved to the patient's chart. If this information needs to be filed to the patient's chart, please document message using a telephone or refill encounter.' The 'Send' button is highlighted with an arrow. The 'Cancel' button is also visible. The 'Call Info' section on the right includes fields for 'Phone Number' and 'Call Me'. The 'Flags' section is expanded, showing 'Delay sending until', 'Due on', and 'Expire on' fields. The 'Expire on' field is set to '9/27/2024' at '2:36 PM'. The 'Notifications' section is also expanded.

CONTACTING PROVIDERS THROUGH EPIC

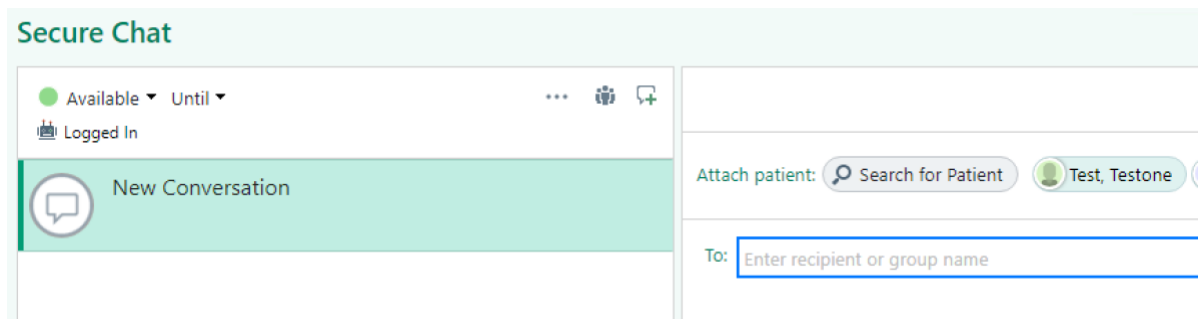
- If a provider or other healthcare personnel needs to be contacted, the telephone encounter can be routed to their in-basket. Under **Call Intake** → **Routing** → You can add the recipients and type a routing comment → Then **Send and Close Workspace**.
- Make sure to route **P TEC ANTICOAGULATION NURSE** as well so the updates can stay in the in-basket.

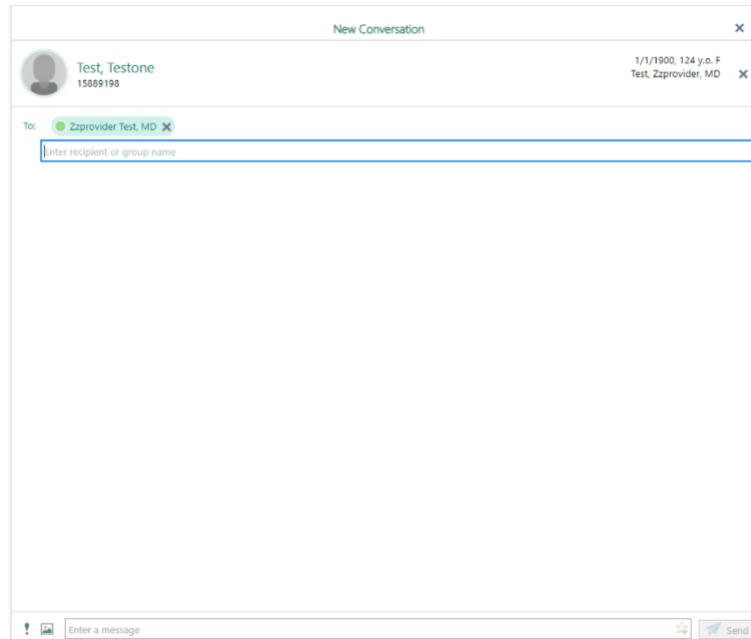


C. Providers can also be reached by Epic Secure Chat in the top right hand corner.

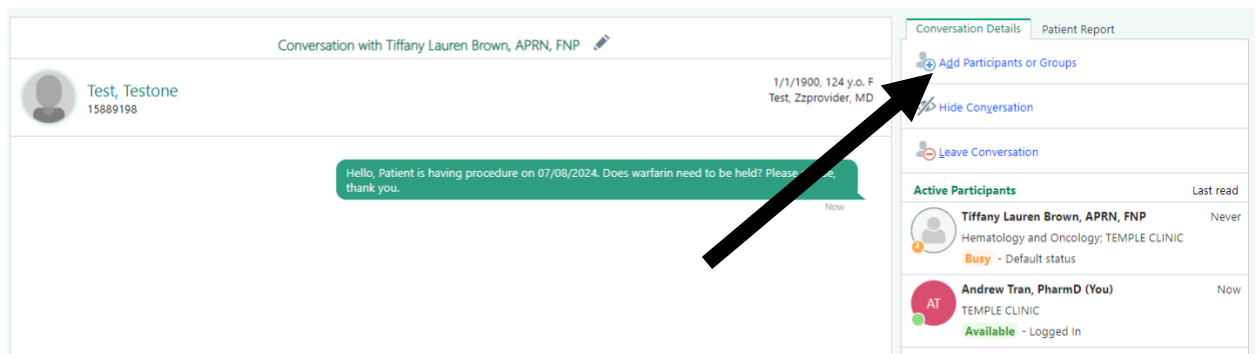


D. Attach the appropriate patient and select to the appropriate provider or healthcare personnel. Then you can type the appropriate message on the bottom.





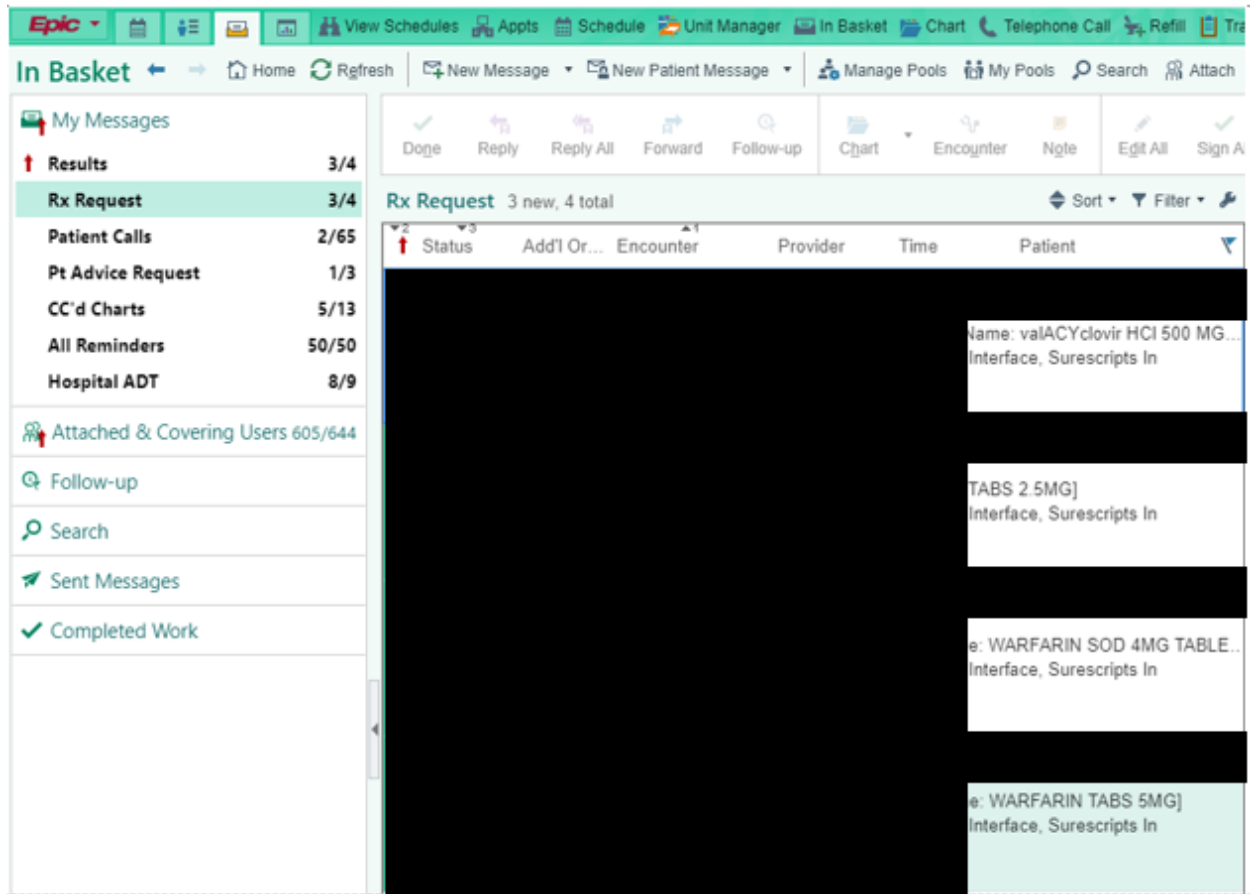
- E. The chat will show whether the provider has seen the message or not. You can track responses in there. If there is no response and the conversation still needs to be followed please add the next Triage pharmacist by click on **Add Participants or Groups**.



Active Participants		Last read
	Tiffany Lauren Brown, APRN, FNP Hematology and Oncology; TEMPLE CLINIC Busy - Default status	Never
	Andrew Tran, PharmD (You) TEMPLE CLINIC Available - Logged In	Now

REFILL REQUESTS

- A. Refills can be found under “Rx Request” in the In-basket.



Epic | View Schedules | Appts | Schedule | Unit Manager | In Basket | Chart | Telephone Call | Refill | Tra

In Basket | Home | Refresh | New Message | New Patient Message | Manage Pools | My Pools | Search | Attach

My Messages

- Results 3/4
- Rx Request 3/4**
- Patient Calls 2/65
- Pt Advice Request 1/3
- CC'd Charts 5/13
- All Reminders 50/50
- Hospital ADT 8/9

Attached & Covering Users 605/644

Follow-up

Search

Sent Messages

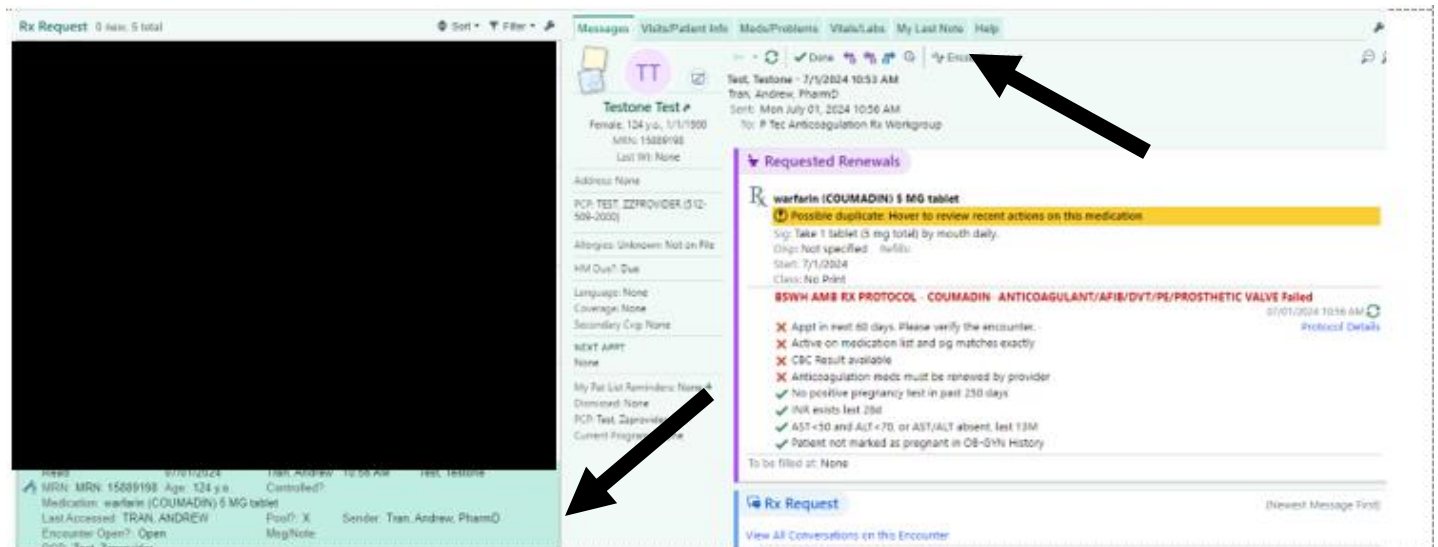
Completed Work

Rx Request 3 new, 4 total

Sort | Filter

Status	Add'l Or...	Encounter	Provider	Time	Patient
					Name: valACYclovir HCl 500 MG... Interface, Surescripts In
					TABS 2.5MG] Interface, Surescripts In
					e: WARFARIN SOD 4MG TABLE... Interface, Surescripts In
					e: WARFARIN TABS 5MG] Interface, Surescripts In

B. Click on the refill you want to work on and click **Encounter** at the top.



Rx Request 0 new, 5 total

Sort | Filter

Messages | Visit/Patient Info | Medic Problems | Vitals/Labs | My Last Note | Help

Teststone Test
Female, 124 y.o., 1/1/1990
Meds: 15689198
Last RN: None

Address: None
PCP: TEST, ZZPROVIDER (312-509-2000)
Allergies: Unknown: Not on File
HIM Out? Due
Language: None
Coverage: None
Secondary Cop: None
NEXT APPT: None
My Pat Last Reminders: None
Diamond: None
PCP Test, Zaprovider
Current Progress: None

warfarin (COUMADIN) 5 MG tablet
Possible duplicate. Hover to review recent actions on this medication.
Sig: Take 1 tablet (5 mg total) by mouth daily.
Disp: Not specified - fulfill.
Start: 7/1/2024
Class: No Point

BSWH AMB RX PROTOCOL - COUMADIN - ANTICOAGULANT/APBI/DVT/PE/PROSTHETIC VALVE Failed
07/01/2024 10:56 AM
Protocol Details

- Appt in next 60 days. Please verify the encounter.
- Active on medication list and sig matches exactly.
- CBC Result available.
- Anticoagulation meds must be reviewed by provider.
- No positive pregnancy test in past 230 days.
- INR exists last 28d.
- AST < 50 and ALT < 70 or AST/ALT absent last 13M.
- Patient not marked as pregnant in OB-GYN History.

To be filled at: None

Rx Request
View All Conversations on this Encounter

MRN: 15689198 Age: 124 y.o.
Medication: warfarin (COUMADIN) 5 MG tablet
Last Accessed: TRAN, ANDREW
Encounter Open?: Open
RxSig: Test, Zaprovider

C. In the encounter, there should be a pending order to the bottom.

Dx Association
Edit Multiple
Estimate
Options

Per Standing Delegated Order

Outpatient
☐ warfarin (COUMADIN) 5 MG tablet

Take 1 tablet (5 mg total) by mouth daily.

No Print

Rx Select a pharmacy

☒ Approve All
☒ Refuse All
☐ Uncheck All

LEVEL OF SERVICE
PEND
SIGN ORDERS

- D. Review chart to determine appropriateness of refill. Open the order by clicking on the medication name.
- E. If you get an alert that patient is taking differently, click **Order with Changes**.
- F. Sig method can typically either be **Specify Dose, Route** or **Use Free Text**.

warfarin (COUMADIN) 2.5 MG tablet [Pharmacy Med Name: WARFARIN SODIUM 2.5MG TABLET]
☒ Accept
☒ Cancel

Patient Taking Differently

The patient reported taking this medication differently from how it was prescribed.
Entered by: McPheters, Debra Lynn, APRN, AGNP On: 4/12/2024

☒ Order as Prescribed

TAKE 1.5 TABS (3.75MG) BY MOUTH EVERY WED * TAKE 1 TAB (2.5) EVERY SUN, MON, TUE, THU, FRI, SAT
Order the medication as it was prescribed.

☐ Order as Reported

2.5 mg, Oral, 6-times-weekly, 1.25 tab on Tuesdays and 2.5mg all other days
The following patient-reported values cannot be used: Frequency (6 times weekly)
Order the medication as the patient currently reports taking it.

☐ Order with Changes

Edit the medication and specify new details.

Next Required
☒ Accept
☒ Cancel

warfarin (COUMADIN) 5 MG tablet
Accept Cancel

Reference Links:

- BSWH In-patient Monitoring Policy
- Micromedex: Pedi
- BSWH In-patient Warfarin Consult Policy
- NeoFax
- Micromedex: Adult
- Black Box Warning

Order Instructions:

Product:
WARFARIN 5 MG TABLET
View Available Strengths

Sig Method:
Specify Dose, Route, Frequency
Taper/Ramp
Combination Dosage
Use Free Text

Dose:
5 mg
5 mg

Calculated dose:
1 tablet

Route:
Oral
Oral

Frequency:
Daily warfarin
Daily

Duration:
Doses
Days

Starting:
7/1/2024
Ending:

Dispense:
Days/Fill:
Full (0 Days)
30 Days
90 Days

Quantity:
table
Refill:
0
1
2
3
4
5
6
11
12

Total Supply:
Unable to calculate

☐ Dispense As Written

Mark long-term:
WARFARIN SODIUM

Patient Sig:
Take 1 tablet (5 mg total) by mouth daily.
Add additional information to the patient sig

Report:
Lab Test Results

Component	Time Elapsed	Value	Range	Status
INR	11 hours (07/01/24 0000)	2.5 !	0.9 - 1.1	Final result
	3 days (06/28/24 0000)	2.5 !	0.9 - 1.1	Final result

Renewal Provider:
☐ Do not send renewal requests to the authorizing provider (Halka, Kathleen Grace, MD)

Include in Refill Protocol?
Yes
No

THIS MEDICATION WILL BE INCLUDED IN BSW REFILL CENTER PROTOCOLS (FYI no response required)

Next Required
Accept Cancel

- G. Typically for warfarin refills, click on **Use Free Text** so we can give the patient a flexible amount due to changing INRs.
- H. In **Patient Sig**: Use SmartPhrase **ACCRFSIG [1669487]** to include the sig to have tabs and day supply.

warfarin (COUMADIN) 5 MG tablet

Accept

Cancel

Reference Links:

BSWH In-patient Monitoring Policy

BSWH In-patient Warfarin Consult Policy

Micromedex: Adult

Micromedex: Pedi

NeoFax

Black Box Warning

Order Instructions:

Product:

WARFARIN 5 MG TABLET

View Available Strengths

Sig Method:

Specify Dose, Route, Frequency

Taper/Ramp

Combination Dosage

Use Free Text

Start Date:

7/1/2024

End Date:

Dispense:

Quantity:

100

tablet

Refill:

1

0

1

2

3

4

5

6

11

12

☐ Dispense As Written

Mark long-term:

WARFARIN SODIUM

Patient Sig:

Take by mouth as directed by the Anticoagulation Clinic # *** tabs = *** D/S.

abc

undo

redo

help

insert smart text

undo

redo

100%

Take by mouth as directed by the Anticoagulation Clinic # *** tabs = *** D/S

Patient sig contains unfilled wildcards (****) or SmartLists.

Report:

Lab Test Results

Component	Time Elapsed	Value	Range	Status
INR	11 hours (07/01/24 0000)	2.5 !	0.9 - 1.1	Final result
	3 days (06/28/24 0000)	2.5 !	0.9 - 1.1	Final result

Renewal Provider:

☐ Do not send renewal requests to the authorizing provider (Halka, Kathleen Grace, MD)

Include in Refill Protocol?

Yes

No

THIS MEDICATION WILL BE INCLUDED IN BSW REFILL CENTER PROTOCOLS (FYI no response required)

OK

Class:

No Print

Normal

Print

Phone In

No Print

OTC

Note to Pharmacy:

+ Add Note to Pharmacy

Dx association:

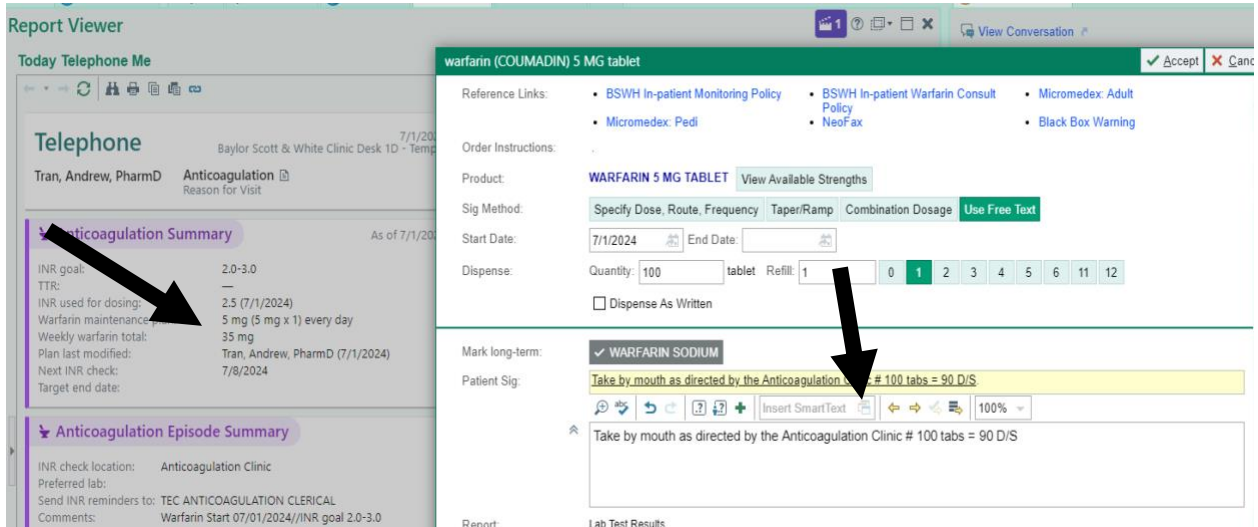
Associate diagnoses

Next Required

Accept

Cancel

- I. For 3 month supply - Typically if patient is stable, some clinicians will DIVIDE the weekly total by the tablet size to determine how many tablets they need per week then MULTIPLY by 13 weeks (for patient to get an extra week for adjusting) then round to the nearest tenth.
- J. For 1 month supply – you can multiply by 5 weeks instead.
- K. Example: 90 day supply → Weekly total = 35 mg. Tablet size = 5 mg. $35/5 = 7$ tabs per week. $7 \times 13 = 91 \rightarrow 100$ tablets = 90 day supply.



Report Viewer

Today Telephone Me

Telephone

Tran, Andrew, PharmD Anticoagulation Reason for Visit

Anticoagulation Summary As of 7/1/2024

INR goal: 2.0-3.0
 TTR: —
 INR used for dosing: 2.5 (7/1/2024)
 Warfarin maintenance: 5 mg (5 mg x 1) every day
 Weekly warfarin total: 35 mg
 Plan last modified: Tran, Andrew, PharmD (7/1/2024)
 Next INR check: 7/8/2024
 Target end date: —

Anticoagulation Episode Summary

INR check location: Anticoagulation Clinic
 Preferred lab: —
 Send INR reminders to: TEC ANTICOAGULATION CLERICAL
 Comments: Warfarin Start 07/01/2024//INR goal 2.0-3.0

warfarin (COUMADIN) 5 MG tablet Accept Cancel

Reference Links: • BSWH In-patient Monitoring Policy • BSWH In-patient Warfarin Consult Policy • Micromedex: Adult Policy • Micromedex: Pedi • NeoFax • Black Box Warning

Order Instructions: —

Product: **WARFARIN 5 MG TABLET** View Available Strengths

Sig Method: Specify Dose, Route, Frequency Taper/Ramp Combination Dosage Use Free Text

Start Date: 7/1/2024 End Date: —

Dispense: Quantity: 100 tablet Refill: 1 0 1 2 3 4 5 6 11 12

☐ Dispense As Written

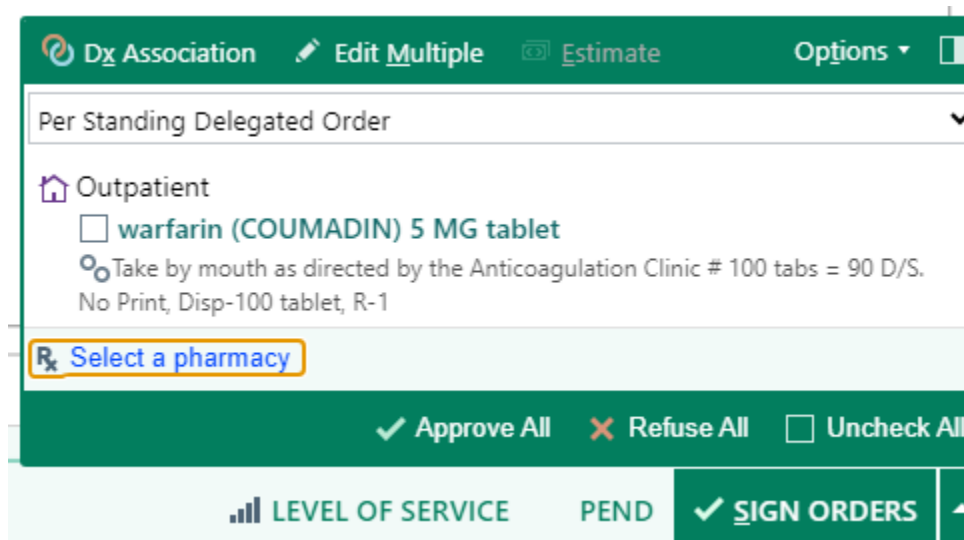
Mark long-term: ☒ **WARFARIN SODIUM**

Patient Sig: Take by mouth as directed by the Anticoagulation Clinic # 100 tabs = 90 D/S

Take by mouth as directed by the Anticoagulation Clinic # 100 tabs = 90 D/S

Renort Lab Test Results

- L. Click Accept when finished.
- M. Make sure to click the appropriate pharmacy below.
- N. If refill is correct and appropriate, click **Sign Orders** on the bottom. Make sure to click **Per Standing Delegated Order**.



Dx Association Edit Multiple Estimate Options

Per Standing Delegated Order

Outpatient

☐ **warfarin (COUMADIN) 5 MG tablet**

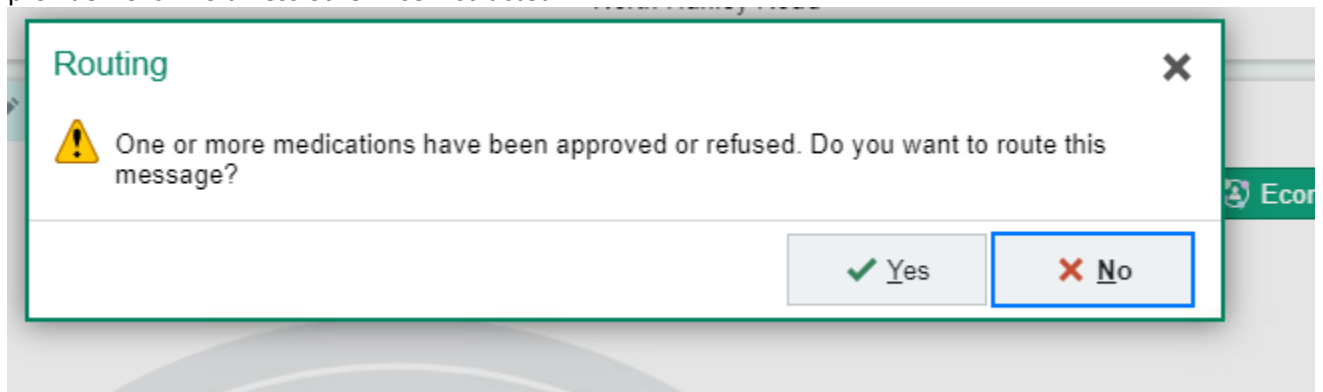
Take by mouth as directed by the Anticoagulation Clinic # 100 tabs = 90 D/S.
 No Print, Disp-100 tablet, R-1

Select a pharmacy

☒ Approve All ☒ Refuse All ☐ Uncheck All

LEVEL OF SERVICE **PEND** **SIGN ORDERS**

- O. Sign the encounter in the bottom right hand corner then it'll ask if you need to route the message to another provider. Click No unless otherwise instructed.

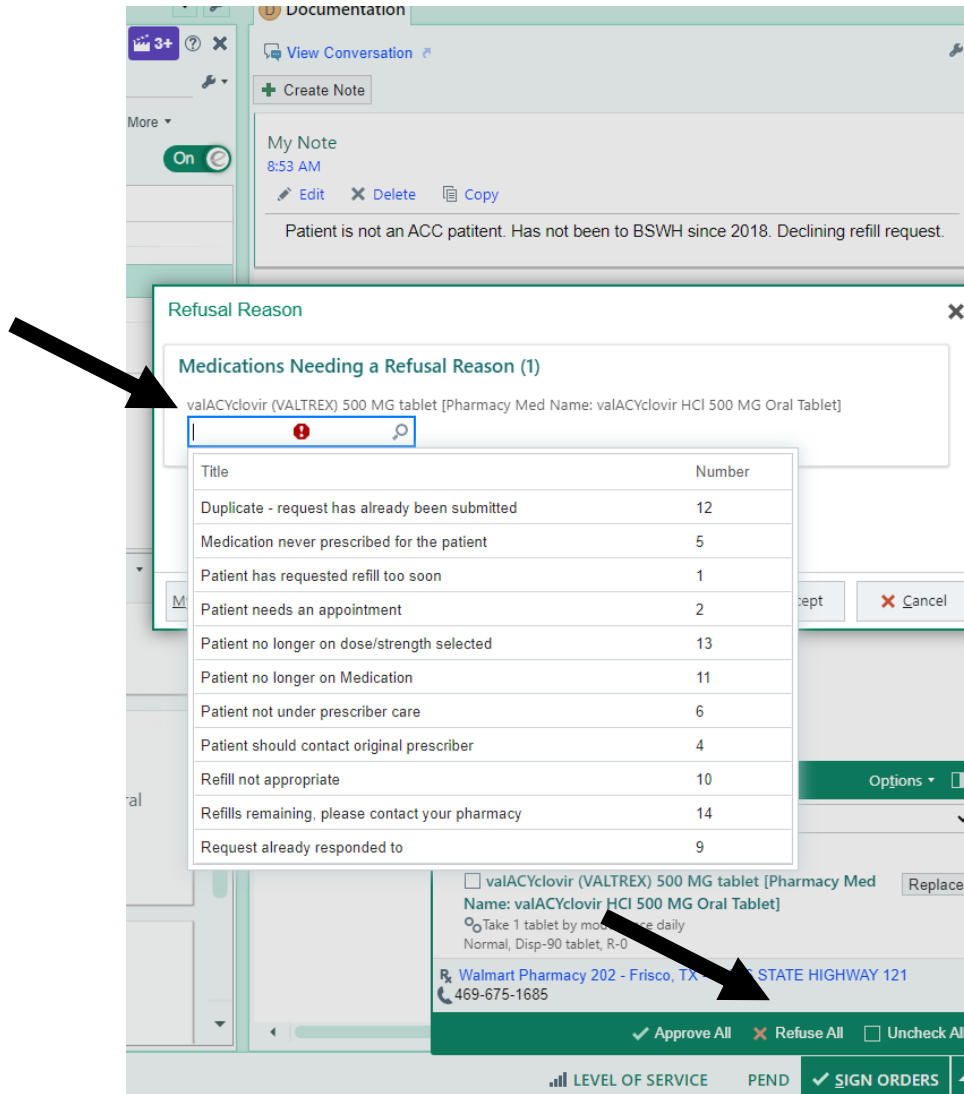


Routing X

⚠ One or more medications have been approved or refused. Do you want to route this message?

☒ Yes ☒ No

- P. To DECLINE an inappropriate refill, click on **Refuse ALL** and when you sign the order, you MUST put a refusal reason.



Documentation

View Conversation

Create Note

My Note
8:53 AM

Edit Delete Copy

Patient is not an ACC patient. Has not been to BSWH since 2018. Declining refill request.

Refusal Reason

Medications Needing a Refusal Reason (1)

valACYclovir (VALTREX) 500 MG tablet [Pharmacy Med Name: valACYclovir HCl 500 MG Oral Tablet]

Title	Number
Duplicate - request has already been submitted	12
Medication never prescribed for the patient	5
Patient has requested refill too soon	1
Patient needs an appointment	2
Patient no longer on dose/strength selected	13
Patient no longer on Medication	11
Patient not under prescriber care	6
Patient should contact original prescriber	4
Refill not appropriate	10
Refills remaining, please contact your pharmacy	14
Request already responded to	9

Accept Cancel

Options

valACYclovir (VALTREX) 500 MG tablet [Pharmacy Med Name: valACYclovir HCl 500 MG Oral Tablet]

Take 1 tablet by mouth twice daily

Normal, Disp-90 tablet, R-0

Walmart Pharmacy 202 - Frisco, TX STATE HIGHWAY 121

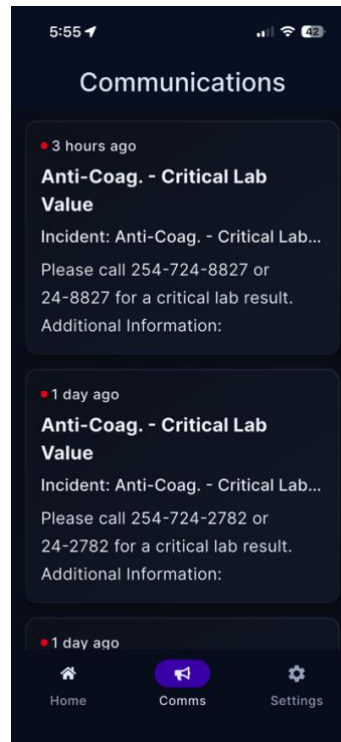
469-675-1685

Approve All Refuse All Uncheck All

LEVEL OF SERVICE PEND SIGN ORDERS

CRITICAL PAGES/EVERBRIDGE – Setting up Everbridge at: [Everbridge Mobile Download](#); Everbridge set up instruction at: [Everbridge Set-Up Instructions](#)

- I. Critical lab results (INR >4) will be called into Everbridge app to the pharmacist. This responsibility will rotate between Triage and INR pharmacist.
- II. Critical lab results (INR > 4) reported between 5-11 PM are paged into Everbridge app to the pharmacist (on-weekends) and nurses (on weeknights). After 11 PM, results will be called to inpatient pharmacy.
- III. Pages will appear on the app with the extension of the lab to call.



PERI-OPERATIVE ANTICOAGULATION

I. Peri-operative Bridge Plan Process

When designing peri-operative procedure plans, clinicians should consider:

- II. What type of procedure and what is the risk of bleeding with continued anticoagulation?
- III. If interruption of anticoagulation is warranted, what is the risk of thromboembolism?
- IV. If thromboembolism risk is associated with therapy interruption, is bridging with parenteral therapy warranted?
- V. Renal function and other safety parameters should be assessed before starting parenteral therapy

Perioperative Procedure Plan Process

1. Clinician will assess surgical bleeding risk vs thromboembolic risk to determine if anticoagulation interruption is necessary

TABLE 1 Suggested Risk Stratification for Patient-specific Periprocedural Thromboembolism^{13,14,25,a}

Risk Category	Mechanical Heart Valve	Atrial Fibrillation	VTE
High (> 10%/y risk of ATE or > 10%/mo risk of VTE)	Mitral valve with major risk factors for stroke ^b Caged ball or tilting-disc valve in mitral/aortic position Recent (< 3 mo) stroke or TIA	CHA ₂ DS ₂ -VASc score ≥ 7 or CHADS ₂ score of 5 or 6 Recent (< 3 mo) stroke or TIA Rheumatic valvular heart disease	Recent (< 3 mo and especially 1 mo) VTE Severe thrombophilia (deficiency of protein C, protein S or antithrombin; homozygous factor V Leiden or prothrombin gene G20210A mutation or double heterozygous for each mutation, multiple thrombophilias) Antiphospholipid antibodies Active cancer associated with high VTE risk ^c
Moderate (4%-10%/y risk of ATE or 4%-10%/mo risk of VTE)	Mitral valve without major risk factors for stroke ^b Bileaflet AVR with major risk factors for stroke ^b	CHA ₂ DS ₂ -VASc score of 5 or 6 or CHADS ₂ score of 3 or 4	VTE within past 3-12 mo Recurrent VTE Non-severe thrombophilia (heterozygous factor V Leiden or prothrombin gene G20210A mutation) Active cancer or recent history of cancer
Low (< 4%/y risk of ATE or < 2%/mo risk of VTE)	Bileaflet AVR without major risk factors for stroke ^b	CHA ₂ DS ₂ -VASc score of 1-4 or CHADS ₂ score of 0-2 (and no prior stroke or TIA)	VTE > 12 mo ago

ATE, arterial thromboembolism; TIA, transient ischemic attack; CHADS₂ = congestive heart failure, hypertension, age ≥ 75 years, diabetes mellitus, prior stroke or transient ischemic attack; CHADS₂-VASc = congestive heart failure, hypertension, age ≥ 75 years, diabetes mellitus, prior stroke or transient ischemic attack, vascular disease history, age ≥ 65 years, female sex. Adapted with permission from Douketis et al.¹⁴

^aEmpiric risk stratification that is a starting point for assessing periprocedural thromboembolism risk should be combined with clinical judgment that incorporates individual patient- and surgery/procedure-related factors.

^bIncludes AF, prior stroke/TIA during anticoagulant interruption or other prior stroke/TIA, prior valve thrombosis, rheumatic heart disease, hypertension, diabetes, congestive heart failure, age ≥ 75 years.

^cIncludes pancreatic cancer, myeloproliferative disorders, primary brain cancer, gastric cancer, and esophageal cancer.

TABLE 2 Suggested Risk Stratification for Procedural Bleed Risk, Based on ISTH Guidance Statements²⁵

High-bleed-risk surgery/procedure ^a (30-d risk of major bleed ≥ 2%)	Major surgery with extensive tissue injury Cancer surgery, especially solid tumor resection (lung, esophagus, gastric, colon, hepatobiliary, pancreatic) Major orthopedic surgery, including shoulder replacement surgery Reconstructive plastic surgery Major thoracic surgery Urologic or GI surgery, especially anastomosis surgery Transurethral prostate resection, bladder resection, or tumor ablation Nephrectomy, kidney biopsy Colonic polyp resection Bowel resection Percutaneous endoscopic gastrostomy placement, endoscopic retrograde cholangiopancreatography Surgery in highly vascular organs (kidneys, liver, spleen) Cardiac, intracranial, or spinal surgery Any major operation (procedure duration > 45 min) Neuraxial anesthesia ^b Epidural injections
Low-to-moderate-bleed-risk surgery/procedure ^c (30-d risk of major bleed 0%-2%)	Arthroscopy Cutaneous/lymph node biopsies Foot/hand surgery Coronary angiography ^d GI endoscopy ± biopsy Colonoscopy ± biopsy Abdominal hysterectomy Laparoscopic cholecystectomy Abdominal hernia repair Hemorrhoidal surgery Bronchoscopy ± biopsy
Minimal-bleed-risk surgery/procedure ^e (30-d risk of major bleed approximately 0%)	Minor dermatologic procedures (excision of basal and squamous cell skin cancers, actinic keratoses, and premalignant or cancerous skin nevi) Ophthalmologic (cataract) procedures Minor dental procedures (dental extractions, restorations, prosthetics, endodontics), dental cleanings, fillings Pacemaker or cardioverter-defibrillator device implantation

ISTH = International Society on Thrombosis and Haemostasis.

^aMinimal to no residual anticoagulant effect at time of procedure (ie, four to five drug half-life interruptions pre-procedure).

^bIncludes spinal and epidural anesthesia or any other neuraxial (eg, pain management) interventions; consider not only absolute risk for major bleeding but potentially devastating consequences of epidural bleeding and associated lower limb paralysis.

^cSome residual anticoagulant effect allowed (ie, two to three drug half-life interruptions pre-procedure).

^dRadial approach may be considered minimal-bleed-risk compared with femoral approach.

^eProcedure can be safely done under full-dose anticoagulation (may consider holding DOAC dose day of procedure to avoid peak anticoagulant effects).

2. If interruption of anticoagulation therapy is warranted, determine duration of interruption and if bridging is necessary. If patient is on antiplatelet therapy, antiplatelet agents may need to be held as well.

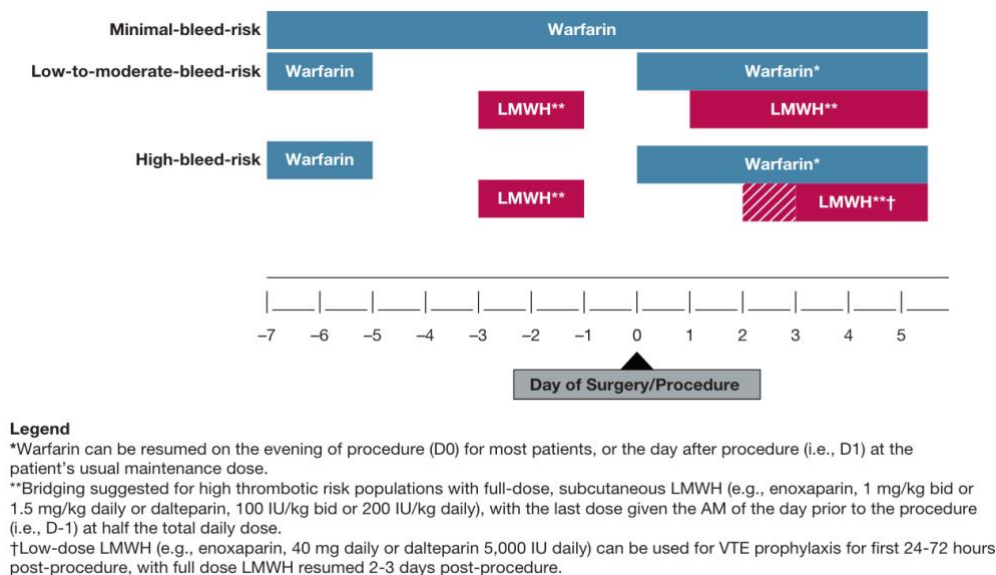


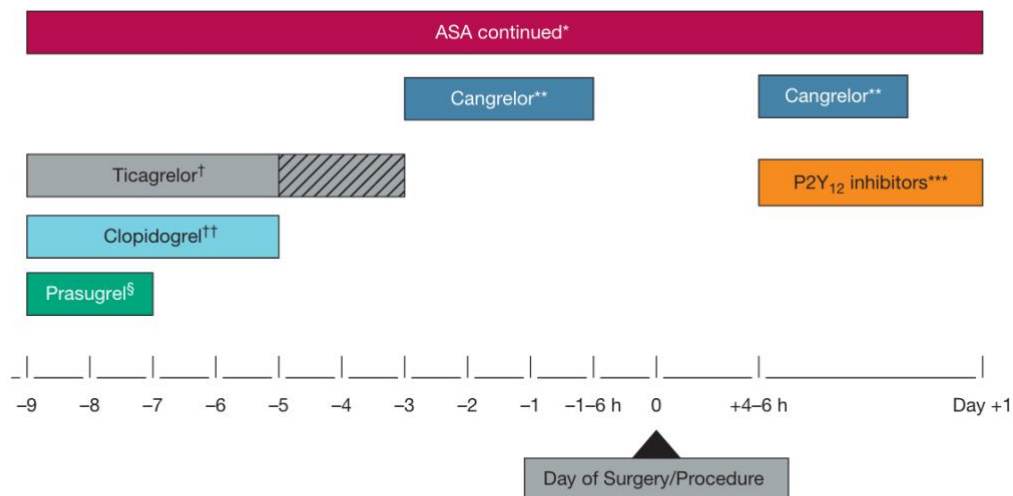
Figure 1 – Perioperative management of vitamin K antagonists (warfarin). LMWH = low-molecular-weight heparin.

Direct Oral Anticoagulant	Procedure Bleeding Risk	Pre-Procedure DOAC Interruption						Surgery/Procedure (Day 0)	Post-Procedure Resumption*			
		Day -6	Day -5	Day -4	Day -3	Day -2	Day -1		Day +1	Day +2	Day +3	Day +4
Apixaban	High											
	Low/Mod											
Dabigatran (CrCl ≥ 50 ml/min)	High											
	Low/Mod											
Dabigatran (CrCl < 50 ml/min)	High											
	Low/Mod											
Edoxaban	High											
	Low/Mod											
Rivaroxaban	High											
	Low/Mod											

 No DOAC administered that day

*DOAC can be resumed ~24 hours after low/moderate-bleed-risk procedures, and 48-72 hours after high-bleed-risk procedures. In selected patients at high risk for VTE, low-dose anticoagulants (i.e., enoxaparin, 40 mg daily or dalteparin, 5,000 IU daily) can be given for the first 48-72 hours post-procedure.

Figure 2 – Perioperative management of direct oral anticoagulants (DOACs). CrCl = creatinine clearance.



Legend:

*Based on surgery/procedure bleed risk assessment.

**Routine use not suggested. If used, initiate within 72 hours from P2Y₁₂ inhibitor discontinuation at dose of 0.75 mg/kg/min; resume within 6 hours post-procedure for minimum of 48 hours and maximum of 7 days total. Very low quality data for antiplatelet bridging with glycoprotein IIb/IIIa inhibitors (e.g., eptifibatide, tirofiban).

***P2Y₁₂ inhibitors can be resumed within 24 hours post-procedure at a maintenance dose.

†For ticagrelor, 3-5 day interruption

††For clopidogrel, 5 day interruption

§For prasugrel, 7-10 day interruption.

Figure 3 – Perioperative management of antiplatelet drugs. ASA = aspirin.

- If bridging is needed, clinician will develop bridge plan using Bridge Plan Maker Excel Workbook located on shared folder.
- Bridge plan tips (variable depending on clinical judgment and specific situations):
 - Bridge plans will **typically** consist of holding warfarin 5 days prior to procedure, bridging with parenteral therapy at therapeutic doses for VTE.
 - LMWH or enoxaparin:

- Once Data is entered, the bridge plan will generate on the “Bridge plan printout” sheet below

	A	B	C	D	E	F
1	test, test			MRN: 111111111		
2	Perioperative Anticoagulation Plan for Colonoscopy on 5/16/2024					
3						
4	Date	Lovenox		Warfarin	Lab	
5		AM	PM			
6	5/11/2024	None	None	None		
7	5/12/2024	80 mg	80 mg	None		
8	5/13/2024	80 mg	80 mg	None		
9	5/14/2024	80 mg	80 mg	None		
10	5/15/2024	80 mg	None	None		
11	SURGERY 5/16/2024	None	None	Previous Dose		
12	5/17/2024	80 mg	80 mg	Previous Dose		
13	5/18/2024	80 mg	80 mg	Previous Dose		
14	5/19/2024	80 mg	80 mg	Previous Dose		
15	5/20/2024	80 mg	80 mg	Previous Dose		
16	5/21/2024	80 mg	80 mg	As Directed	INR + CBC at 1D	
17						
18						
19						
20						
21	**Continue Lovenox until therapeutic one time					
22						
23	**Lovenox does come in 80mg, they will be dispensed in 0.8 mL syringes - Ask the pharmacist to show you the 0.8mL syringe					
24						
25						
26						
27	Prepared by Test, PharmD					
28						
29						
30						
		Instructions	Patient information	Bridge plan printout		

Ready

- Clinician will copy and paste bridge plan as a word document and save in bridge plans folder for future reference. There are also blank bridge plan templates in the folder as well.

test, test			
MRN: 11111111			
Perioperative Anticoagulation Plan for Colonoscopy on 5/16/2024			
Date	<u>Lovenox</u>	Warfarin	Lab
	AM	PM	
5/11/2024	None	None	None
5/12/2024	80 mg	80 mg	None
5/13/2024	80 mg	80 mg	None
5/14/2024	80 mg	80 mg	None
5/15/2024	80 mg	None	None
SURGERY 5/16/2024	None	None	Previous Dose
5/17/2024	80 mg	80 mg	Previous Dose
5/18/2024	80 mg	80 mg	Previous Dose
5/19/2024	80 mg	80 mg	Previous Dose
5/20/2024	80 mg	80 mg	Previous Dose
5/21/2024	80 mg	80 mg	As Directed
			INR + CBC at 1D

****Continue Lovenox until therapeutic one time**

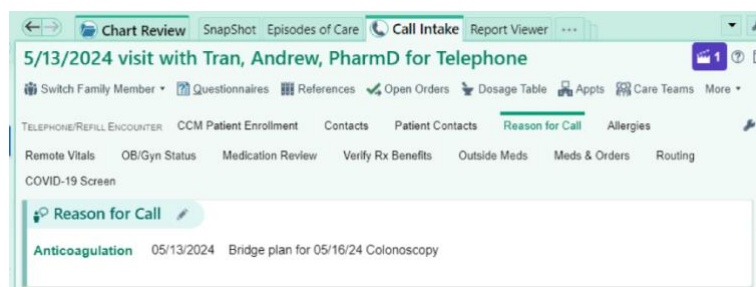
****Lovenox dose comes in 80mg, they will be dispensed in 0.8 mL syringes - Ask the**

Bridge Plans			
File Home Share View			
This PC > \\isilon-data\Data (K) > Anticoagulation Clinic Staff > Bridge Plans >			
Name	Date modified	Type	Size
KACC 2019	12/12/2023 12:10 ...	File folder	
KACC2020	5/6/2024 10:26 AM	File folder	
KACC2021	2/27/2024 10:53 AM	File folder	
KACC2022	4/25/2024 4:55 PM	File folder	
KACC2023	4/4/2024 10:39 AM	File folder	
WACC 2016	9/19/2023 4:16 PM	File folder	
WACC 2017	9/19/2023 4:16 PM	File folder	
WACC 2018	9/19/2023 4:16 PM	File folder	
WACC 2019	11/21/2023 11:33 ...	File folder	
WACC 2020	9/19/2023 4:16 PM	File folder	
WACC 2021	11/13/2023 1:11 PM	File folder	
WACC 2022	11/13/2023 1:33 PM	File folder	
WACC 2023	1/24/2024 9:55 AM	File folder	
WACC 2024	5/1/2024 3:20 PM	File folder	
approaching a BRIDGE patient	7/9/2013 10:55 AM	Adobe Acrobat D...	74 KB
Arixtra bridge template.doc	2/15/2018 9:44 AM	Microsoft Word D...	15 KB
ASA bridge	4/3/2024 3:58 PM	Microsoft Word 9...	36 KB
Bridge Plan Maker	5/6/2024 1:21 PM	Microsoft Excel 97...	74 KB
debug	4/20/2021 4:01 PM	Text Document	1 KB
Lovenox DAILY hrs template	8/31/2020 11:21 AM	Microsoft Word 9...	37 KB
Lovenox Q12 hrs template	6/7/2023 8:59 AM	Microsoft Word 9...	37 KB
Outpatient Bridging for Patients on Warf...	3/13/2023 10:22 AM	Adobe Acrobat D...	109 KB
The Bridge Plan Maker	4/26/2011 12:55 AM	Microsoft PowerP...	1,419 KB
Unfractionated heparin for bridging	2/18/2022 3:53 PM	Microsoft Word D...	13 KB
46 items 1 item selected 37.0 KB			

- Formatting for document name: [Last Name, First name MRN Procedure Date and type]; [i.e. Test, Test 111111_051624 colonoscopy]

Name
Unfractionated heparin for bridging
The Bridge Plan Maker
test, test 111111_051624 colonoscopy
Outpatient Bridging for Patients on Warf...
Lovenox Q12 hrs template
Lovenox DAILY hrs template

8. Clinician will document in the patient's chart under a telephone encounter with peri-operative plan for anticoagulation along with clinical rationale.



Progress Notes

Tran, Andrew, PharmD at 5/13/2024 12:19 PM

Status: Sign at exiting of workspace

CrCl Calculator	
Height(in.):	70
Weight (kg):	80
Age:	50
SCr (mg/dL):	1.2
Male/Female:	Male

IBW:	73.0kg
TBW/IBW:	1.10
Adj. BW:	75.8kg
CrCl (IBW):	76.0mL/min
CrCl (ABW):	79.0mL/min

Test Patient is currently anticoagulation with warfarin for atrial fibrillation with CHA2DS2VASC = 5 (C, H, D, S2) with history of stroke in 2018. For patient's colonoscopy on 05/16/2024 it is recommended to hold warfarin 5 days prior to procedure and bridge with enoxaparin 1 mg/kg every 12 hours. Recommend to hold warfarin starting on 05/11, initiate enoxaparin 80 mg subcutaneous every 12 hours starting 05/12 to 05/15 (morning only). Ok to resume to warfarin evening of procedure 05/16 and start bridging concurrently with enoxaparin 80 mg subcutaneous 12 hours until INR is therapeutic. Post procedure INR +CBC scheduled on 05/21/24.

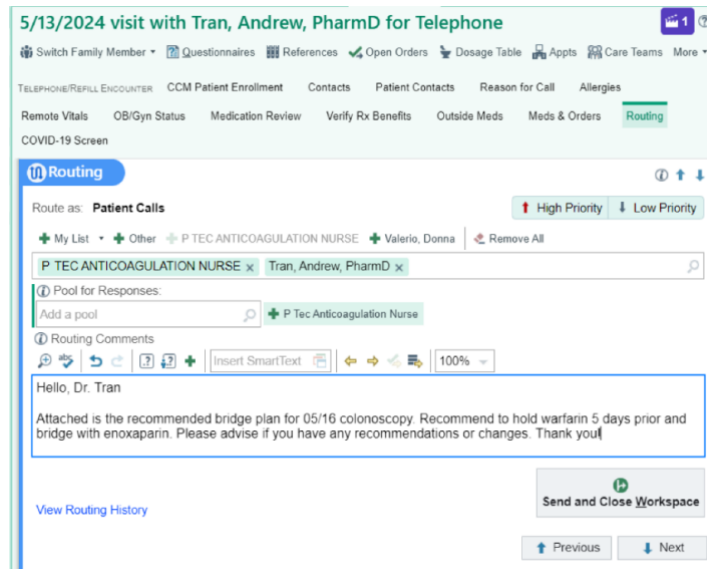
Routed to surgeon for approval and will follow-up with patient once complete.

Test, Test MRN: 111111111

Perioperative Anticoagulation Plan for Colonoscopy on 5/16/2024

Date	Lovenox	Warfarin	Lab
------	---------	----------	-----

9. Clinician will route to surgeon/surgical team, any other applicable parties (i.e. nurse, PCP, attending physicians, etc.) for approval/recommendations. Additionally, route to Triage pharmacist pool to follow up (P TEC ANTICOAGULATION NURSE).



5/13/2024 visit with Tran, Andrew, PharmD for Telephone

Switch Family Member Questionnaires References Open Orders Dosage Table Appts Care Teams More

TELEPHONE/REFILL ENCOUNTER CCM Patient Enrollment Contacts Patient Contacts Reason for Call Allergies

Remote Vitals OB/Gyn Status Medication Review Verify Rx Benefits Outside Meds Meds & Orders **Routing**

COVID-19 Screen

Routing

Route as: **Patient Calls** High Priority Low Priority

My List Other P TEC ANTICOAGULATION NURSE Valerio, Donna Remove All

P TEC ANTICOAGULATION NURSE x Tran, Andrew, PharmD x

Pool for Responses:

Add a pool P Tec Anticoagulation Nurse

Routing Comments

Insert SmartText 100%

Hello, Dr. Tran

Attached is the recommended bridge plan for 05/16 colonoscopy. Recommend to hold warfarin 5 days prior and bridge with enoxaparin. Please advise if you have any recommendations or changes. Thank you!

View Routing History

Send and Close Workspace

Previous Next

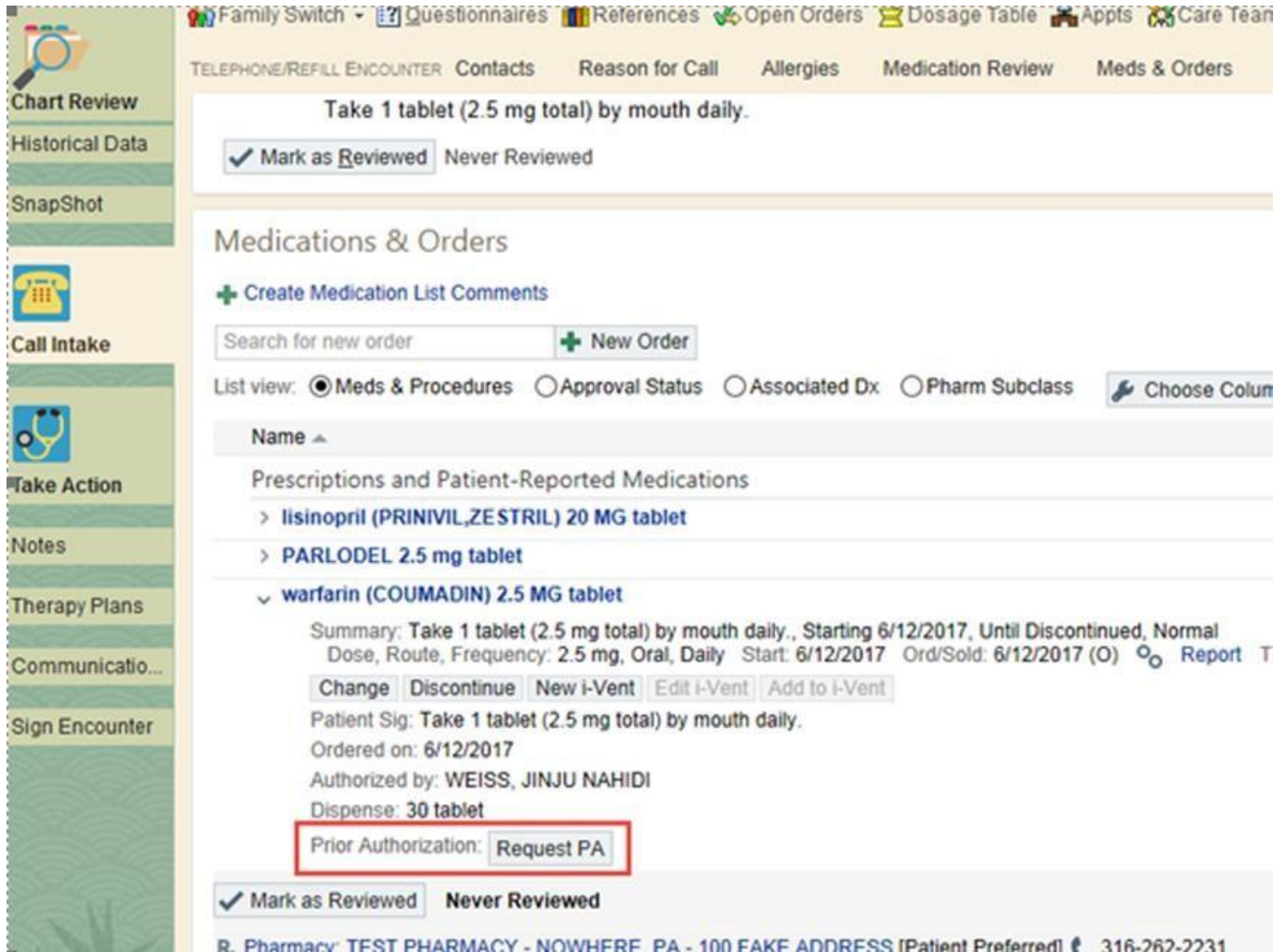
10. Once approved, clinician will discuss bridge plan with patient, order necessary prescription for bridging (LMWH or fondaparinux), and schedule post-procedural lab for INR+CBC. Anticoagulation staff will follow-up as needed.

Douketis, J. D., Spyropoulos, A. C., Murad, M. H., Arcelus, J. I., Dager, W. E., Dunn, A. S., Fargo, R. A., Levy, J. H., Samama, C. M., Shah, S. H., Sherwood, M. W., Tafur, A. J., Tang, L. V., & Moores, L. K. (2022). Perioperative Management of Antithrombotic Therapy: An American College of Chest Physicians Clinical Practice Guideline. *CHEST*, 0(0). <https://doi.org/10.1016/j.chest.2022.07.025>

PRIOR AUTHORIZATIONS

Reference: [ePA via orders only document](#)

- I. Inside the **Meds & Orders** section a prior authorization request can be sent by clicking on the **Prior Authorization** button under the medication information.



Family Switch Questionnaires References Open Orders Dosage Table Appts Care Team

TELEPHONE/REFILL ENCOUNTER Contacts Reason for Call Allergies Medication Review Meds & Orders

Take 1 tablet (2.5 mg total) by mouth daily.

☒ Mark as Reviewed Never Reviewed

Medications & Orders

+ Create Medication List Comments

Search for new order

List view: ☒ Meds & Procedures ☐ Approval Status ☐ Associated Dx ☐ Pharm Subclass

Name ▲

Prescriptions and Patient-Reported Medications

- > lisinopril (PRINIVIL,ZESTRIL) 20 MG tablet
- > PARLODEL 2.5 mg tablet
- ▼ warfarin (COUMADIN) 2.5 MG tablet
 - Summary: Take 1 tablet (2.5 mg total) by mouth daily., Starting 6/12/2017, Until Discontinued, Normal Dose, Route, Frequency: 2.5 mg, Oral, Daily Start: 6/12/2017 Ord/Sold: 6/12/2017 (O)
 -
 - Patient Sig: Take 1 tablet (2.5 mg total) by mouth daily.
 - Ordered on: 6/12/2017
 - Authorized by: WEISS, JINJU NAHIDI
 - Dispense: 30 tablet
 - Prior Authorization:

☒ Mark as Reviewed Never Reviewed

R. Pharmacy: TEST PHARMACY - NOWHERE PA - 100 FAKE ADDRESS [Patient Preferred] ☎ 316-262-2231

- II. It will then ask you to **select a payer**. We will **default** in the **Auto Search Patient's Payer**, which will pull in the **payer from the Rx benefits section in Rooming** if it has been queried recently. *(User may need to re-run the query by clicking Check Again in the Verify Rx Benefits section of the Rooming Activity.)*

Medications & Orders

[+ Create Medication List Comments](#)

Search for new order



New Order

List view: ☒ Meds & Procedures ☐ Approval Status ☐ Associated Dx ☐ Pharm Subclass

[Choose Columns](#)

Name ▲

Dose, Frequency

Prescriptions and Patient-Reported Medications

> lisinopril (PRINIVIL,ZESTRIL) 20 MG tablet

> PARLODEL 2.5 mg tablet

▼ warfarin (COUMADIN) 2.5 MG tablet

Summary: Take 1 tablet (2.5 mg total) by mouth daily., Starting 6/12/2017, Until Discontinued, Normal

Dose, Route, Frequency: 2.5 mg, Oral, Daily Start: 6/12/2017 Ord/Sold: 6/12/2017 (O) [Report](#) Taking: ☐ Long-term: ☐ Pharmacy: TEST PHARMACY - NOWHERE

[Change](#) [Discontinue](#) [New i-Vent](#) [Edit i-Vent](#) [Add to i-Vent](#)

Patient Sig: Take 1 tablet (2.5 mg total) by mouth daily.

Ordered on: 6/12/2017

Authorized by: WEISS, JINJU NAHIDI

Dispense: 30 tablet

Prior Authorization: [Request PA](#)

☒ Mark as Reviewed ☐ Never Reviewed

[Rx Pharmacy: TEST PHARMACY - NOWHERE, PA - 100 FAKE ADDRESS \[Patient Preferred\]](#) [316-262-2231](#)

[Associate](#) [Providers](#) [Current Interactions](#)

Initiate Prior Authorization

Payer:

[AUTO SEARCH PATIENT'S PA](#)

[Accept](#)

[Cancel](#)

- III. Once the order is **signed**, a **request is sent to CoverMyMeds**, who will then send back an authorization questionnaire. This **questionnaire** will be in an **InBasket message** in the RxWorkgroup pool for your department.

Medications & Orders

[+ Create Medication List Comments](#)

Search for new order [+ New Order](#) [+ Patient-Reported Medications](#)

List view: ☐ Meds & Procedures ☐ Associated Dx ☒ Pharm Subclass [Choose Columns](#)

Show: ☒ Summary ☐ Med History ☒ Med Notes

!! New medications from outside sources

Medications need attention. [Go Reconcile](#)

Name ▲	Start	Prior Authorization	
Antiparkinson Therapy - Ergot Alkaloids and Derivatives			✓
> PARLODEL 2.5 mg tablet	6/5/2017	Pending	Report ☐ Change Reorder Discontinue

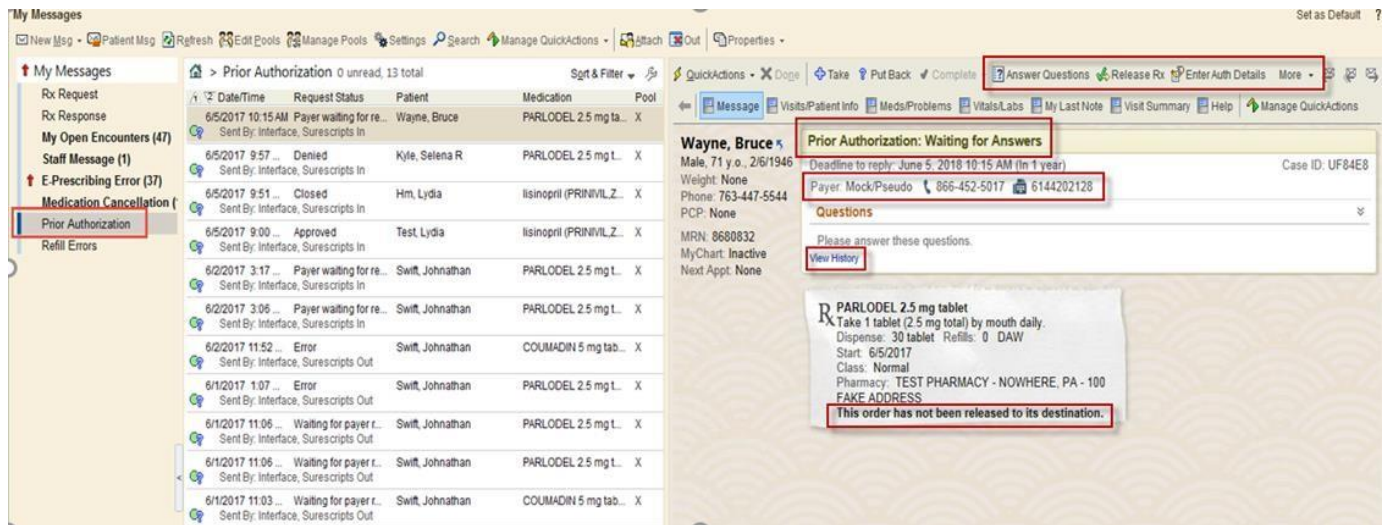
Summary: Take 1 tablet (2.5 mg total) by mouth daily., Starting 6/5/2017, Until Discontinued, Normal

☒ Mark All Taking ☒ Mark as Reviewed ☐ Never Reviewed

[Rx Pharmacy: TEST PHARMACY - NOWHERE, PA - 100 FAKE ADDRESS \[Patient Preferred\]](#) [316-262-2231](#) [Order Entry](#)

[Associate](#) [Providers](#)

- IV. There will be a message in the **Prior Authorization folder** of the In Basket.



My Messages

12 Date/Time Request Status Patient Medication Pool

12	Date/Time	Request Status	Patient	Medication	Pool
6/5/2017 10:15 AM	Payer waiting for re...	Wayne, Bruce	PARLODEL 2.5 mg ta...	X	
6/5/2017 9:57 ...	Denied	Kyle, Selena R	PARLODEL 2.5 mg L...	X	
6/5/2017 9:51 ...	Closed	Hm, Lydia	lisinopril (PRINIVIL...	X	
6/5/2017 9:00 ...	Approved	Test, Lydia	lisinopril (PRINIVIL...	X	
6/2/2017 3:17 ...	Payer waiting for re...	Swift, Johnathan	PARLODEL 2.5 mg L...	X	
6/2/2017 3:06 ...	Payer waiting for re...	Swift, Johnathan	PARLODEL 2.5 mg L...	X	
6/2/2017 11:52 ...	Error	Swift, Johnathan	COUMADIN 5 mg tab...	X	
6/1/2017 1:07 ...	Error	Swift, Johnathan	PARLODEL 2.5 mg L...	X	
6/1/2017 11:06 ...	Waiting for payer r...	Swift, Johnathan	PARLODEL 2.5 mg L...	X	
6/1/2017 11:06 ...	Waiting for payer r...	Swift, Johnathan	PARLODEL 2.5 mg L...	X	
6/1/2017 11:03 ...	Waiting for payer r...	Swift, Johnathan	COUMADIN 5 mg tab...	X	

Wayne, Bruce

Male, 71 y.o., 2/6/1946
Weight: None
Phone: 763-447-5544
PCP: None
MRN: 8680832
MyChart: Inactive
Next Appt: None

Prior Authorization: Waiting for Answers

Deadline to reply: June 5, 2018 10:15 AM (in 1 year)

Payer Mock/Pseudo 866-452-5017 6144202128

Questions

Please answer these questions.

View History

PARLODEL 2.5 mg tablet
Take 1 tablet (2.5 mg total) by mouth daily.
Dispense: 30 tablet Refills: 0 DAW
Start: 6/5/2017
Class: Normal
Pharmacy: TEST PHARMACY - NOWHERE, PA - 100
FAKE ADDRESS
This order has not been released to its destination.

- V. If the message states **"Prior Authorization Waiting for Answers"**, you can:
- Contact the Payer directly
 - Release the Rx manually
 - Click the **More** drop down, and select **Change Payer** if you need to put that information in later.
 - Click **Answer Questions** and you will be **taken to Chart Review** to have more resources available to you to answer the questions that appear in your sidebar. **Respond** to the questions and/or click **Enter Auth Details**. Once you click **Send to Payer**, a message stating either Approved, Denied, or Closed will come to your **Prior Authorization** folder in In Basket.
- VI. **Chart Review > Meds tab** provides you with more information, including the ePA status from specific encounters.

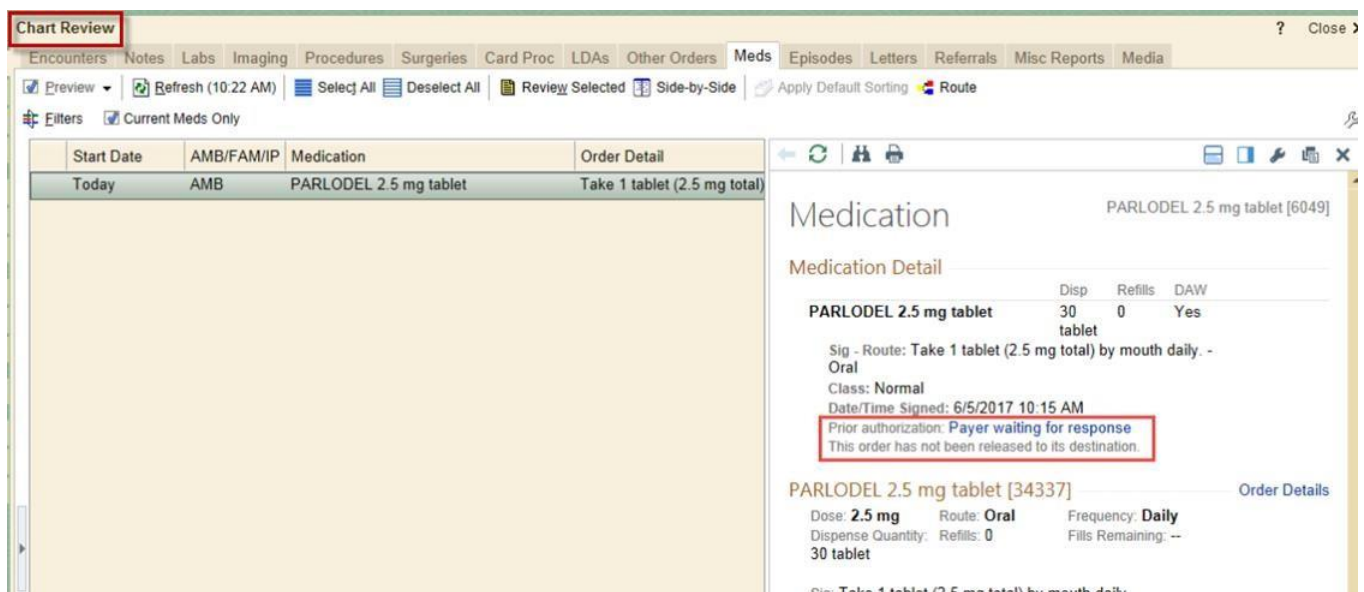


Chart Review

Encounters Notes Labs Imaging Procedures Surgeries Card Proc LDAs Other Orders **Meds** Episodes Letters Referrals Misc Reports Media

Preview Refresh (10:22 AM) Select All Deselect All Review Selected Side-by-Side Apply Default Sorting Route

Filters Current Meds Only

Start Date	AMB/FAM/IP	Medication	Order Detail
Today	AMB	PARLODEL 2.5 mg tablet	Take 1 tablet (2.5 mg total)

Medication PARLODEL 2.5 mg tablet [6049]

Medication Detail

	Disp	Refills	DAW
PARLODEL 2.5 mg tablet	30 tablet	0	Yes

Sig - Route: Take 1 tablet (2.5 mg total) by mouth daily. - Oral

Class: Normal

Date/Time Signed: 6/5/2017 10:15 AM

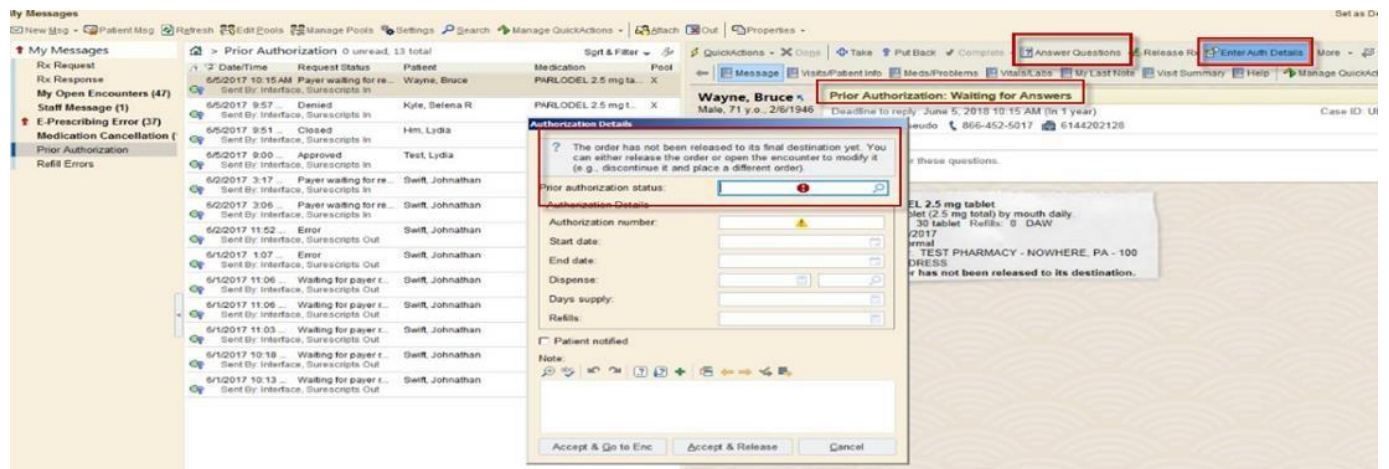
Prior authorization: Payer waiting for response
This order has not been released to its destination.

PARLODEL 2.5 mg tablet [34337] Order Details

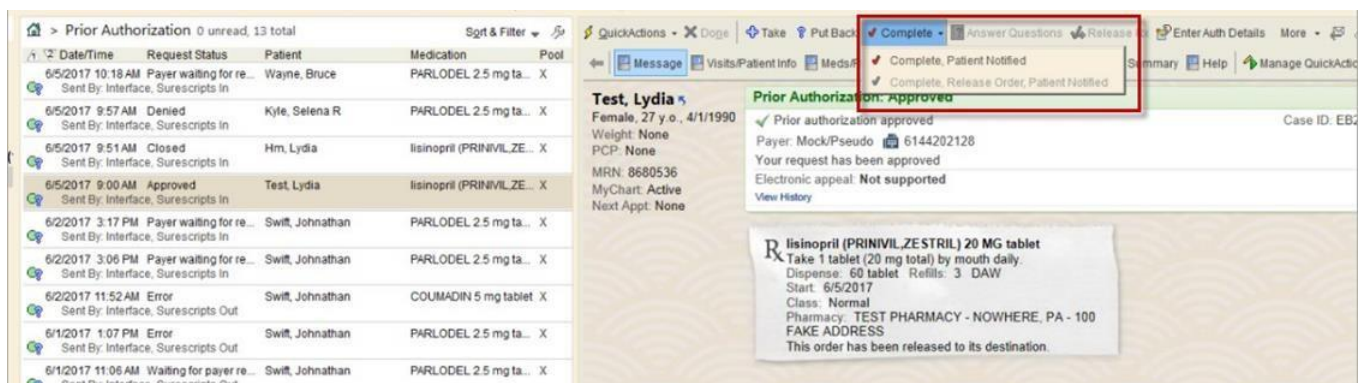
Dose: 2.5 mg Route: Oral Frequency: Daily
Dispense Quantity: 30 tablet Refills: 0 Fills Remaining: --

Sig: Take 1 tablet (2.5 mg total) by mouth daily

- VII. In the **Authorization Details** pop-up window back in In Basket, **document** if prior authorization was obtained **outside the electronic process**, as well as any other details that need to be documented. (i.e. Call the patient on "date") You will need to select the **status** for that order. The order has not been released to its final destination yet. You can either **release the order** manually or open the encounter to **modify** it.



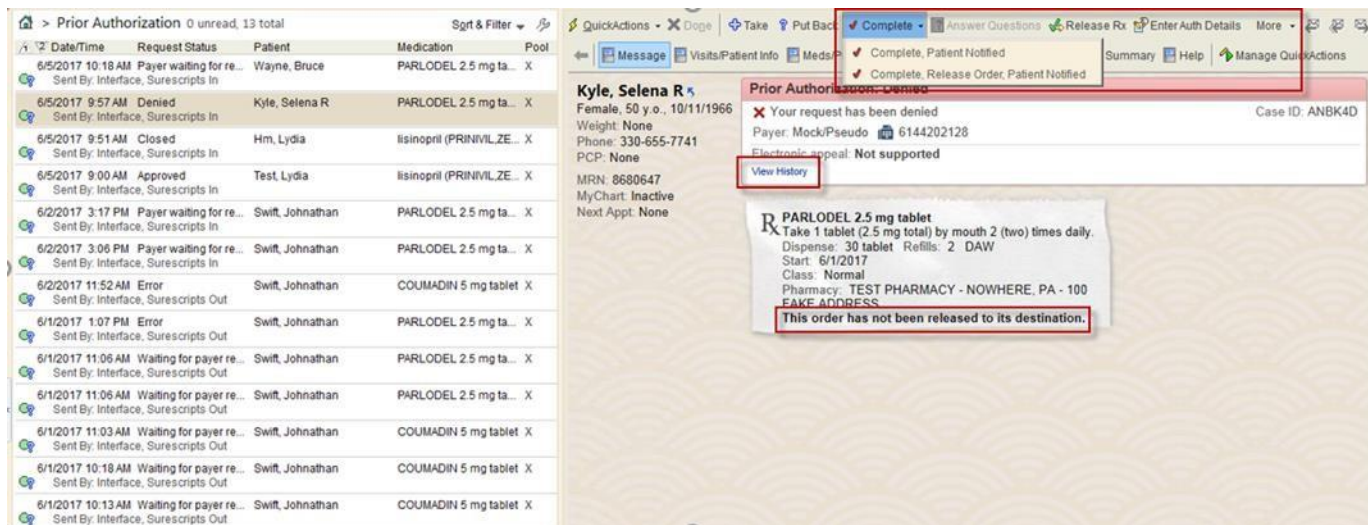
- VIII. Once the ePA is approved, the banner turns green and you can click Complete. The order is released.
- IX. Contact the patient to let them know, or click the Patient Notified check box and add a comment in the window with details (ex: date).
- X. The message will disappear from your In Basket when you click **Complete**.
- XI. You have the option of clicking **Complete**, **Patient Notified**, or **Complete, Release Order, Patient Notified**. *Note: if the order has been released, click the 1st option.*



- XII. If the ePA is denied you can release it manually by clicking **Release Rx**, or go through the **appeals** process. There is also a **Prior Authorization Details** button that will contain information that was sent by the payer to Cover My Meds. You may need to look under the **More** button to find it.



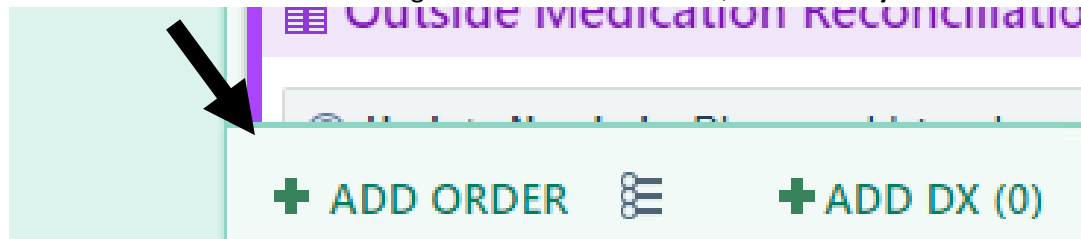
- XIII. Discontinue the medication and/or re-order an alternative (can be done in Orders Only encounter). You can also click **View History** for information about reasons for denial.



The screenshot displays the Prior Authorization interface. On the left, a table lists various requests with columns for Date/Time, Request Status, Patient, Medication, and Pool. The right pane shows details for a denied request for Kyle, Selena R. A red box highlights the 'Complete' button in the top right. Another red box highlights the 'View History' button. A third red box highlights the denial message: 'Your request has been denied. Payer: Mock/Pseudo. Electronic appeal: Not supported.' Below this, a red box highlights the text: 'This order has not been released to its destination.'

PREScription ASSISTANCE

- A. FOR BSW QA Patients: If patient needs prescription assistance for DOAC therapy, a referral can be placed to Community Health Workers (CHWs) or Medication Access Team.
- B. **Community Health Workers (CHWs) can assist patients with the PAP application process**
 - Place a referral order to the following: **Amb Referral to BSWQA Community Health Worker**



- Designate "Medication Affordability" as the reason for referral
- Include any additional information in the "comments" box, i.e. name of the manufacturer PAP
 - May use the following Smartphrase: **.TEMPLEANTICOAGCLINICPAP**

Amb Referral to BSWQA Community Health Worker Accept Cancel

Class: **Internal Referral** Internal Referral External Referral

Referral: ☐ Override Restrictions

To Geographic Areas: + Add

Default Areas ▼

To Department:

To Provider:

Priority: Urgent Expedited **Routine**

Reason For Referral: + Add ☐ Chronic Disease Support ☐ Self Management Support ☐ Community Resources Support ☐ Med Education ☐ Medication Affordability ☐ Home Health Care ☐ DME ☐ Palliative Care ☐ Hospice ☐ SICP/ACP ☐ Other

Comments: abc ↶ ↷ ? + Insert SmartText ↶ ↷ 100%

Please give any information that you think is important to guide care of patient.

- Select “Per Standing Delegated Order: no cosign needed”
- Click options, then enter the PCP as the “ordering provider”
- Click “Sign Orders” to complete the referral process

Admissions on 2/20/24 and 3/23/24

Dx Association Edit Multiple Estimate Options

Per Standing Delegated Order: no cosign required ▼

Outpatient

Amb Referral to BSWQA Community Health Worker

Internal Referral, Routine, Routine

Rx Dallas Gaston Drive-Thru - Baylor Scott & White Pharmacy (#111)

214-818-1611

✓ Approve All ✗ Refuse All □ Uncheck All

PRINT AVS PEND ✓ SIGN ORDERS

Providers ×

Ordering Information

Order mode ⚙

Per Standing Delegated Order: no cosign required

Ordering provider ⚙

Authorizing Providers

For procedures ⚙

For medications ⚙

Entry Comments

Accept Cancel

C. For Non BSW QA patients.

- Medication Access Rx Team Information.
 - Main Phone: 254-215-9946 (Med Access Rx)
 - Denise Larson (Pharmacy Technician) - P: 254-724-1757
 - Lorena Gonzalez (Pharmacy Technician) - P: 254-215-9021
 - RightFax: 254-215-9501 (Patient Rx Solution CTX Fax)
 - Alt Fax: 254-724-0986
 - Email: MedAccessRx@BSWHealth.org
 - Epic Referrals: Ambulatory Referral to Patient Prescription Solutions – PPS Pharmacy

Amb Referral to Patient Prescription Solutions - PPS Pharmacy ✓ Accept ✗ Cancel

Class: **Internal Referral** External Referral

Referral: ☐ Override Restrictions

To Geographic Areas: + Add **✓ TEMPLE** Default Areas ▾

To Department:

To Provider:

Priority:

What is this referral for:

Comments:

Next Required ✓ Accept ✗ Cancel

Dx Association **Options** ▾

Per Standing Delegated Order ▾

Amb Referral to Patient Prescription Solutions - PPS Pharmacy

Rx

LEVEL OF SERVICE **PEND** **✓ SIGN ORDERS** ▴

SELF-TEST REQUESTS

Self-test eligibility

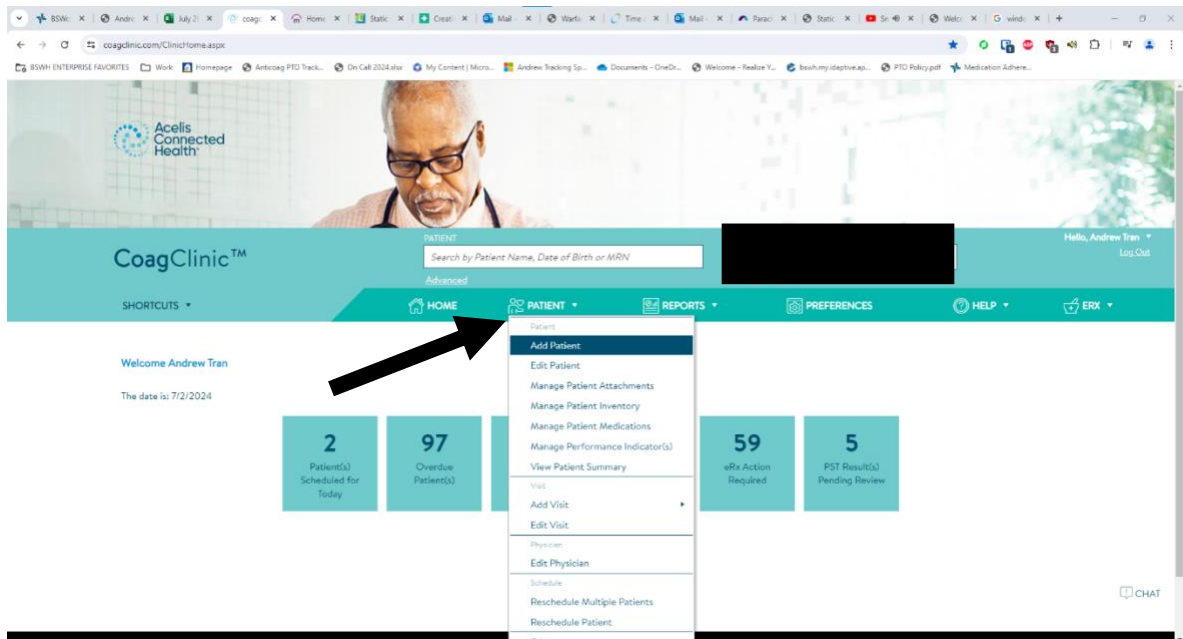
- A. Patients have to meet certain criteria to be considered for home self-testing. This is subject to clinical and professional judgment but we have some guidelines.
- B. The following patients should be considered for self-testing:
 - a. Have difficulty getting time off from work.
 - b. Have to rely on a caregiver for transportation.
 - c. Have a long drive to physician's office or lab or clinic (>30 miles).
 - d. Travel frequently.
 - e. Home-bound.
- C. Criteria for Home INR Monitor include:
 - a. Has been on warfarin for 90 days or more.
 - b. Demonstrate good compliance with anticoagulation management.
 - c. Stable INRs.
 - d. Manual and visual dexterity to perform testing or have a committed support person to assist with testing.
 - e. Insurance coverage.

- f. Has to be approved by managing clinic.
- g. Not recommended for patients who have APLA syndrome.

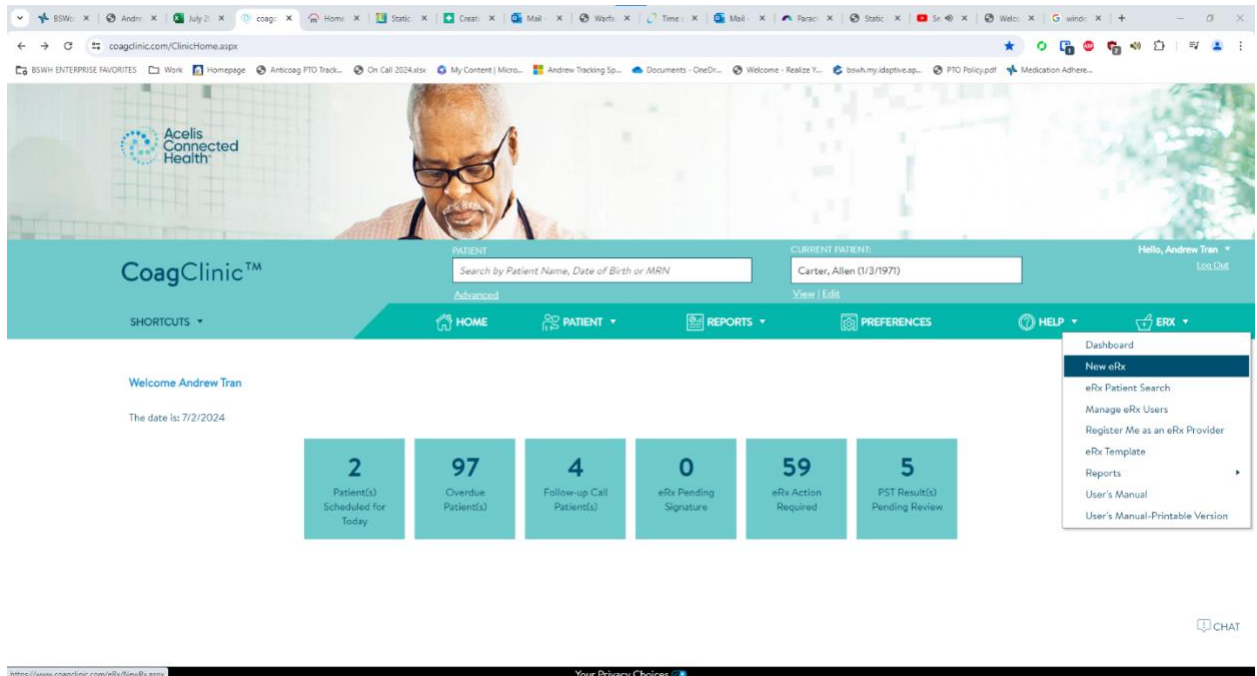
D. If patient is appropriate, we may enroll them for self-test companies such as Acellis, MDINR, or RCS.

ACELIS PRESCRIPTION

- A. Go to <https://coagclinic.com>
- B. If patient isn't already in the CoagClinic portal, they must be added. Go Under **Patient** and click **ADD Patient**.



- C. Fill out all the necessary information that has an asterisk*:
 - a. Patient Last name, First name, Age, Birth Sex, Referring Physician, Patient Office/Location, Home – Phone number and insurance information.
- D. Click Save when done.
- E. If patient is already in the portal, go to **ERX** and click **New eRX**



F. In ACH eRX, check **INR** under ACH Services.

Prescription Date *

7/2/2024

ACH Services *

☒ INR ☐ VAD Equipment

☐ VAD Supplies ☐ VADWatch

PATIENT DEMOGRAPHIC INFORMATION

G. Under **PATIENT DEMOGRAPHIC INFORMATION**, search your patient and click **FILL PATIENT INFORMATION** and it'll autopopulate the available information.

PATIENT DEMOGRAPHIC INFORMATION

Patient

DOB *

Birth Sex *

FILL PATIENT INFORMATION

Last Name *

H. You will need insurance information.

I. The insurance information can be found on the left side of the patient's EPIC chart in **Chart Review**. Hover over **Coverage** and it'll present the information.

COVERAGE PRIME TEXAS		Primary Coverage			
PHARMACY COVERAGE(S) RX BCBS OF TEXAS (011552) (BCTX)		Payer BCBS	Plan BCBS OF TX PPO/POS	Sponsor Code [REDACTED]	Group Name [REDACTED]
[REDACTED]		Primary Subscriber			
ID	Name	SSN	Address		
[REDACTED]					

- J. Back on Acelis, you can click **Manual Data Entry** to add the insurance information you got from EPIC.

PATIENT INSURANCE INFORMATION

Insurance is required. Please select at least one option (Upload Form or Manual Data Entry) to complete this prescription.

Submission Type *

☐ Upload Form

☒ Manual Data Entry

PRIMARY INSURANCE

Insurance Provider *

Name Of Insured (If Different)

Date Of Birth(MM/DD/YYYY)

Street 1

Street 2

City

State

Zip

Insurance Phone

Fax

Insurance Policy ID *

Insurance Group ID

- K. Make sure to fill in prescriber information as Dr. Halka or Tiffany Brown then click **Next**

PRESCRIBING AUTHORITY INFORMATION

Prescriber Name *

Search by Last Name

Brown, Tiffany

Halka, Kathleen

Kimball, Holly

NPI #

Street 2

State

Zip

Phone

Fax

NEXT

SAVE FOR LATER

- L. Make sure to add Account Location as Temple Coumadin Clinic. Add EMR and MRN relevant ICD-10 codes that are found in EPIC.

Account Location *

BSW Health / Scott & White - Temple Coumadin Clinic

EMR MRN

QUALIFYING DIAGNOSIS

ICD10 *

Long term (current) use of anticoagulants / Z79.01

OTHER DIAGNOSIS

ICD10

Presence of prosthetic heart valve / Z95.2

ICD10

Search by code or description

ICD10

Search by code or description

Warfarin Start Date *

- M. Now fill in the necessary information: Warfarin Start Date, Fill with prescriber information, Target INR range for the patient, Alert me when INR is (typically 1.3-5.0), Training arranged by Acelis Connected Health, and Test Frequency (weekly or two-four times per month). When done, click **Next**.

OTHER DIAGNOSIS

ICD10

Presence of prosthetic heart valve / Z95.2

ICD10

Search by code or description

ICD10

Search by code or description

Warfarin Start Date *

MANAGING AUTHORIZED HEALTHCARE PROVIDER

FILL WITH PRESCRIBER INFORMATION

Phone *

(254) 724-7123

Name *

Halka, Kathleen

Fax *

(254) 724-7946

PATIENT MANAGEMENT PREFERENCES

Acelis Connected Health will receive test results directly from the patient, fax ALL results and transfer calls with values <1.5 and >5.0 unless Authorized Healthcare Provider Office Preferences are requested.

Target INR Range *

2.0

to

3.0

Alert Me When INR Is:

1.3

and

5.0

Training *

Training arranged by Acelis Connected Health

Test Frequency *

Two to Four Times Per Month

- N. The next page will let you confirm all the information. If correct, Check **The patient has been evaluated based on following criteria**. Under **signed by**, put your log-in information and then click **Save**.

☒ The patient has been evaluated based on following criteria:

STATEMENT OF MEDICAL NECESSITY/ PRESCRIPTION

Equipment and supplies may be provided by either Acellis Connected Health or its third party vendors. Incomplete items will revert to Authorized Healthcare Provider Office Preferences. I acknowledge that to initiate INR self-testing for the patient can take more than five (5) calendar days from the date of this prescription. My patient has consented to be contacted by Acellis Connected Health Services.

ITEMS PRESCRIBED: One (1) Home INR Monitoring System, and related testing materials (i.e. Test Strips and Lancets). I certify that it is medically necessary for the patient to self-test frequently in order to maintain a stable INR, optimize its therapeutic effects and avoid the complications identified on warfarin's product labeling.

Medical Necessity: I further certify that the patient's medical record contains supporting documentation to substantiate this medical need I certify that this patient has been on warfarin therapy for >90 days. The patient or their caregiver has no condition that makes self-testing unsafe (e.g. cognitive disorders). I agree to notify Acellis Connected Health Services if the patient or their caregiver develops a condition that makes self-testing unsafe.

eRx submitted by users who are not registered as physicians in the system will appear in the eRx dashboard with 'Pending Physician Signature' status. Physician signature is required to complete the prescription.

Signed By:

User ID *

A.Tran

Password *

SAVE

CANCEL

SAVE FOR LATER

- O. Now on the dashboard, the completed order should be under **pending signature** for provider to sign. Notify provider that prescription is ready for them to sign. Ensure patient is aware that we have started the process.

Welcome to eRx!

The date is: 7/2/2024



Pending Signature

Date Range:

Bulk Actions:

Page size: 25

1 items in 1 pages

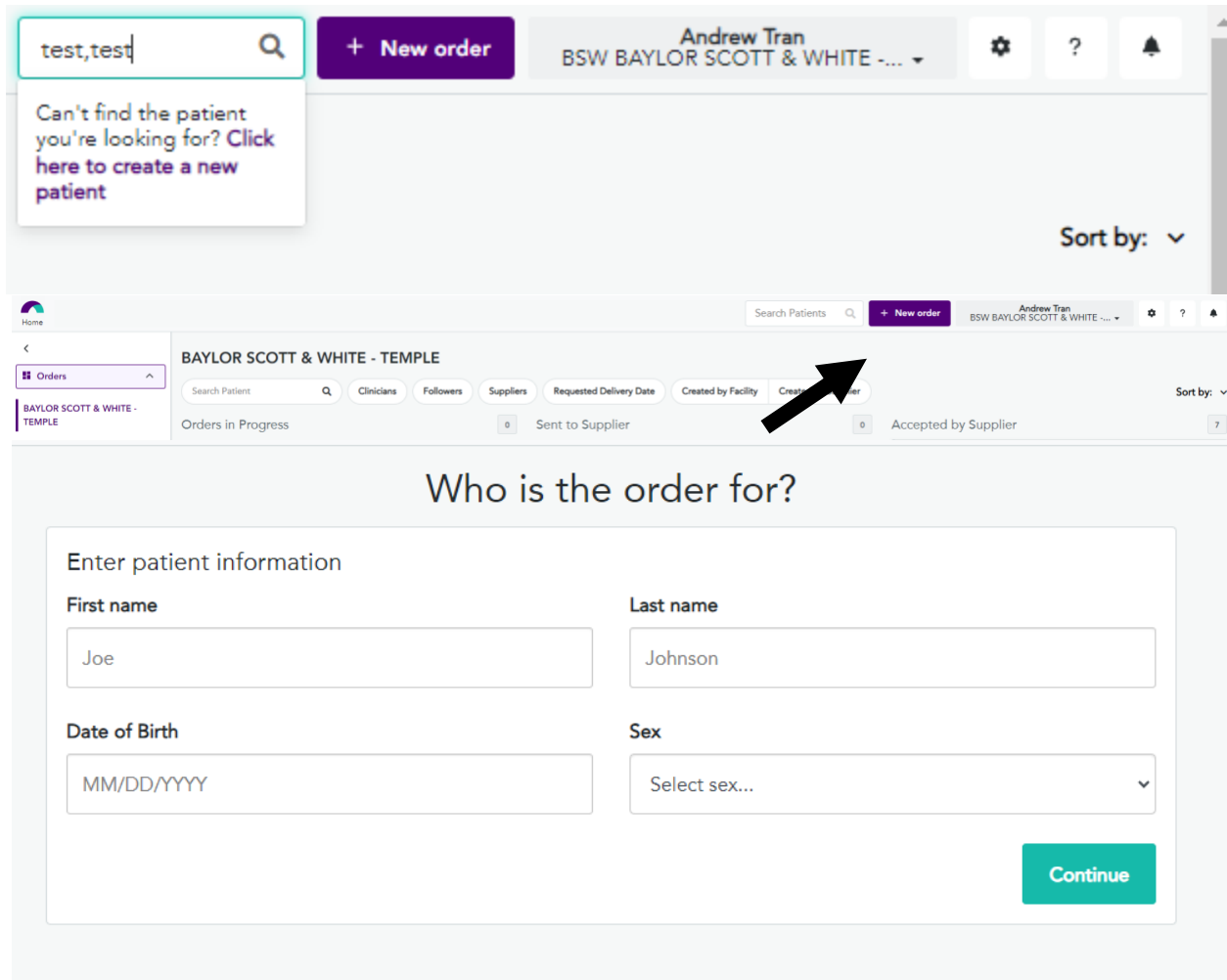
Patient	Program	Rx Date	Status	Status Date	Prescription Information	Created By	Physician
	INR	7/2/2024	Pending Physician Signature	7/2/2024	Qualifying Dx: Long term (current) use of anticoagulants / Z79.01 INR: 2 - 3 Test Frequency: Two to Four Times Per Month Training: Acellis will train	Tran, Andrew	Halika, Kathleen

Page size: 25

1 items in 1 pages

MDINR/RCS - PARACHUTE

- Go to <https://dme.parachutehealth.com/>
- Log in with your credentials.
- Click on **New Order** and then fill out the patient information.
- If new patient, you have to add the patient.



test, test

Can't find the patient you're looking for? Click here to create a new patient

Sort by: ▾

Home

Search Patients

+ New order

Andrew Tran
BSW BAYLOR SCOTT & WHITE -...

BAYLOR SCOTT & WHITE - TEMPLE

Search Patient

Clinicians Followers Suppliers Requested Delivery Date Created by Facility Create New Patient

Orders in Progress

Sent to Supplier

Accepted by Supplier

Who is the order for?

Enter patient information

First name

Joe

Last name

Johnson

Date of Birth

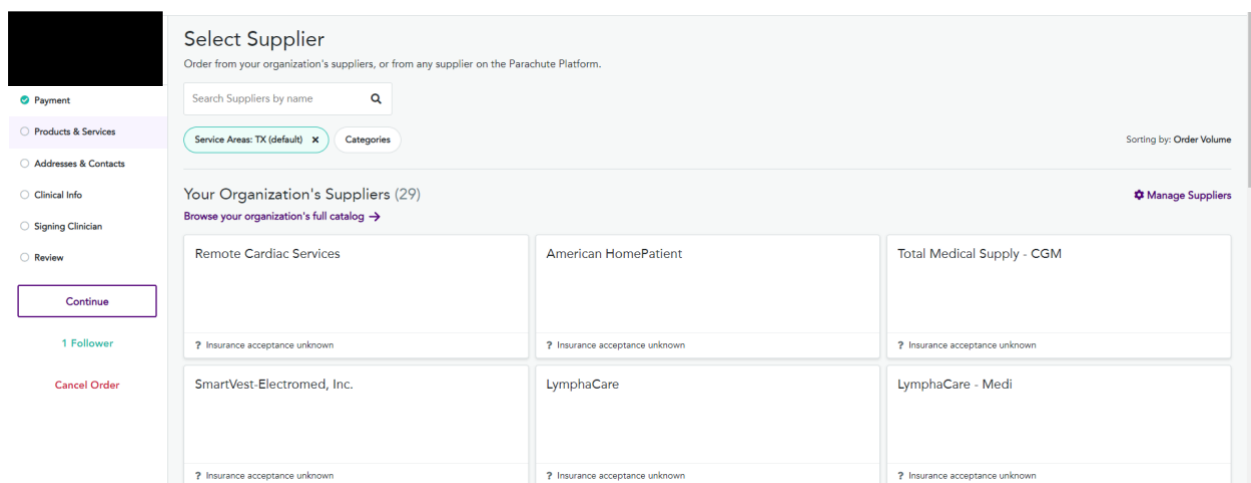
MM/DD/YYYY

Sex

Select sex...

Continue

- E. Select the preferred supplier Remote cardiac services for RCS and AmericanHomePatient for MDINR. The process will be similar for both.



Select Supplier

Order from your organization's suppliers, or from any supplier on the Parachute Platform.

Search Suppliers by name

Service Areas: TX (default) X Categories

Sorting by: Order Volume

Your Organization's Suppliers (29)

Browse your organization's full catalog →

Remote Cardiac Services	American HomePatient	Total Medical Supply - CGM
? Insurance acceptance unknown	? Insurance acceptance unknown	? Insurance acceptance unknown
SmartVest-Electromed, Inc.	LymphaCare	LymphaCare - Medi
? Insurance acceptance unknown	? Insurance acceptance unknown	? Insurance acceptance unknown

Manage Suppliers

Continue

1 Follower

Cancel Order

- F. Under **Testing& Monitoring** for either company, you will find Home Prothrombin Time (PT/INR) Monitoring.

Payment

Products & Services

Addresses & Contacts

Clinical Info

Signing Clinician

Review

Continue

1 Follower

Cancel Order

Give us feedback

Select Supplier

American HomePatient

Heads up! We don't know if the patient's insurance is accepted by this supplier

All Categories

Ambulatory

Continued Use & Benefit

CPAP & BiLevel

Enteral

High Frequency Chest Wall Oscillation (HFC...

Hospital Beds

Mobility

Nebulizer

O2 Recert and Revision

Oxygen

Service Requests

Suction/Trach

Testing & Monitoring

OSA Monitoring

Overnight Oximetry

Enter product name or ID... (e.g., oxygen concentrator or E1390)

Overnight Oximetry Test

Home Prothrombin Time (PT/INR) Monitoring

Oral Appliance Therapy

Help Center

Payment

Products & Services

Addresses & Contacts

Clinical Info

Signing Clinician

Review

Continue

1 Follower

Cancel Order

Select Supplier

Remote Cardiac Services

Heads up! We don't know if the patient's insurance is accepted by this supplier

All Categories

Testing & Monitoring

Enter product name or ID... (e.g., oxygen concentrator or E1390)

Home Prothrombin Time (PT/INR) Monitoring

1 / 1

G. Complete the questions using patient's epic chart to confirm allergies, treatment for active infection, fax number, phone number, fax option: fax every result, fax and live call; no voicemail, patient has been on warfarin for 90 days or longer, training by company, INR range, and INR panic values.

BSWH Anticoagulation Protocol and Procedure – Last updated: January 2026
Developed By Andrew Tran, Clinical Pharmacist III

Prescription Details

Does the patient have any known allergies?

☐ Yes

☒ No

Is the patient being treated for an active infection?

☐ Yes

☒ No

What fax number should test results be sent to?

2547247946

☐ not known/physician to fill in

In the case that a patient's test result falls in the 'panic' category, what phone number should be called to report?

2547247123

☐ not known/physician to fill in

Save Press Enter ↵ or Tab →

If you are not the prescribing physician/part of the physician's team, leave the rest of the questions blank and press the 'x' in the corner to continue. Otherwise proceed.

☒ Understood

Fax Option

☒ Fax Every Result

☐ Only Fax Out of Range Results

☐ Fax Out of Range + Monthly Summary

Notification of Panic Values

☐ Fax and phone call, voicemail allowed

☒ Fax and Live call, no voicemail

Has the patient been on Warfarin for 90 days or longer?

☒ Yes

☐ No

Patient Training

☒ supplier's IDTF

☐ physician

H. You can also add any additional instructions (such as testing duration or replace old machine). Click **Continue** when done.

Patient Training

☒ supplier's IDTF

☐ physician

Lower Target Range Value

2

Upper Target Range Value

3

Panic Value Below (s)

1.4

Panic Value Above (z)

5

Patient's condition requires long-term Warfarin therapy to reduce the risks of thromboembolism. I am ordering PT/INR self-testing service to enable this patient to test more frequently in order to help maintain a stable INR. The patient or patient's care-giver is capable of performing these tests, understanding implications of the test results, and contacting INR services as directed. I believe that patient self-testing is reasonable and necessary for this patient. If you require additional information, please contact me. This service is for weekly self-testing and reporting of test results.

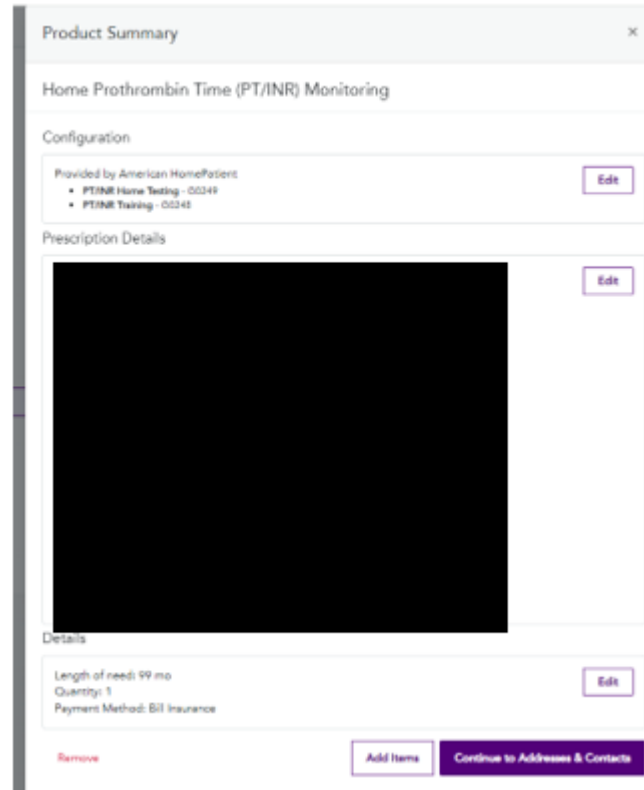
☒ I agree

Additional instructions

☐ not applicable

Continue

- I. If everything is correct, click **Continue To Addresses And Contacts**.



Product Summary

Home Prothrombin Time (PT/INR) Monitoring

Configuration

Provided by American HomePatient

- PT/INR Home Testing - 00349
- PT/INR Testing - 00348

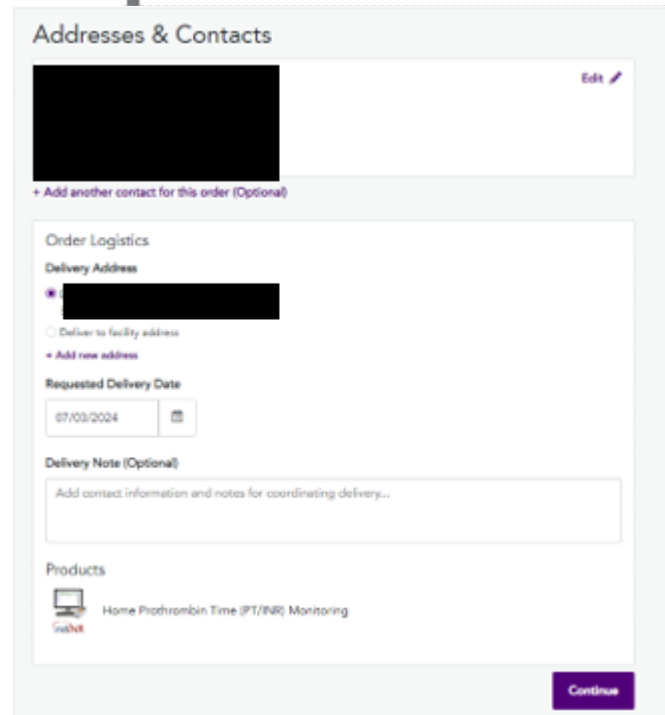
Prescription Details

Details

Length of need: 99 mo
Quantity: 1
Payment Method: Bill Insurance

[Remove](#) [Add Items](#) [Continue to Addresses & Contacts](#)

- J. Confirm delivery address is correct (most of the time should go to patient) and click **Continue**.



Addresses & Contacts

[Edit](#)

+ Add another contact for this order (Optional)

Order Logistics

Delivery Address

☒ [Add new address](#)

☐ Deliver to facility address

Requested Delivery Date

07/03/2024

Delivery Note (Optional)

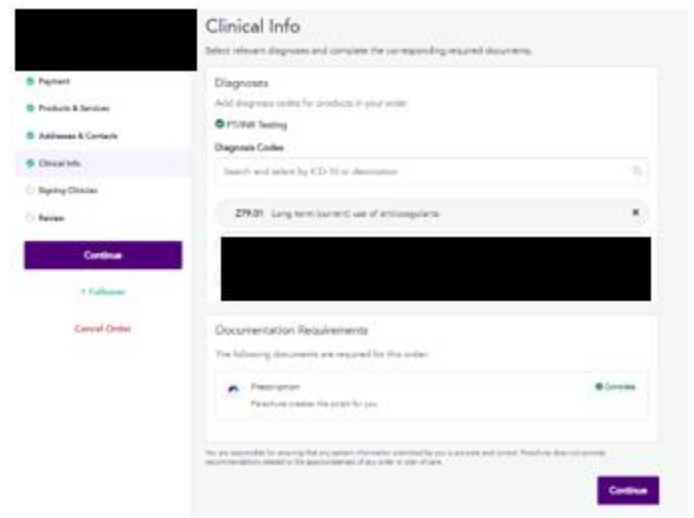
Add contact information and notes for coordinating delivery...

Products

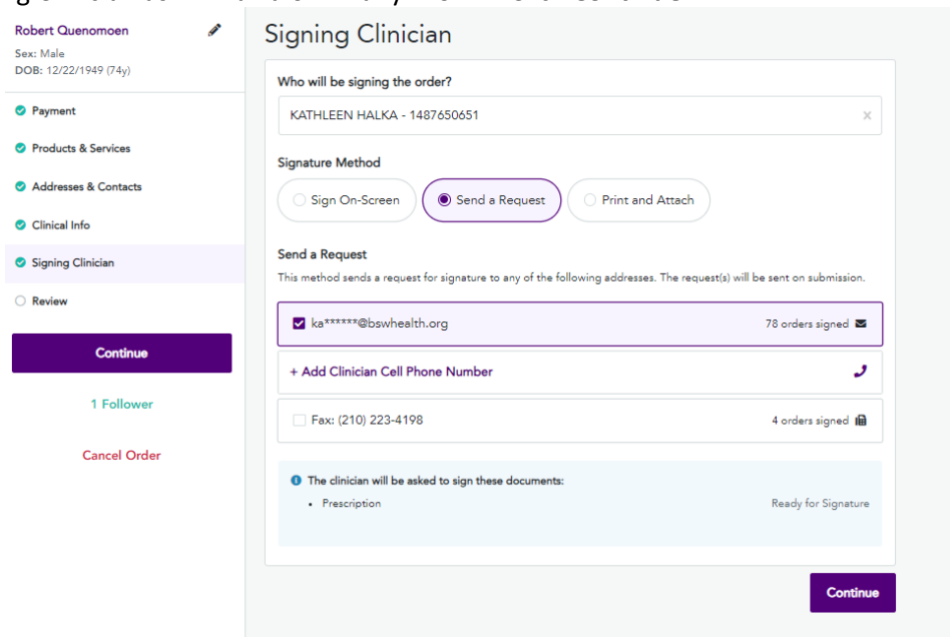
 Home Prothrombin Time (PT/INR) Monitoring

[Continue](#)

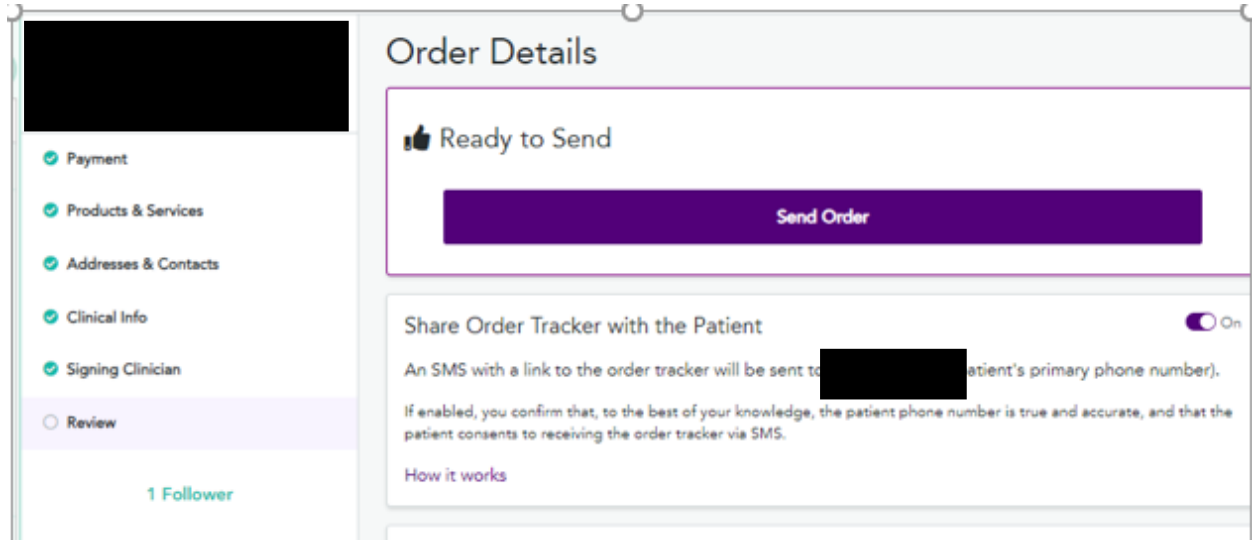
- K. Ensure the correct diagnosis codes are selected and click **Continue**.



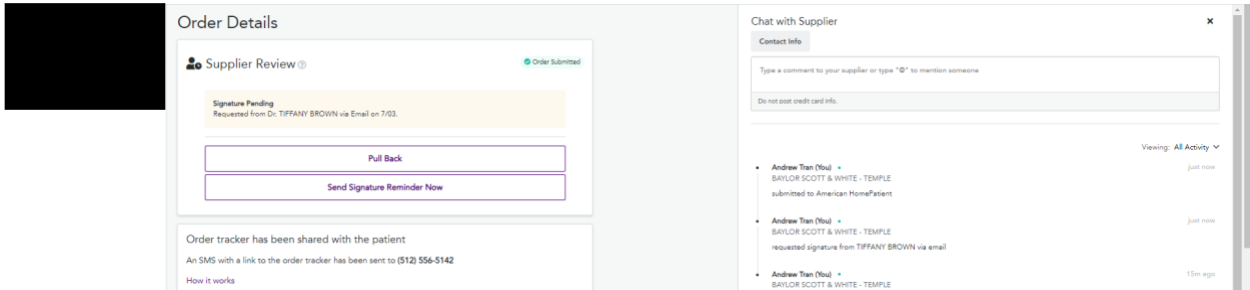
- L. Select the Signing Clinician as Dr. Halka or Tiffany Brown. Click **Continue**.



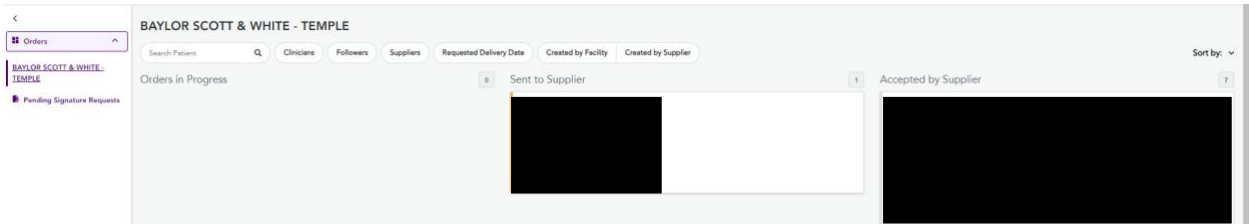
- M. When ready, click **Send Order** and patient will get tracker information once approved.



- N. Before the order finalizes, it can still be pulled back but otherwise you can keep track of the progress. Let provider know to sign off on the order.



- O. Also back on the dashboard, you can keep track of the other orders on Parachute.



XII. ON-CALL RESPONSIBILITIES

- I. Weekdays – Critical lab results (INR > 4) reported between 5-11 PM are paged into Everbridge app to the pharmacist (on-weekends) and nurses (on weeknights). After 11 PM, results will be called to inpatient pharmacy.
- II. Weekends/Holidays – On-call pharmacists will work on INR patients that need dosing over the weekend and may assist with the hospital follow-up if needed.

XIII. NON-COMPLIANCE/NON-ADHERENCE PROCESS

Non-Compliance

A non-compliant patient is a patient who does not follow a course of treatment plan or recommendation. Examples may include followup appointments, labs, imaging, testing, etc.



It is ok for the patient to choose not to follow the recommended plan of treatment. Document the patient's chart that the risks/benefits have been discussed with the patient and they understand.

When faced with a non-compliant patient:

- use empathic listening skills to understand the reason for non-compliance,
- ask the patient direct questions to review their understanding of the treatment plan, and
- reach out for support from colleagues or patient family members.

1st Warning - Discuss the importance of being compliant. Document the conversation. Give the patient reasonable amount of time to be compliant (30-60 days).

2nd Warning - Document the non-compliance in the patient's chart. Draft a non-compliance warning letter (use template below). Give the warning letter to the provider for review and signature. Provider will sign the warning letter. Add certified number to the letter. Make a copy of the letter. Scan a copy of the warning (should have signature and certified number) into patient's chart under the media tab, labeled warning letter. Mail the letter via certified mail and regular mail.

[Physician Warning Letter - Non Compliance](#)

Termination – Document non-compliance in detail in patient's chart. Draft a non-compliance termination letter (use template letter). Add the date of the warning letter to the non-compliance termination letter. Give the termination letter to the provider for review and signature. Provider will sign the termination letter. Add certified number to the letter. Make a copy of the letter. Scan a copy of the termination letter (should have signature and certified number) into patient's chart under the media tab, label as termination letter. Mail the letter via certified mail and regular mail.

[Physician Term Letter - Non Compliance](#)

Once the termination letter has been mailed & scanned into the chart, you can dismiss the patient in EPIC.

Once the termination letter has been mailed & scanned into the chart, you can dismiss the patient in EPIC.

You must ALWAYS dismiss from the provider.

- Dismissal Level is Provider, enter the provider's name
- Reason is Non-Compliance
- Start date is 30 days from date of termination letter.

Dismiss the patient from the department (clinic).

- Dismissal Level is Department, enter department (clinic name)
- Reason is Non-Compliance
- Start date is 30 days from date of termination letter.

[Dismissing Patient - EPIC workflow](#)

You may also remove the provider from the care team - **AFTER** the letter has been mailed and the patient has been dismissed in EPIC. Please take note of the termination date and use that date when removing the provider from the care team.

- I. Non-compliance/Non-adherence Process (Not answering/returning call, no self-test results, cancelled labs, 6MF2F, POC, MD, SNF discharge f/u)
 - a. Call #1 → Move 2 business days
 - b. Call #2 → Move 1 week from Call #1
 - c. Call #3 → If no answer, document non-compliance and move out 4 weeks (Route to TEC Anticoag Clerical)
 - d. If no contact from patient, send physician warning letter non-compliance → Move out 2 weeks
 - e. If no contact from patient, review for termination → sent physician termination letter.
 - f. Any patient who has been discharged from the Temple Anticoagulation clinic must follow the non-compliance protocol (see Temple Anticoagulation clinic policy located on Baylor Scott and White Connect) before patient can re-enroll with the clinic. The patient must show compliance with their current Baylor Scott and White provider who is managing the anticoagulant. The patient must have three consecutive INRs within a 6-week time frame and then Baylor Scott and White provider can send a new referral with prior authorization in order to be approved for re-enrollment with the Temple Anticoagulation Clinic. Hospitalization INRs do NOT count.

II. Letters

- a. No show
 - i. [1st No Show Letter \(Established\)](#)
 - ii. [1st No Show Letter \(New Patient\)](#)
 - iii. [2nd No Show Letter \(Established\)](#)
 - iv. [3rd No Show Termination Letter](#)
- b. [Physician Warning Letter \(Non-Compliance\)](#)
- c. [Physician Termination Letter \(Non-Compliance\)](#)

XIV. HOME HEALTH NUMBERS/LOCAL LABS

-- Accent Care (Texas HH)
Temple- Phone 254-218-7070 Fax 254-774-6844
Waco- Phone 254-755-6179 Fax 254-227-6066
Taylor/Hutto - Phone 512-755-8005 Fax 512-852-4770
Victoria- Phone 361-576-4683 Fax 361-574-7887
Brenham- Phone 979-830-0105 Fax 979-421-9631
Bryan 979-693-0459 979-846-1283
Longview (carthage) phone 903-690-9203 fax 903-690-0019
Round Rock phone 512-238-6000 fax 512-238-9559

Accent Care Hospice- Phone 512-372-4194 Fax 254-757-1468
--Ascension at Home- fax 844-308-5074
--Advent Health Rollins- Phone 512-556-3682
--At Home Health (Vital Caring)
Temple- Phone 254-751-1600 Fax 254-751-1604
Georgetown- Phone 512-212-1943 Fax 512-212-7886
Henderson- Phone 903-657-1004 Fax 903-657-2260
--Advent Health Rollins- Phone 512-556-3682 ext 6 (lab)
--Alumin HH -Phone 979-704-6684 Fax 979-704-6690
--Amedisys HH -Phone 254-231-0825 Fax- 833-764-1709
--Brookdale Western HH- Phone 512-372-1595 Fax 254-791-8003
--Broadmoor Court Assisted Living- Phone 979 589 7938 Fax 979 776 0675

Blue Bonnett HH - Phone 254-772-5577 Fax 254-772-5588

-- Center Well (Kindred) HH
Temple- Phone 254-778-6334 Fax 254-778-6524
Waco- Phone 254-405-6800 Fax 254-741-1559
--Coryell Lab phone 254-248-6270
--Daybreak fax 254-757-3434
--Enhabit (Encompass) HH Phone 254-773-7740 Fax 254-773-7745
College Station Phone (979) 764-9000 Fax 979-764-9001
--Elara Care (Jordan) HH Phone 254-554-3500 Fax 254-554-3458
--Teague Phone 903-723-1657 Fax 903-723-5227
--Enclave Assisted Living Fax 512-996-9722
--Faith Shining Light HH Fax 512-727-9589
--Garden Estates Fax 254-326-5166
--Interim HH



Temple- Phone 254-771-4131 Fax 254-771-0752
Waco- Phone 254 751-9393 Fax 254-301-7037

--Integrity HH fax 254-628-7905

--Killeen Nursing and Rehab Fax 254-698-2355

--Lee HH Phone 512-564-5002 Fax 512-564-5102
Hamilton -Fax 254-386-5040

--Lynn's Pharmacy- Phone 254-666-5000 Fax 254-666-5002

-- Lillie's Lab Fax 254-312-5020

--Missions HH Phone 210-524-2400 Fax 210-524-2414

--mdINR enrollment Fax 855-975-4072

--Morada Fax 254-771-3425 or 254774-7472
--Outerbanks hospital (med records)- Phone 252-449-4520
--Pharmacy Alternatives Fax 866-370-4347

--Providence HH- Fax 254-399-6474
--Paradigm HH Fax 855-391-0372
--Ridgecrest Assisted living Phone 254-776-9681 Fax 254-776-7960
--Rosewood Assisted living Fax 254-690-0135 or 866-208-5596



--Reliable (Halcyon) hh Phone 254-772-1025 Fax 254-772-1029

--Standards HH Phone 254-778-7000 Fax 254-778-7002

--Stoney Brook

Temple Phone(254)613-2400/254-933-3800 fax 254-933-2550

Hewitt- Fax 254-420-1401

Copperas Cove 254-547-1788

--Soldalis Assisted Living Phone 254-778-2222 Fax 254-778-2845

--Sotera HH phone 979-693-3208 Fax 979-314-9002

--Sienna HH Phone 281-516-0255 Fax 281-516-0223

--Tex Med HH Phone 254-526-8188 Fax 254-526-8120

--Texas Quality HH Phone 254-803-3000 Fax 254-883-3530

--Traditions HH Phone 979-822-5511 Fax 979-822-3709

--Weston Inn Phone 254 298-7300 Fax 201-917-2571 or 254-742-000

Western Hills Phone 254-742-7500 Fax 201-845-9599

--Wildflower Place Fax 254-742-0425

--Waco Hillcrest ACC Fax-254-202-9959